Answer Key & Rationales · Form 6

Quick key. **P** = pretest (unscored; do not count). Bank 2026.10.23-0423.

#	Key	Area	Sub-area	P
1	**B**	III	III.B	
2	**A**	II	II.A	
3	**C**	II	II.B	
4	**B**	III	III.B	
5	**A**	I	I.C	
6	**A**	II	II.C	
7	**B**	I	I.B	
8	**D**	II	II.B	
9	**A**	I	I.B	
10	**B**	I	I.A	
11	**A**	III	III.B	
12	**C**	III	III.B	
13	**B**	I	I.A	
14	**D**	II	II.B	
15	**C**	II	II.A	
16	**A**	III	III.A	
17	**B**	II	II.A	
18	**D**	III	III.A	
19	**B**	I	I.C	
20	**C**	II	II.C	
21	**B**	II	II.A	
22	**D**	I	I.A	
23	**C**	I	I.C	
24	**B**	II	II.A	
25	**A**	I	I.A	
26	**D**	I	I.B	
27	**B**	III	III.A	
28	**D**	I	I.B	
29	**A**	III	III.B	**P**
30	**C**	I	I.B	**P**
31	**C**	II	II.B	**P**
32	**B**	II	II.C	
33	**A**	III	III.D	
34	**A**	III	III.B	
35	**C**	I	I.B	
36	**B**	III	III.B	
37	**A**	I	I.C	
38	**A**	III	III.C	
39	**C**	II	II.B	
40	**D**	I	I.C	
41	**C**	I	I.B	
42	**D**	II	II.A	
43	**B**	I	I.B	
44	**A**	II	II.A	
45	**C**	III	III.C	
46	**C**	I	I.A	
47	**B**	I	I.B	**P**
48	**A**	II	II.B	**P**
49	**D**	III	III.B	
50	**B**	I	I.B	**P**
51	**B**	II	II.A	
52	**A**	I	I.B	
53	**A**	III	III.B	
54	**B**	II	II.B	
55	**C**	I	I.A	
56	**A**	II	II.B	
57	**B**	II	II.A	
58	**A**	III	III.B	
59	**A**	I	I.B	
60	**D**	III	III.D	
61	**C**	III	III.B	
62	**B**	III	III.B	
63	**A**	II	II.A	
64	**C**	I	I.C	
65	**A**	II	II.A	
66	**C**	III	III.A	
67	**D**	I	I.B	
68	**C**	II	II.C	
69	**A**	III	III.B	**P**
70	**C**	I	I.A	
71	**A**	I	I.C	
72	**C**	III	III.B	**P**
73	**A**	II	II.A	
74	**B**	II	II.B	
75	**B**	III	III.B	**P**
76	**A**	III	III.D	
77	**A**	III	III.B	
78	**A**	I	I.B	
79	**B**	I	I.B	
80	**C**	II	II.A	
81	**A**	III	III.B	
82	**B**	I	I.B	
83	**A**	III	III.B	
84	**A**	II	II.B	**P**
85	**D**	II	II.B	
86	**D**	II	II.A	
87	**B**	III	III.B	
88	**B**	III	III.B	
89	**C**	II	II.A	
90	**B**	II	II.B	
91	**A**	II	II.A	**P**
92	**D**	I	I.C	
93	**C**	III	III.C	
94	**C**	II	II.A	
95	**A**	I	I.A	
96	**D**	III	III.B	
97	**B**	II	II.A	
98	**D**	III	III.A	
99	**C**	I	I.C	
100	**A**	II	II.A	
101	**A**	I	I.B	
102	**C**	I	I.B	
103	**C**	I	I.B	**P**
104	**A**	II	II.C	
105	**C**	I	I.A	
106	**C**	III	III.A	
107	**A**	I	I.B	
108	**C**	I	I.A	
109	**B**	III	III.D	
110	**A**	III	III.B	
111	**C**	I	I.C	
112	**C**	I	I.A	
113	**A**	II	II.B	
114	**A**	I	I.B	
115	**B**	I	I.A	
116	**D**	III	III.B	
117	**B**	II	II.B	
118	**B**	I	I.C	
119	**A**	III	III.C	
120	**B**	III	III.C	
121	**C**	III	III.B	
122	**B**	II	II.B	

Scoring worksheet

Count correct answers on **scored items only**. Item numbers for each sub-area are listed; pretest items are left out.

Area / sub-area	Scored item numbers	Items	Correct
I. Values and Ethics		**40**	**____ / 40**
I.A Ethical Principles and Responsibilities	10, 13, 22, 25, 46, 55, 70, 95, 105, 108, 112, 115	12	____
I.B Ethical Service Delivery	7, 9, 26, 28, 35, 41, 43, 52, 59, 67, 78, 79, 82, 101, 102, 107, 114	17	____
I.C Diversity and Social Justice	5, 19, 23, 37, 40, 64, 71, 92, 99, 111, 118	11	____
II. Assessment and Planning		**35**	**____ / 35**
II.A Assessment Concepts	2, 15, 17, 21, 24, 42, 44, 51, 57, 63, 65, 73, 80, 86, 89, 94, 97, 100	18	____
II.B Assessment Methods and Techniques	3, 8, 14, 39, 54, 56, 74, 85, 90, 113, 117, 122	12	____
II.C Assessment Practices	6, 20, 32, 68, 104	5	____
III. Intervention and Practice		**35**	**____ / 35**
III.A Practice Concepts	16, 18, 27, 66, 98, 106	6	____
III.B Intervention Methods and Techniques	1, 4, 11, 12, 34, 36, 49, 53, 58, 61, 62, 77, 81, 83, 87, 88, 96, 110, 116, 121	20	____
III.C Practice Evaluation and Research	38, 45, 93, 119, 120	5	____
III.D Supervision and Administration	33, 60, 76, 109	4	____

Pretest items (skip when scoring): 29, 30, 31, 47, 48, 50, 69, 72, 75, 84, 91, 103

Total correct (scored items)	_____ / 110
Practice pass line	77 / 110 (70%)
Correct needed to reach the line	77 − your total = _____

The real exam sets its passing score separately for each form (it is reported as a scaled score). 77 of 110 is a deliberately cautious practice target that builds a margin; it is not a prediction of your real result.

Score conversion (raw out of 110 → percent)

The boxed row is the practice pass line: **77 / 110 = 70%**. Any score at or above it meets the line.

Raw	%	Raw	%	Raw	%	Raw	%	Raw	%	Raw	%
110	100%	91	83%	72	65%	53	48%	34	31%	15	14%
109	99%	90	82%	71	65%	52	47%	33	30%	14	13%
108	98%	89	81%	70	64%	51	46%	32	29%	13	12%
107	97%	88	80%	69	63%	50	45%	31	28%	12	11%
106	96%	87	79%	68	62%	49	45%	30	27%	11	10%
105	95%	86	78%	67	61%	48	44%	29	26%	10	9%
104	95%	85	77%	66	60%	47	43%	28	25%	9	8%
103	94%	84	76%	65	59%	46	42%	27	25%	8	7%
102	93%	83	75%	64	58%	45	41%	26	24%	7	6%
101	92%	82	75%	63	57%	44	40%	25	23%	6	5%
100	91%	81	74%	62	56%	43	39%	24	22%	5	5%
99	90%	80	73%	61	55%	42	38%	23	21%	4	4%
98	89%	79	72%	60	55%	41	37%	22	20%	3	3%
97	88%	78	71%	59	54%	40	36%	21	19%	2	2%
96	87%	**77**	**70%**	58	53%	39	35%	20	18%	1	1%
95	86%	76	69%	57	52%	38	35%	19	17%	0	0%
94	85%	75	68%	56	51%	37	34%	18	16%		
93	85%	74	67%	55	50%	36	33%	17	15%		
92	84%	73	66%	54	49%	35	32%	16	15%		

Per-area practice guide: aim for at least 70% in each area as well as overall (Area I 28 of 40; Areas II and III 25 of 35 each).

Rationales for every item

Section 1

1. INT-1042 (rev 2) · Best answer: B

In a family session about a 12-year-old who refuses to go to school, the social worker asks the older sibling, 'When your grandparent and your parent argue about getting to school, who steps in first? Who seems most worried afterward?' Which technique is the social worker **MOST** likely using?

A. Process questioning
B. Circular questioning
C. Tracking
D. Relative influence questioning

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Bowen process questions invite a family member to reflect calmly on their own thinking and part in the pattern. Here the sibling is asked to report on and compare other members' reactions, which is the Milan approach.

B **Correct.** Circular questioning, from the Milan systemic team, asks one family member about relationships and differences among others, such as who reacts first or who is most upset (Selvini Palazzoli et al., 1980). Asking the sibling to describe the grandparent-parent disagreement and rank reactions brings out the family's relational patterns around the symptom. It beats A because Bowen process questions ask a person to think about their own reactions and role, to lower emotional reactivity, rather than to report on how others relate.

C **Incorrect** (trap: Wrong model or concept). Tracking is a structural joining skill: following the content of what the family says and encouraging them to go on. It does not ask one member to compare how others react.

D **Incorrect** (trap: Wrong model or concept). Relative influence questions, from narrative therapy, map how the problem affects people and how people affect the problem. These questions compare family members' reactions to one another instead.

Takeaway: Milan circular questions ask one member about differences and relationships among others, revealing the family's relational patterns.

Sources: Selvini Palazzoli, M., Boscolo, L., Cecchin, G., & Prata, G. (1980). Hypothesizing-circularity-neutrality: Three guidelines for the conductor of the session. Family Process, 19(1), 3-12
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

2. ASM-1532 (rev 1) · Best answer: A

Research on families of adults with schizophrenia links which family pattern to higher relapse rates?

A. High expressed emotion
B. Double-bind communication
C. Disengaged boundaries
D. Emotional cutoff

Why each option is right or wrong

A **Correct.** Expressed emotion refers to relatives' critical comments, hostility and emotional overinvolvement toward a family member with mental illness. A meta-analysis found that living with relatives high in expressed emotion predicts relapse in schizophrenia (Butzlaff & Hooley, 1998), which is why family psychoeducation targets these patterns. It beats B because double-bind theory claimed that contradictory parental messages cause schizophrenia, an idea research did not support; expressed emotion affects the course of the illness, not its origin.

B **Incorrect** (trap: Wrong model or concept). Bateson and colleagues (1956) proposed that contradictory messages in families cause schizophrenia. The theory shaped early family therapy but was not supported by research, and blaming families for causing the illness is now rejected.

C **Incorrect** (trap: Wrong model or concept). Disengaged boundaries describe family members who are emotionally distant and underinvolved. Relapse research points instead to criticism and overinvolvement, which sit closer to the opposite of disengagement.

D **Incorrect** (trap: Wrong model or concept). Emotional cutoff is Bowen's term for managing unresolved family attachment through distance or silence. It is not the pattern that relapse studies in schizophrenia have measured.

Takeaway: Relatives' criticism, hostility and overinvolvement (high expressed emotion) predict schizophrenia relapse; families influence the course, not the cause.

Sources: Butzlaff, R. L., & Hooley, J. M. (1998). Expressed emotion and psychiatric relapse: A meta-analysis. Archives of General Psychiatry, 55(6), 547-552, whole article; Bateson, G., Jackson, D. D., Haley, J., & Weakland, J. (1956). Toward a theory of schizophrenia. Behavioral Science, 1(4), 251-264, double-bind hypothesis
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.2 Family functioning and dynamics and their effects · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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3. ASM-1133 (rev 3) · Best answer: C

In a hospice bereavement program, a 79-year-old client whose spouse died 4 months ago mentions hearing the spouse say 'good night' most evenings. The client then looks away and asks quietly, 'Does that mean I'm losing my mind?' The client has not told anyone about this before. What should the social worker do **FIRST**?

A. Refer the client for a psychiatric evaluation of hallucinations
B. Tell the client that hearing a late spouse is common in grief
C. Ask what the experience is like and how the client makes sense of it

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). A psychiatric evaluation fits if the voice brings commands, distress, other psychotic signs or a decline in functioning. None of that is known yet, and an immediate referral would confirm the client's fear of being seen as mentally ill.

B **Incorrect** (trap: Right action, wrong time). This is probably true, and normalizing will likely help once the social worker learns that the client knows the spouse died, finds the voice comforting and is functioning well. Given before any exploration, it answers a question whose facts are still unknown and may cut off what the client wanted to say.

C **Correct.** Before answering, the social worker needs to know what the experience is like: whether the client knows the spouse died, whether the voice comforts or frightens, and whether commands, low mood or poor functioning are present. Asking also shows the client the question is safe to raise. Sensing a dead spouse is common in widowhood (Rees, 1971), and DSM-5-TR treats uncomplicated bereavement as a focus of attention, not a disorder, but that judgment rests on these details. It beats B because reassurance offered before the facts are known may be wrong and can close off a frightened client.

Takeaway: When a grieving client reports sensing the deceased and fears it means mental illness, explore the experience before normalizing or referring.

Sources: Rees, W. D. (1971). The hallucinations of widowhood. British Medical Journal, 4(5778), 37-41, whole article; DSM-5-TR (APA, 2022), Other Conditions That May Be a Focus of Clinical Attention: Uncomplicated Bereavement

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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4. INT-1035 (rev 2) · Best answer: B

Before discharge from an inpatient unit, a client with social anxiety must call a landlord to ask about a lease. The client watches the social worker's confident manner and says, 'I could never sound as smooth as you.' How should the social worker **BEST** demonstrate the call?

A. Model the call smoothly and confidently from start to finish

B. Model the call with some hesitation and spoken coping self-talk

C. Describe each step aloud and let the client try the call first

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). A smooth demonstration shows the skill clearly and suits confident learners. This client has just said they could never sound that smooth, so a flawless model may widen the gap they feel.

B **Correct.** Observers who doubt their ability gain more from a coping model, who shows some struggle and works through it, than from a mastery model who performs flawlessly (Schunk & Hanson, 1989). The client's remark shows they see the worker as unlike them. Modeling a pause, a small stumble, and spoken self-talk such as 'Slow down, just ask the question' shows a path the client can picture following (Bandura, 1977). It beats A because a polished demonstration confirms the client's belief that the gap is too wide to cross.

C **Incorrect** (trap: Right action, wrong time). Talking through the steps helps, but sending an anxious client to try first skips the demonstration that builds confidence. Modeling usually comes before the client's first attempt.

Takeaway: For clients who doubt themselves, demonstrate as a coping model, showing some struggle and self-talk, rather than a flawless performance.

Sources: Schunk, D. H., & Hanson, A. R. (1989). Influence of peer-model attributes on children's beliefs and learning. Journal of Educational Psychology, 81(3), 431-434, coping versus mastery models; Bandura, A. (1977). Social Learning Theory. Prentice-Hall, observational learning: attention, retention, reproduction, motivation
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.26 Using role modeling to teach behavior · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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5. ETH-1613 (rev 1) · Best answer: A

In a rural primary care clinic, a 52-year-old male farmer is referred to the social worker after telling the physician he is 'just worn out.' He has been irritable, drinking more beer at night and snapping at his family. When the social worker mentions depression, he says, 'I'm not some weak guy who needs to cry about it.' Which response is **MOST** helpful?

A. Talk about the problems in his terms, such as stress and sleep
B. Explain that depression is a medical illness like any other
C. Refer him to a substance use program for the drinking

Why each option is right or wrong

A **Correct.** Traditional masculine norms of self-reliance and emotional control shape how many men experience and show distress, often as irritability, drinking or fatigue, and make labels like 'depression' feel threatening (Addis & Mahalik, 2003). Starting with the client's own words, such as being worn out, sleep and stress, lowers that barrier and builds engagement while assessment continues. It beats B because correcting his view, though accurate, argues with him at the first sign of reluctance and can end engagement before it starts.

B **Incorrect** (trap: Too directive). Teaching about depression can reduce stigma later. Offered right after he rejects the label, it argues against his view of himself and may increase defensiveness.

C **Incorrect** (trap: Hands off too soon). The increased drinking needs assessment, and a substance use referral may follow. Referring now, before engaging him or assessing the pattern, hands off a client who has not yet agreed there is a problem.

Takeaway: Gender norms shape how distress is shown and named; engage clients in their own terms rather than pressing a label they reject.

Sources: Addis, M. E., & Mahalik, J. R. (2003). Men, masculinity, and the contexts of help seeking. American Psychologist, 58(1), 5-14, whole article
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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6. ASM-1611 (rev 2) · Best answer: A

A hospice social worker visits an 86-year-old patient with advanced heart failure. The patient's mood is low, and the patient says, 'I keep going over the past. I want to know it all counted for something before I go.' The patient is alert and has a few months to live. Which approach is **MOST** helpful?

A. Guide a structured life review
B. Begin exposure therapy for death-related anxiety
C. Schedule pleasant activities to lift daily mood

Why each option is right or wrong

A **Correct.** Modality should fit life stage. Butler described the life review as a natural late-life process of revisiting the past that can bring resolution (Butler, 1963), and Erikson framed the final stage as integrity versus despair (Erikson, 1963). This patient is already reviewing the past and wants to find meaning, with only months to live, so a guided life review builds on what the patient is doing. It beats C because activity scheduling targets low mood in general but does not answer the patient's stated need for meaning.

B **Incorrect** (trap: Wrong model or concept). Exposure therapy targets fears that drive avoidance, such as phobias or trauma reminders. The patient is not avoiding thoughts of death but seeking meaning in the past, which calls for a different approach.

C **Incorrect** (trap: Good but not best). Scheduling pleasant activities can lift mood and may help as well, within the limits of advanced heart failure. It does not address the patient's request to make sense of a whole life, the central task at this stage.

Takeaway: Match modality to life stage: older adults near the end of life often benefit from guided life review aimed at integrity and meaning.

Sources: Butler, R. N. (1963). The life review: An interpretation of reminiscence in the aged. Psychiatry, 26(1), 65-76, whole article; Erikson, E. H. (1963). Childhood and Society (2nd ed.). W. W. Norton, eight ages: integrity versus despair

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.3 Choosing intervention modality by ability, development, culture, life stage · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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7. ETH-1527 (rev 1) · Best answer: B

A client with bipolar I disorder has been stable for a year at a community mental health center. The client says that during past manic episodes, hospital staff gave medications that caused severe side effects and ignored the client's wishes. The client wants future care to reflect those preferences. What is the **BEST** way for the social worker to help?

A. Ask the psychiatrist to add the preferences to the chart
B. Help the client complete a psychiatric advance directive
C. Suggest the client tell a relative which medications to refuse
D. Reassure the client that hospitals follow stated wishes

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A chart note helps the local team, but it may not reach a hospital during a future crisis and carries little legal weight there. It is a useful extra step, not the best way to protect the client's choices.

B **Correct.** A psychiatric advance directive is a legal document, written while the client is well, that records treatment preferences and can name a trusted agent for future crises (SAMHSA, 2019). It turns this client's wishes about medications into a document hospital staff can find and weigh, which supports self-determination even when capacity is reduced (NASW Code 1.02). It beats A because a chart note at one clinic may never reach an emergency hospital and carries little legal weight; the directive travels with the client and is built for this purpose.

C **Incorrect** (trap: Good but not best). Telling a relative helps, but informal word of mouth can be doubted or forgotten in a crisis, and the relative has no recognized role. A directive can formally name that person as an agent if the client wishes.

D **Incorrect** (trap: Misreads law or ethics). This reassurance is inaccurate. Wishes stated only in conversation can be hard to verify during a crisis, and treatment decisions vary with law and circumstances. Recording the preferences in a directive gives them more weight.

Takeaway: Help clients with recurring crises record treatment preferences in a psychiatric advance directive while they are well.

Sources: SAMHSA (2019). A Practical Guide to Psychiatric Advance Directives, What is a psychiatric advance directive; NASW Code of Ethics (2021), 1.02
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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8. ASM-1031 (rev 2) · Best answer: D

A client meets criteria for a major depressive episode and, on most days of the episode, also has racing thoughts, pressured speech and increased goal-directed activity. Which DSM-5-TR specifier applies?

A. With anxious distress
B. With rapid cycling
C. With atypical features
D. With mixed features

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Anxious distress means feeling keyed up, restless, worried or afraid something awful may happen. Restlessness can look similar, but racing thoughts, pressured speech and more goal-directed activity are manic symptoms.

B **Incorrect** (trap: Wrong model or concept). Rapid cycling is a specifier for bipolar disorders when 4 or more mood episodes occur within 12 months. It describes how often episodes recur across a year, not manic symptoms occurring within a single depressive episode.

C **Incorrect** (trap: Wrong model or concept). Atypical features involve mood that brightens with good events, oversleeping, increased appetite and sensitivity to rejection. They do not include racing thoughts or pressured speech.

D **Correct.** The mixed-features specifier applies when at least three manic or hypomanic symptoms, such as racing thoughts, pressured speech or increased goal-directed activity, occur on most days of a depressive episode without meeting full manic or

hypomanic criteria (DSM-5-TR). It matters because mixed features are a risk factor for later bipolar disorder and shape treatment planning. It beats A because anxious distress describes tension, worry and fear of losing control, not the speeded-up thinking and activity of manic symptoms.

Takeaway: Three or more manic symptoms on most days of a depressive episode call for the 'with mixed features' specifier, a bipolar-risk signal.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Specifiers for Depressive Disorders (With Mixed Features)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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9. ETH-1690 (rev 2) · Best answer: A

Under the McKinney-Vento Act, what may a student do when the family becomes homeless and moves to a shelter in another district?

A. Remain in the school of origin, with transportation provided
B. Enroll in the new district once school records are transferred
C. Stay at the original school until the semester ends, then transfer

Why each option is right or wrong

A **Correct.** The McKinney-Vento Act presumes that keeping a homeless student in the school of origin, the school last attended when housed or last enrolled, is in the student's best interest unless the parent asks otherwise, and transportation must be provided at the parent's request (42 U.S.C. 11432(g)(3)(B); (g)(1)(J)(iii)). The family may instead choose the local school, which must enroll the student immediately. It beats C because the right lasts for the duration of homelessness, not just to the end of a term.

B **Incorrect** (trap: Misreads law or ethics). The family may choose the new district's school, but the law requires immediate enrollment even without the records normally required. Waiting for records would wrongly keep the student out of school.

C **Incorrect** (trap: Misreads law or ethics). The right to stay is not limited to the current semester, and the student need not transfer when it ends. It lasts for the duration of homelessness and through the end of the school year in which the student becomes permanently housed.

Takeaway: Homeless students may stay in their school of origin with transportation, or enroll immediately in the local school; the family's preference gets priority.

Sources: McKinney-Vento Homeless Assistance Act, 42 U.S.C. 11432(g)(3); McKinney-Vento Homeless Assistance Act, 42 U.S.C. 11432(g)(1)(J)(iii)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.8 How local, state, federal policy affects service delivery · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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10. ETH-1019 (rev 3) · Best answer: B

A client who has finished treatment at a community agency asks the social worker for a full copy of their record. The social worker worries the client will be upset by the frank descriptions of the client's anger outbursts in the progress notes. What is the **BEST** response?

A. Release the copy after a meeting to review it together
B. Provide the copy and offer to go over it together
C. Give a written summary instead of the requested copy

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Reviewing the record together can help, so this is tempting. Requiring a meeting before release, though, puts a condition on a right of access the client already holds; the client decides whether to go over the record with the worker (NASW Code 1.08(a); 45 CFR 164.524).

B **Correct.** Clients have a right to reasonable access to their records, and the social worker helps them understand what the records say (NASW Code 1.08(a)). Access may be limited only in exceptional cases with compelling evidence of serious harm. Likely upset does not meet that bar, and HIPAA gives a similar right of access (45 CFR 164.524). Offering to review the record together supports the client in reading frank notes. B beats C because a summary falls short of the full copy the client asked for and is entitled to.

C **Incorrect** (trap: Good but not best). A summary can be offered as an extra if the client wants one, but the client asked for the full record and has a right to it. Substituting a summary limits access without a valid reason.

Takeaway: Give clients access to their records and help them understand the content; limit access just for compelling evidence of serious harm.

Sources: NASW Code of Ethics (2021), 1.08(a); HIPAA Privacy Rule, 45 CFR 164.524
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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11. INT-1166 (rev 1) · Best answer: A

Which family therapy model focuses on hidden loyalties and on what each generation owes and is owed?

A. Contextual therapy
B. Bowenian therapy
C. Symbolic-experiential therapy
D. Milan systemic therapy

Why each option is right or wrong

A **Correct.** Contextual therapy, developed by Ivan Boszormenyi-Nagy, centers on relational ethics: invisible loyalties to family members and a multigenerational ledger of what each person owes and is owed (Boszormenyi-Nagy & Spark, 1973). Therapy aims to restore fairness and trust among members. It beats B, Bowenian therapy, because Bowen also looks across generations but focuses on differentiation of self and emotional transmission, not on loyalty, entitlement, and fairness.

B **Incorrect** (trap: Wrong model or concept). Bowen therapy is also multigenerational, using genograms and the multigenerational transmission process. Its core concept is differentiation of self, not loyalty and fairness.

C **Incorrect** (trap: Wrong model or concept). Whitaker's symbolic-experiential therapy emphasizes spontaneity, the battles for structure and initiative, and the therapist's use of self. It does not center on fairness between generations.

D **Incorrect** (trap: Wrong model or concept). Milan therapy uses hypothesizing, circular questions, neutrality, and positive connotation to change family patterns. Loyalty ledgers are not part of the model.

Takeaway: Contextual therapy addresses invisible loyalties and intergenerational fairness; Bowen therapy addresses differentiation and emotional transmission across generations.

Sources: Boszormenyi-Nagy, I., & Spark, G. M. (1973). Invisible Loyalties: Reciprocity in Intergenerational Family Therapy. Harper & Row, invisible loyalties; the family ledger

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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12. INT-1008 (rev 1) · Best answer: C

In an integrated primary care clinic, a client with type 2 diabetes tells the social worker, 'I bought walking shoes over the weekend and went around the block twice this week.' In motivational interviewing, which type of change talk is this?

A. Commitment
B. Ability
C. Taking steps
D. Desire

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Commitment language states an intention or promise about the future, as in 'I'm going to start walking.' This client is describing actions already completed, not a pledge.

B **Incorrect** (trap: Wrong model or concept). Ability language expresses that change is possible, such as 'I could probably walk after dinner.' This client reports having done it, not being able to.

C **Correct.** Motivational interviewing sorts change talk into preparatory types (desire, ability, reasons, need) and mobilizing types (commitment, activation, taking steps) (SAMHSA TIP 35, 2019). Buying shoes and walking twice describe actions the client has already taken toward change, which is taking steps. It beats A because commitment language states future intent, such as 'I will walk every day,' while this client reports what they have already done.

D **Incorrect** (trap: Wrong model or concept). Desire language expresses wanting change, such as 'I wish I were more active.' The statement here reports behavior already carried out, not a wish.

Takeaway: Change talk describing actions already taken toward change is taking steps; commitment language states future intent.

Sources: SAMHSA TIP 35 (2019), Ch. 3: Motivational Interviewing as a Counseling Style
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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13. ETH-1015 (rev 3) · Best answer: B

A social worker in a rural county wants advice from an out-of-town peer consultation group about a client with complex grief. The client is the town's only veterinarian, and the case involves a well-known local accident. How can the social worker **MOST** effectively protect the client's identity while consulting?

A. Refer to the client by initials during the consultation
B. Omit or change details that could reveal who the client is
C. Wait to consult until the client agrees to the consultation
D. Share full details, since consultants also owe confidentiality

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Initials are a common shortcut, but they hide just the name. A unique occupation and a widely reported local accident can still identify the client, so the identifying details themselves must be removed.

B **Correct.** When consulting, social workers share the least information needed and avoid revealing a client's identity unless the client consents or there is a compelling need (NASW Code 1.07(v); NASW Code 2.05(c)). In a small community, a job and a well-known event can identify someone even without a name, so the worker removes or alters such details. B beats A because initials hide the name but leave the unique job and local accident that could identify the client.

C **Incorrect** (trap: Good but not best). Consent would allow sharing identifying details, but it is unnecessary for de-identified consultation and delays help the case may need now. It also does nothing to protect identity, which is the stated goal.

D **Incorrect** (trap: Misreads law or ethics). Consultants do owe confidentiality, but the Code still limits what is shared to what the consultation needs. Full details expose identifying facts that the consultation does not require.

Takeaway: In consultation, remove every detail that could identify the client, not just the name, especially in small communities.

Sources: NASW Code of Ethics (2021), 1.07(v); NASW Code of Ethics (2021), 2.05(c)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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14. ASM-1176 (rev 1) · Best answer: D

In a violence risk assessment, which of these risk factors is dynamic and therefore a direct target for a risk-management plan?

A. Two prior assault convictions
B. First arrest at age 15
C. Witnessing violence between parents
D. Current heavy weekend drinking

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Past violence is the strongest single predictor of future violence, which makes it tempting. It is a static, historical factor, though: no plan can change it, so it raises the baseline risk estimate rather than serving as a treatment target.

B **Incorrect** (trap: Near-miss diagnosis). Early onset of offending is a static historical factor. It informs the overall level of risk, but nothing done now can change the age of first arrest.

C **Incorrect** (trap: Near-miss diagnosis). Childhood exposure to violence between parents is a historical risk factor. It shapes the risk formulation and may guide trauma-informed care, but the exposure itself cannot be changed by a plan.

D **Correct.** Dynamic risk factors are changeable conditions, such as current substance use, active symptoms or unstable housing. Static factors are fixed history. Risk-management plans target dynamic factors because reducing them can lower risk (Douglas & Skeem, 2005). Current heavy drinking can be treated and monitored. It beats A because, although prior violence is the strongest predictor, it is static and cannot be changed by any plan.

Takeaway: Static risk factors set baseline risk; dynamic factors such as current substance use are what a violence risk-management plan can change.

Sources: Douglas, K. S., & Skeem, J. L. (2005). Violence risk assessment: Getting specific about being dynamic. Psychology, Public Policy, and Law, 11(3), 347-383, whole article
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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15. ASM-1531 (rev 1) · Best answer: C

When one family member's symptoms improve, another member sometimes develops problems. In family systems theory, what term describes the family's pull to keep its usual patterns?

A. Equifinality
B. Morphogenesis
C. Homeostasis
D. Family projection process

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Equifinality is the systems idea that different starting points can lead to the same outcome, so a family's history does not fix its future. It does not describe resistance to change after a member improves.

B **Incorrect** (trap: Wrong model or concept). Morphogenesis is a system's ability to change its structure and rules in response to new demands. It is the counterpart of the tendency described, which keeps old patterns in place.

C **Correct.** Homeostasis is a family system's tendency to keep its familiar balance of patterns and roles. Jackson (1957) observed that when one member improves, another may develop symptoms or the family may pull the member back into the old role, because the change upsets the system. It beats B because morphogenesis is the opposite process, a system's capacity to change its structure; the tendency described here works against change.

D **Incorrect** (trap: Wrong model or concept). The family projection process is Bowen's term for parents passing their anxiety onto a child, who becomes the focus of family concern. It explains how one child becomes symptomatic, not why symptoms shift to another member after improvement.

Takeaway: Families tend to preserve familiar patterns; when one member improves, another may become symptomatic unless the whole system adjusts.

Sources: Jackson, D. D. (1957). The question of family homeostasis. Psychiatric Quarterly Supplement, 31, 79-90, whole article

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.2 Family functioning and dynamics and their effects · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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16. INT-1556 (rev 1) · Best answer: A

During a home hospice visit, the social worker finds the patient moaning and grimacing with each movement. The patient's adult child, the main caregiver, says they have been skipping some prescribed pain medicine doses because they fear it will cause addiction or hasten death. What should the social worker do **FIRST**?

A. Ask the on-call nurse to reassess the pain now

B. Reassure the caregiver that pain medicine rarely hastens death

C. Explore the caregiver's fears about the patient's pain medicine

Why each option is right or wrong

A **Correct.** Uncontrolled pain at the end of life is an urgent symptom, and assessing pain and adjusting medication belong to the hospice nurse and physician, not the social worker (NASW Code 1.04(a)). The patient is in visible distress now, so the first step is bringing in the nurse to reassess today; the team then addresses the caregiver's fears together (NASW Serious Illness Care Standard 5). It beats C because exploring fears matters but leaves the patient in pain while the conversation unfolds.

B **Incorrect** (trap: Outside the social work role). Addressing the fear is worthwhile, but explaining medication effects is the clinical team's role, and reassurance does not relieve pain that is happening now. The nurse can give accurate information while adjusting the plan.

C **Incorrect** (trap: Right action, wrong time). Exploring the caregiver's fears is a key part of the response, and the social worker should do it. Done first, it delays help for a patient in visible pain; it fits once the nurse has been called.

Takeaway: When a hospice patient's symptoms are uncontrolled, bring in the nurse or physician immediately, then address caregiver fears alongside the team.

Sources: NASW Code of Ethics (2021), 1.04(a); NASW Practice Standards for Serious Illness Care: Hospice and Palliative Social Work, Standard 5: Collaboration
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.5 End-of-life practice: hospice, palliative care, continuity, dying stages · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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17. ASM-1045 (rev 2) · Best answer: B

At a community mental health center, the parents of a 23-year-old with schizophrenia say their adult child now showers rarely, speaks little and sits for hours without starting anything. The voices have faded with treatment. The parents call it laziness and have started criticizing the client every day. What is the social worker's **BEST** response?

A. Help the parents set firm rewards and consequences for chores
B. Explain that these behaviors are common symptoms of the illness
C. Link the client to a vocational rehabilitation program

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Structure and positive reinforcement can help with negative symptoms later. Starting with consequences while the parents see the behavior as laziness is likely to increase the criticism linked to relapse.

B **Correct.** Reduced motivation, sparse speech and poor self-care are negative symptoms of schizophrenia (avolition and alogia), signs of the illness rather than choices (DSM-5-TR). Daily criticism is a form of high expressed emotion, which predicts relapse (Butzlaff & Hooley, 1998), so helping the parents see these behaviors as symptoms comes first. It beats A because structured routines can help later, but a consequence plan framed around laziness would likely add to the criticism that raises relapse risk.

C **Incorrect** (trap: Right action, wrong time). Vocational services help many people with schizophrenia and may fit later. It does not address the family's misreading of symptoms or the daily criticism at home.

Takeaway: Help families recognize negative symptoms as illness, not laziness; reducing criticism lowers relapse risk.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia; Butzlaff, R. L., & Hooley, J. M. (1998). Expressed emotion and psychiatric relapse: A meta-analysis. Archives of General Psychiatry, 55(6), 547-552, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.5 Signs of mental and emotional illness across the lifespan · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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18. INT-1655 (rev 2) · Best answer: D

On a palliative care unit, a patient with advanced pancreatic cancer asks the social worker, 'How long do I have?' The oncologist reviewed the latest scan results with the patient yesterday. No family members are present. What should the social worker do **FIRST**?

A. Tell the patient the physician is the one to answer that
B. Share the typical survival range for this stage of cancer
C. Arrange a family meeting with the oncologist this week
D. Invite the patient to say what they understand and hope to learn

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Prognosis is medical information, and the oncologist should give it. Redirecting immediately, though, skips learning what the patient understood and wants, and may feel dismissive. The handoff fits once the patient's question is clearer.

B **Incorrect** (trap: Outside the social work role). Survival estimates are medical judgments for the physician. Offering general figures also assumes the patient wants numbers, which many patients do not, and may conflict with what the oncologist has said.

C **Incorrect** (trap: Right action, wrong time). A family meeting may help later with planning and support. Scheduling it now responds to a question the patient has not asked and brings in others before learning what the patient wants to know or share.

D **Correct.** Before prognosis is discussed, sound end-of-life communication learns what the patient understands and how much they want to know, the perception and invitation steps of the SPIKES protocol (Baile et al., 2000). The patient heard the scan results yesterday, so the question may reflect confusion, fear, or a wish to plan; asking first shows which, and the social worker can then bring in the oncologist for medical details. It beats A because redirecting at once can feel like a closed door at a vulnerable moment and misses what the patient is really asking.

Takeaway: When a dying patient asks about prognosis, first learn what they understand and want to know before arranging information.

Sources: Baile, W. F., Buckman, R., Lenzi, R., Glober, G., Beale, E. A., & Kudelka, A. P. (2000). SPIKES: A six-step protocol for delivering bad news: Application to the patient with cancer. The Oncologist, 5(4), 302-311, perception and invitation steps

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.5 End-of-life practice: hospice, palliative care, continuity, dying stages · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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19. ETH-1710 (rev 1) · Best answer: B

Which policy approach sets one shared goal for an entire population and then designs different strategies for each group based on the barriers it faces?

A. Universal provision

B. Targeted universalism

C. Means-tested targeting

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Universal provision offers the same program to everyone. It shares the population-wide reach but not the tailored strategies, so groups facing greater barriers may still fall short of the goal.

B **Correct.** Targeted universalism sets a universal goal, then uses targeted strategies matched to how different groups are situated so that all can reach it (powell, Menendian & Ake, 2019). The two features in the question, one shared goal and group-specific strategies, both point here. It beats C because means-tested programs direct help only to people below an income line and set no goal for the whole population.

C **Incorrect** (trap: Wrong model or concept). Means-tested programs serve only people who pass an income or asset test. They target help, but they lack the shared, population-wide goal that defines this approach.

Takeaway: Targeted universalism pairs one goal for everyone with strategies tailored to each group's situation, so differently placed groups can all reach it.

Sources: powell, j. a., Menendian, S., & Ake, W. (2019). Targeted Universalism: Policy & Practice. Haas Institute for a Fair and Inclusive Society, Defining targeted universalism

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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20. ASM-1070 (rev 1) · Best answer: C

A client with depression has taken sertraline for 6 months with partial benefit and also takes an oral contraceptive. In session, the client shows a bottle of St. John's wort bought that morning on a friend's advice and plans to start it tonight. The next prescriber visit is in 3 weeks. What is the social worker's **BEST** response?

A. Support the plan, since herbal remedies are the client's choice

B. Explore the appeal of herbs and revisit at the prescriber visit

C. Urge a call to the prescriber or pharmacist before the first dose

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Clients may choose complementary remedies, and that choice deserves respect. Endorsing it without flagging interaction risks with sertraline and the contraceptive, however, leaves the client deciding without key safety information.

B **Incorrect** (trap: Right action, wrong time). Understanding why herbal remedies appeal is worthwhile and can continue. Waiting for the visit in 3 weeks, though, lets the client start a combination that can raise serotonin dangerously and weaken contraception.

C **Correct.** St. John's wort is a common alternative remedy, but combined with antidepressants such as sertraline it can cause a dangerous rise in serotonin, and it weakens many prescription drugs, including birth control pills (NCCIH, 2017). Because the client plans to start tonight and the prescriber visit is 3 weeks away, the interaction question needs a prescriber or pharmacist now. It beats B because exploring the client's interest, by itself, lets a risky combination begin before anyone with prescribing expertise weighs in. The client still makes the final, informed choice.

Takeaway: Before a client adds an herbal remedy to prescribed medicines, link them promptly with the prescriber or pharmacist to check interactions.

Sources: National Center for Complementary and Integrative Health (2017). St. John's Wort and Depression: In Depth, Safety

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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21. ASM-1541 (rev 2) · Best answer: B

Four months after their infant died, a couple meets with a hospital bereavement social worker for the first time. Within minutes, each partner says the other 'doesn't care' about the baby. One partner is tearful; the other sits quietly and mentions working long hours since the death. What should the social worker do **FIRST**?

A. Encourage the quieter partner to express feelings openly in sessions
B. Invite each partner to describe how they have been carrying the loss
C. Screen both partners for prolonged grief disorder
D. Help both partners see that each grieves in a different, valid way

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Pressing the quieter partner to show feelings assumes that open expression is the healthy way to grieve. Some people grieve mainly through thinking and action, and pressure can deepen that partner's sense of not being understood while siding with one style.

B **Correct.** Each partner reads the other's grief as indifference, but the social worker has not yet heard how either one is grieving. Inviting each to describe how they have been carrying the loss lets both feel heard and brings their styles into view, perhaps talk and tears for one and work or private routines for the other. Partners often grieve in different, equally valid ways, intuitive or instrumental (Doka & Martin, 2010), and seeing this eases blame, but the reframe lands best once each style has been described. It beats D because naming the pattern first can sound like a lecture that skips their pain.

C **Incorrect** (trap: Skips assessment). Prolonged grief disorder cannot be diagnosed in adults until at least 12 months after the death, and nothing here suggests impairment beyond expected grief (DSM-5-TR). Screening now treats a couple's conflict about grieving as an individual disorder.

D **Incorrect** (trap: Right action, wrong time). This reframe often helps couples who grieve differently, and it may well fit here. Offered before either partner has described their grief, it rests on an assumption and can feel dismissive of the hurt each one is carrying.

Takeaway: When grieving partners accuse each other of not caring, first hear how each one grieves; then help them see differing styles as valid.

Sources: Doka, K. J., & Martin, T. L. (2010). Grieving Beyond Gender: Understanding the Ways Men and Women Mourn (rev. ed.). Routledge, intuitive and instrumental grieving patterns; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.3 Relationship dynamics and development: families, couples, groups, polyamory · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

22. ETH-1184 (rev 1) · Best answer: D

Which core value in the NASW Code asks social workers to promote clients' right to make their own choices?

A. Social justice
B. Integrity
C. Service
D. Dignity and worth

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Social justice centers on challenging poverty, discrimination and other injustice and on equal access and meaningful participation. Participation in decisions sounds related, but the Code places the self-determination language under dignity and worth.

B **Incorrect** (trap: Wrong model or concept). Integrity means acting honestly and trustworthily, in line with the profession's values, and since 2021 caring for oneself. It guides the worker's own conduct rather than the client's right to choose.

C **Incorrect** (trap: Wrong model or concept). Service means helping people in need and addressing social problems, putting service above self-interest. It does not contain the Code's language on client self-determination.

D **Correct.** The Code's Dignity and Worth of the Person principle states that social workers promote clients' socially responsible self-determination, alongside treating each person with care and respect and balancing clients' interests with society's. Self-determination therefore rests on respect for each person's inherent worth. It beats A, social justice, which focuses on challenging injustice and ensuring access and participation for vulnerable groups rather than on the individual client's right to choose (NASW Code, Ethical Principles).

Takeaway: In the NASW Code, promoting socially responsible self-determination is part of the Dignity and Worth of the Person principle.

Sources: NASW Code of Ethics (2021), Ethical Principles: Dignity and Worth of the Person
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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23. ETH-1586 (rev 2) · Best answer: C

A school social worker reviews a year of the worker's own referral records. Black students were referred for behavior evaluations at about twice the rate of white students with similar incident reports. The worker has already flagged the five most recent referrals. What should the social worker do **NEXT**?

A. Re-review the flagged referrals alone and withdraw biased ones
B. Take an online implicit association test to gauge the bias
C. Review the referrals and the pattern with a supervisor
D. Raise the referral threshold for Black students going forward

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Rechecking one's own decisions is a sound instinct, but implicit bias works outside awareness, so a solo review may miss it. Withdrawing referrals without an outside check could also deny evaluations to students who need them.

B **Incorrect** (trap: Right action, wrong time). A self-test can raise awareness and may belong in a later plan for growth. It measures associations rather than correcting decisions already affecting students, so reviewing the flagged referrals with an outside check comes first.

C **Correct.** The Code calls for critical self-reflection and self-correction about bias, and for seeking colleagues' counsel when it serves clients (NASW Code 1.05(c); 2.05(a)). Implicit bias operates outside awareness, so a worker reviewing their own decisions alone may repeat it. The worker has found a consistent disparity and flagged recent cases; reviewing them with a supervisor adds an outside check, helps correct affected referrals and builds a plan to prevent recurrence. It beats A because withdrawing referrals on one's own relies on the same judgment that produced the pattern.

D **Incorrect** (trap: Misreads law or ethics). A different standard by race replaces one disparity with another and may harm Black students who need evaluation. The remedy is a consistent, criteria-based process reviewed with others.

Takeaway: When a worker finds bias in their own decisions, an outside review with a supervisor or consultant should guide the correction.

Sources: NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 2.05(a)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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24. ASM-1058 (rev 2) · Best answer: B

A child protective services social worker receives a report that the parent of a 3-year-old is 'on methadone.' With the parent's consent, the opioid treatment program confirms 8 months of steady attendance, negative drug screens and newly approved take-home doses. The child appears healthy at the first home visit. What is the social worker's **BEST** approach?

A. Close the report, since the parent's methadone is lawful care
B. Assess parenting and safe storage of the take-home doses
C. Require tapering the doses before the case can close

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Methadone is lawful, effective treatment, and the report should not be treated as proof of neglect. Closing without any assessment, though, skips the storage of take-home doses around a 3-year-old, the risk that matters here.

B **Correct.** Medication for opioid use disorder (MOUD) is evidence-based treatment (SAMHSA TIP 63), so receiving it is not by itself evidence of maltreatment. The real child-safety questions are the parent's daily functioning and whether the new take-home methadone is stored out of a young child's reach, since accidental ingestion can be fatal. Federal rules require programs to use child-resistant packaging and teach

safe home storage (42 CFR 8.12). It beats A because closing without checking storage skips the one concrete risk the facts present.

C **Incorrect** (trap: Outside the social work role). Medication decisions belong to the parent and the prescriber. Requiring a taper reflects stigma, raises relapse and overdose risk, and runs against evidence that MOUD supports stable parenting.

Takeaway: A parent's MOUD is treatment, not maltreatment; assess actual parenting and safe storage of take-home doses.

Sources: SAMHSA TIP 63 (2021), Part 1: Introduction to Medications for Opioid Use Disorder Treatment; 42 CFR Part 8, 42 CFR 8.12(i)(4)
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.8 Addiction and substance use: signs and impact on systems · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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25. ETH-1185 (rev 3) · Best answer: A

A client who has met with the social worker twice about job stress asks for a letter stating a need for an emotional support dog. The landlord will begin eviction over the unapproved dog in 10 days. Neither session has touched on the dog or the client's housing. What should the social worker do **FIRST**?

A. Assess whether a disability-related need for the animal exists
B. Refer the client to a provider who evaluates support animal requests
C. Write the requested letter so the eviction can be stopped

Why each option is right or wrong

A **Correct.** The NASW Code's Integrity principle requires honest, trustworthy conduct, and the Code bars association with deception (4.04). A disability-related letter must rest on the social worker's own assessment; federal housing guidance treats notes from health professionals with personal knowledge of the person as reliable documentation (HUD, 2020). The eviction deadline makes a prompt assessment important but does not replace it. It beats B because the social worker already knows the client and can assess within the 10 days, and personal knowledge is what makes such documentation reliable.

B **Incorrect** (trap: Hands off too soon). A specialist evaluator could assess the request, which makes this tempting. This social worker already has a relationship with the client and can assess promptly within the 10-day window, so referring out first adds delay and a new relationship without need.

C **Incorrect** (trap: Right action, wrong time). If the assessment supports a disability-related need, a letter is appropriate. Writing it now, without any assessment, would state something the social worker does not know, which conflicts with integrity and the Code's standard on dishonesty.

Takeaway: Document a client's need only after assessing it yourself; urgency speeds the assessment but never replaces it.

Sources: NASW Code of Ethics (2021), Ethical Principles: Integrity; NASW Code of Ethics (2021), 4.04; U.S. Department of Housing and Urban Development (HUD) (2020). FHEO Notice FHEO-2020-01: Assessing a Person's Request to Have an Animal as a Reasonable Accommodation Under the Fair Housing Act, Documentation of disability-related need from health care professionals
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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26. ETH-1034 (rev 1) · Best answer: D

A client who gave birth 6 weeks ago tells the social worker at an outpatient perinatal mental health clinic about sudden, unwanted images of the baby slipping under the water during baths. The client cries, calls the images 'horrifying,' and now asks a partner to give all baths. The social worker finds the client's thinking clear and organized, and the client wants help. What is the **BEST** response?

A. Notify the child abuse hotline about the disclosure
B. Send the client to the emergency department for evaluation
C. Tell the client's partner to oversee every feeding and bath
D. Continue treatment and record the risk assessment

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Child abuse reports require reasonable suspicion of abuse or neglect. Unwanted, feared thoughts with protective avoidance are symptoms, not maltreatment. A report would be warranted if the parent described intent, harmful acts or psychotic thinking about the baby.

B **Incorrect** (trap: Skips assessment). Emergency evaluation is right when there is confusion, psychotic features or intent to harm, as in postpartum psychosis. This client's thinking is clear and the images are unwanted, so emergency care overreacts and may frighten the client away from treatment.

C **Incorrect** (trap: Overrides the client's choice). Directing the partner to supervise all care treats the client as dangerous and discloses without consent. The partner can join in support if the client chooses, but restricting the parent's care of the baby is not indicated.

D **Correct.** Unwanted, distressing harm images that a new parent avoids acting on are typical of perinatal obsessions, which are not linked to a higher risk of harming the infant when psychosis is absent (Hudak & Wisner, 2012). With clear thinking, distress and protective avoidance, there is no reportable maltreatment or imminent danger, so the worker continues treatment and documents the assessment. D beats A because a report needs reasonable suspicion of maltreatment, and reporting feared thoughts would breach confidentiality and discourage disclosure (NASW Code 1.07(c)).

Takeaway: Distinguish unwanted, distressing harm obsessions from intent; obsessions without psychosis call for treatment and documentation, not a report.

Sources: Hudak, R., & Wisner, K. L. (2012). Diagnosis and treatment of postpartum obsessions and compulsions that involve infant harm. American Journal of Psychiatry, 169(4), 360-363, whole article; NASW Code of Ethics (2021), 1.07(c)

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

27. INT-1639 (rev 1) · Best answer: B

A hospital social worker learns that a patient recovering from hip surgery will be discharged home this afternoon. The patient lives alone up a flight of stairs, cannot yet climb them, and home health services cannot start for 4 days. The patient says they are afraid of falling. The social worker shared these facts and the physical therapy evaluation with the attending physician, who says the order stands. What should the social worker do **NEXT**?

A. Ask the physician again to reconsider the discharge time
B. Take the concern up the chain of command today
C. Document the concern and complete the discharge plan

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Direct communication with the physician is the right first step, and it has been taken with the supporting facts. Asking again without new information is unlikely to change the decision before the afternoon discharge.

B **Correct.** When a team decision raises ethical concerns, social workers try to resolve the disagreement through appropriate channels and, if that fails, pursue other avenues consistent with client well-being (NASW Code 2.03(b)). The social worker has already gone directly to the physician with the facts, and the patient faces a same-day discharge that is unsafe as planned. Taking the concern up the hospital's chain, such as the social work supervisor or a physician advisor, is the next step. It beats A because the direct conversation has already happened; repeating it without new information spends the little time left before the afternoon discharge.

C **Incorrect** (trap: Ignores risk). Documenting the concern is necessary, but completing an unsafe plan leaves a patient who cannot manage stairs alone at home for 4 days. Recording a risk does not end the duty to pursue the concern.

Takeaway: When direct discussion fails to resolve a team decision that endangers a patient, pursue the concern through the organization's escalation channels.

Sources: NASW Code of Ethics (2021), 2.03(b)
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.3 Inter- and intradisciplinary teamwork (co-therapy, care conferences, IEPs) · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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28. ETH-1057 (rev 1) · Best answer: D

Under the HIPAA Privacy Rule, which content can be kept in psychotherapy notes, apart from the main record?

A. Session start and stop times
B. The treatment plan and progress summary
C. Current diagnosis and symptoms
D. The worker's analysis of session conversation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Session start and stop times are expressly excluded from psychotherapy notes. They belong in the clinical record, where they support billing and continuity.

B **Incorrect** (trap: Wrong model or concept). Summaries of the treatment plan and progress are excluded from the definition. They must appear in the clinical record so that care can continue if another provider takes over.

C **Incorrect** (trap: Wrong model or concept). A summary of diagnosis and symptoms is excluded from psychotherapy notes. Diagnosis is core clinical record content used for treatment and billing.

D **Correct.** HIPAA defines psychotherapy notes as a mental health professional's notes documenting or analyzing the contents of counseling conversations, kept separate from the rest of the record (45 CFR 164.501). The definition excludes session times, medication monitoring, test results and summaries of diagnosis, symptoms, treatment plan and progress. D beats B because the plan and progress belong in the clinical record that other providers rely on. Psychotherapy notes add protection but never replace that record (NASW Code 3.04(b)).

Takeaway: Psychotherapy notes hold the worker's analysis of session content; times, diagnosis, plan and progress belong in the clinical record.

Sources: HIPAA Privacy Rule, 45 CFR 164.501 (definition: psychotherapy notes); NASW Code of Ethics (2021), 3.04(b)

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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29. INT-0110 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

A client with bipolar disorder was discharged from a psychiatric unit 3 days ago after a suicidal crisis. The client does not arrive for the first appointment at the outpatient clinic and has not called. What should the outpatient social worker do **FIRST**?

A. Phone the client today to check in and reschedule
B. Mail a letter offering a new appointment time
C. Close the referral under the clinic's no-show policy
D. Request a police welfare check at the client's home

Why each option is right or wrong

A **Correct.** The weeks after psychiatric discharge carry sharply raised suicide risk, especially after an admission for suicidal behavior (Chung et al., 2017). Care-transition guidance tells outpatient providers to contact a patient right away after a missed first appointment and to keep pursuing contact (National Action Alliance for Suicide Prevention, 2019). A same-day call lets the social worker reconnect, check on safety and rebook. It beats D because a welfare check is an escalation for imminent risk or for when direct outreach fails, and the clinic has not yet tried to reach the client.

B **Incorrect** (trap: Good but not best). A letter is slow and passive during the highest-risk period after discharge. It can be part of persistent outreach but should not replace prompt, direct contact.

C **Incorrect** (trap: Ignores risk). Applying a routine no-show policy to a client 3 days after a suicidal crisis ignores the danger of this period. Recently discharged clients need active outreach, not case closure.

D **Incorrect** (trap: Skips assessment). A police welfare check is appropriate when risk appears imminent or when the client and the client's safety contacts cannot be reached. As a first step, it escalates before the simplest contact has been tried.

Takeaway: After psychiatric discharge, treat a missed first appointment as a risk signal: reach out directly and promptly, escalating if contact fails.

Sources: National Action Alliance for Suicide Prevention (2019). Best Practices in Care Transitions for Individuals with Suicide Risk: Inpatient Care to Outpatient Care, outpatient provider actions after a missed first appointment; Chung, D. T., Ryan, C. J., Hadzi-Pavlovic, D., Singh, S. P., Stanton, C., & Large, M. M. (2017). Suicide rates after discharge from psychiatric facilities: A systematic review and meta-analysis. JAMA Psychiatry, 74(7), 694-702, whole article

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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30. ETH-1662 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

A client with bipolar I disorder has been stable on medication for 3 years and works full-time. The client tells the community mental health social worker that they and their partner plan to try for a baby next year. At a team meeting, a colleague suggests that the worker urge the client to reconsider. What is the **MOST** appropriate response?

A. Encourage the client to wait for 5 years of stability
B. Share the team's concern that pregnancy may trigger relapse
C. Help the client plan pregnancy care with the prescriber

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). An arbitrary stability target puts the team's comfort in charge of a personal decision. Three stable years and full-time work show the client is well placed to plan; what matters now is good preparation.

B **Incorrect** (trap: Too directive). Relapse risk after birth is real and belongs in the conversation, but as information for the client's planning, not as a case for changing the decision. Framed as a reason to reconsider, it pressures the client and may discourage openness about the pregnancy.

C **Correct.** Decisions about having children belong to the client, and the social worker helps clients pursue their own goals (NASW Code 1.02). Planning ahead is what guidelines favor: medication decisions made with a prescriber before and during pregnancy, because some mood stabilizers carry fetal risks and stopping medication abruptly raises relapse risk (ACOG, 2023). Helping the client prepare for that conversation, and for postpartum support, respects the choice and addresses the real risks. It beats B because presenting the team's worry as a reason to reconsider pressures the client instead of equipping them.

Takeaway: Support a client's reproductive choices; help them plan with the prescriber instead of steering them away because of a diagnosis.

Sources: NASW Code of Ethics (2021), 1.02; American College of Obstetricians and Gynecologists (2023). Clinical Practice Guideline No. 5: Treatment and Management of Mental Health Conditions During Pregnancy and Postpartum, psychiatric medication decisions in pregnancy
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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31. ASM-1140 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

At an area agency on aging, an adult child asks for help placing their 71-year-old parent in memory care. Over the past 5 months, the parent has developed a shuffling, unsteady walk, new bladder accidents and growing forgetfulness. The adult child, the main caregiver, says they are exhausted. The parent has had no checkup since the changes began. What should the social worker do **FIRST**?

A. Help the family compare memory care facilities
B. Set up in-home respite care for the tired caregiver
C. Arrange a prompt visit with the parent's physician
D. Screen the parent's memory with a brief cognitive test

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Placement planning may be appropriate later if a progressive, untreatable condition is confirmed. Choosing memory care now assumes dementia and could move the parent from home for a condition that treatment might improve.

B **Incorrect** (trap: Right action, wrong time). Respite is a valuable support for an exhausted caregiver and can be arranged soon. It comes after setting up the physician visit, which could change the diagnosis and the need for placement.

C **Correct.** Potentially reversible causes must be ruled out before planning around dementia. A shuffling gait, bladder incontinence and cognitive decline developing together over months form the classic pattern of normal pressure hydrocephalus, which can improve with treatment. DSM-5-TR places such conditions under neurocognitive disorder due to another medical condition. It beats B because respite eases real caregiver strain but should not delay finding a treatable cause that could change the whole plan (DSM-5-TR; Kaplan & Sadock's Synopsis).

D **Incorrect** (trap: Good but not best). A cognitive screen measures memory but cannot identify the cause. With gait and bladder changes appearing alongside forgetfulness, a medical evaluation is needed, and a screen alone may simply suggest dementia.

Takeaway: New gait problems, incontinence and forgetfulness together suggest a treatable brain condition; arrange medical evaluation before dementia care planning.

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major or Mild Neurocognitive Disorder Due to Another Medical Condition; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Neurocognitive disorders: normal pressure hydrocephalus
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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32. ASM-1685 (rev 2) · Best answer: B

An emergency department social worker assesses a 38-year-old whose partner brought them in after they said, 'Everyone would be better off without me,' following a job loss. When asked directly, the patient describes passive wishes to be dead but no intent or plan, has no firearms at home, and has never attempted suicide. The partner will stay home this week, and the patient has an established outpatient mental health provider. What is the **BEST** plan?

- **A.** Admit the patient to the inpatient psychiatric unit for observation
- **B.** Discharge home with a written safety plan and a therapy visit within 2 days
- **C.** Keep the patient in the emergency department overnight for reassessment
- **D.** Refer the patient to a partial hospitalization program next week

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Inpatient admission fits intent, a plan, a recent attempt or an inability to stay safe. None is present, so admission would be more restrictive than the risk requires and could disrupt the supports already in place.

B **Correct.** Level of care should be the least restrictive setting that can keep the person safe. The direct assessment shows passive ideation without intent, plan, firearm access or prior attempts, plus a supportive partner and an established outpatient provider. These facts support discharge with a collaborative safety plan, a brief intervention designed for emergency settings, and prompt follow-up (Stanley & Brown, 2012). Without serious, foreseeable and imminent risk, the patient's own choices carry weight (NASW Code 1.02). It beats D because partial hospitalization adds intensity the risk does not require and starts later than the existing provider can.

C **Incorrect** (trap: Good but not best). An overnight hold for reassessment adds little when the direct assessment is complete and protective factors are clear. Extended observation suits intoxication or an unclear history, not this picture.

D **Incorrect** (trap: Good but not best). Partial hospitalization suits people who need structured daily treatment but not 24-hour care. Here risk is lower, an outpatient provider is already in place, and waiting a week delays follow-up.

Takeaway: Match level of care to assessed risk: with no intent or plan and solid supports, outpatient follow-up with a safety plan is least restrictive.

Sources: Stanley, B., & Brown, G. K. (2012). Safety planning intervention: A brief intervention to mitigate suicide risk. Cognitive and Behavioral Practice, 19(2), 256-264, whole article; NASW Code of Ethics (2021), 1.02
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.2 Goal setting, treatment planning, triage, and service plans · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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33. INT-1602 (rev 1) · Best answer: A

An integrated primary care clinic hires 5 new associate social workers at once. All of them must learn the clinic's brief behavioral health protocol, and the one licensed supervisor has limited hours. Each associate already has a weekly individual meeting for case-specific risk questions. Which added format is **MOST** appropriate for teaching the protocol?

A. Group supervision for the associates as a cohort
B. A peer consultation group the associates run themselves
C. Live supervision of each associate's patient sessions

Why each option is right or wrong

A **Correct.** Group supervision is efficient when several supervisees share a learning need, and it adds peer learning and mutual support while a qualified supervisor guides the content (Kadushin & Harkness, 2014). All 5 associates must learn the same protocol, the supervisor's hours are limited, and individual meetings already cover case-specific risk. It beats C because live observation of every associate would use far more supervisor time to teach the same material; live supervision fits better for refining one person's skills once the protocol has been learned.

B **Incorrect** (trap: Good but not best). Peer consultation has no member with supervisory authority or expertise in the protocol. Five newcomers teaching each other a model none of them knows risks spreading errors. The Code points workers to colleagues with demonstrated expertise (NASW Code 2.05(b)); peer groups suit experienced practitioners.

C **Incorrect** (trap: Good but not best). Live supervision gives in-the-moment feedback and is valuable for skill refinement. Doing it for each of 5 associates would consume more of the supervisor's limited hours than one shared group session covering the same protocol.

Takeaway: When several supervisees share one learning need and supervisor time is scarce, supervisor-led group supervision teaches efficiently and adds peer learning.

Sources: Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 9: Group Supervision; NASW Code of Ethics (2021), 2.05(b)
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.2 Supervision and consultation models: individual, peer, group · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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34. INT-1076 (rev 2) · Best answer: A

Two hours after an apartment fire, a parent sits in a hospital emergency department while nurses treat the parent's two children for minor injuries. The parent keeps repeating, 'I don't know what to do,' stares at the floor, and cannot answer when asked whom to call. What is the **BEST** approach for the social worker?

A. Give brief, concrete directions for the immediate next steps
B. Reflect the parent's feelings and wait for the parent to choose
C. Complete a full psychosocial history for the record

Why each option is right or wrong

A **Correct.** In crisis work, the worker's level of directiveness matches the client's level of functioning: nondirective for a mobile client, collaborative for a partly mobile one, and directive for one who is immobilized (James & Gilliland, 2017). This parent cannot answer a simple question and keeps repeating that they don't know what to do, which are signs of immobility. Brief, concrete steps restore a sense of order. It beats B because reflection respects autonomy but leaves an immobilized person stuck; choices can return as functioning improves.

B **Incorrect** (trap: Misuses the technique). Reflection and waiting fit a client who can think through choices. A parent who cannot say whom to call is immobilized right now, and waiting prolongs the disorganization. As the parent steadies, the worker can shift to a more collaborative stance.

C **Incorrect** (trap: Right action, wrong time). A full history matters later, but it asks a disorganized parent to answer many questions at the moment they can answer none. Stabilization and practical next steps come first; detailed history can follow once functioning returns.

Takeaway: Match crisis directiveness to functioning: the more immobilized the client, the more directive and concrete the worker, stepping back as coping returns.

Sources: James, R. K., & Gilliland, B. E. (2017). Crisis Intervention Strategies (8th ed.). Cengage, assessing client mobility; directive, collaborative and nondirective action continuum
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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35. ETH-1566 (rev 2) · Best answer: C

The clinical director of a psychiatric hospital learns that a unit supervisor has begun a romantic relationship with a social worker the supervisor directly supervises. Both say the relationship is consensual and does not affect their work. What should the director do **FIRST**?

A. Ask both to keep the relationship separate from work
B. Refer the supervisor for a human resources conduct review
C. Move the supervisee to a different supervisor right away
D. Wait unless a complaint or performance problem arises

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Keeping the relationship out of daily work leaves the supervisor evaluating a romantic partner. The conflict comes from the supervisory authority itself, which this request leaves in place.

B **Incorrect** (trap: Right action, wrong time). A conduct review under agency policy is a likely later step. Starting with it leaves the supervisee under a romantic partner's evaluation while the process runs, so the conflict continues.

C **Correct.** Supervisors should not have sexual relationships with people over whom they hold professional authority (NASW Code 2.06(a)), and a social worker in a relationship with a colleague has a duty to transfer professional responsibilities to avoid a conflict of interest (2.06(b)). The supervision standards say romantic involvement with a supervisee should be strictly avoided because it creates marked role conflict

(Supervision Standards, Standard 4: Boundaries). Consent does not remove the power imbalance. The first step is to end the supervisory line by reassigning supervision. It beats B because a conduct review may follow, but it takes time, and the supervisee should not stay under the partner's evaluation meanwhile.

D **Incorrect** (trap: Misreads law or ethics). Waiting for a problem misreads the rule: the relationship itself creates the conflict, whatever the pair report about their work. Acting only after harm leaves the supervisee, and fairness to other staff, at risk (NASW Code 3.01(c)).

Takeaway: A romantic relationship between supervisor and supervisee requires ending the supervisory line at once; consent does not cure the power conflict.

Sources: NASW Code of Ethics (2021), 2.06(a); NASW Code of Ethics (2021), 2.06(b); NASW Code of Ethics (2021), 3.01(c); NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 4 (Ethical Issues): Boundaries

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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36. INT-1096 (rev 2) · Best answer: B

At a syringe services program, a participant shows the social worker a swollen, hot area on the forearm where the redness has spread since yesterday. The participant feels feverish and explains clearly that they will not go to the emergency department after being mistreated there. The program partners with a walk-in wound clinic two blocks away that serves people who use drugs. What is the **BEST** action for the social worker?

A. Give wound care supplies and check the arm next week
B. Go with the participant to get wound care today
C. Call an ambulance despite the participant's objection
D. Explore the participant's readiness to stop injecting

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Supplies help with minor wounds, but redness spreading over a day with fever signals a worsening infection. Waiting a week risks serious illness. Same-day medical care is needed, with supplies as an add-on.

B **Correct.** Harm reduction keeps people engaged without judgment and links them to care on terms they can accept (SAMHSA, 2023). Spreading redness, heat and fever suggest a worsening infection that needs medical treatment now, and people who inject drugs often delay care after stigma or mistreatment in hospitals (Gilbert et al., 2019). Going with the participant to a nearby, trusted clinic today meets the urgency while honoring the refusal of the emergency department. It beats C because an ambulance overrides a capable adult's choice when an acceptable same-day option exists.

C **Incorrect** (trap: Overrides the client's choice). Emergency transport over objection fits someone who is unresponsive or unable to decide. This participant is making a clear choice about one setting, not refusing all care, and a same-day alternative exists that the participant may accept.

D **Incorrect** (trap: Good but not best). Talking about change has its place, but it does not address an infection that is getting worse today. Raising treatment for drug use now may also feel like a condition of help and push the participant away.

Takeaway: Harm reduction links people to urgent medical care on terms they accept; worsening infection signs need same-day treatment, not routine follow-up.

Sources: SAMHSA (2023) Harm Reduction Framework, pillars and principles; linkage to care; Gilbert, A. R., Hellman, J. L., Wilkes, M. S., Rees, V. W., & Summers, P. J. (2019). Self-care habits among people who inject drugs with skin and soft tissue infections: A qualitative analysis. Harm Reduction Journal, 16, 69, whole article
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.12 Harm-reduction approaches · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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37. ETH-1628 (rev 2) · Best answer: A

At a refugee resettlement agency, a client who arrived 8 months ago is behind on the electric bill. While going over the household budget, the client mentions sending about a third of each paycheck to parents and siblings in a refugee camp, then stops and watches the social worker's reaction. What should the social worker do **FIRST**?

A. Ask the client about the money sent home and what it means
B. Advise pausing the transfers until the electric bill is current
C. Build a budget that treats the transfers as a fixed expense

Why each option is right or wrong

A **Correct.** Many immigrants keep close financial and emotional ties with family abroad, and money sent home is often an obligation rather than a spending choice (Falicov, 2007). The client has just raised the transfers and paused, watching for a reaction, and the worker does not yet know what the money means to the client or the family. Asking openly treats the client as the expert on their own culture (NASW Code 1.05(c)) and shapes the budget that follows. It beats C because treating the transfers as fixed may well be right, but deciding it before the client explains assumes the meaning instead of learning it.

B **Incorrect** (trap: Too directive). Pausing the transfers might clear the bill fastest, but it imposes the worker's priorities before the worker even knows what the money does for the family. Whether to change the amount is the client's decision to make after weighing options (NASW Code 1.02).

C **Incorrect** (trap: Right action, wrong time). Building the budget around the transfers is often the right plan when they are a family obligation, and it may follow. Setting it up before the client explains what the money means skips the client's own account, which should shape the plan.

Takeaway: Ask what money sent to family abroad means before budgeting; then plan around it rather than asking the client to cut it.

Sources: Falicov, C. J. (2007). Working with transnational immigrants: Expanding meanings of family, community, and culture. Family Process, 46(2), 157-171, whole article; NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 1.02
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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38. INT-1680 (rev 1) · Best answer: A

To meet a new funder requirement, a community mental health program added a 45-minute questionnaire battery to every intake. Over the next quarter, missed second appointments doubled. Exit calls with clients who left point to the long first visit spent on paperwork. The funder requires data on 3 outcomes. Which action is **BEST**?

A. Trim the battery to brief versions of the funder's measures
B. Pause the funder's battery until attendance recovers
C. Survey clients who dropped out about the intake visit

Why each option is right or wrong

A **Correct.** Evaluation should support services, and measurement burden that drives clients away harms both care and the data. The funder needs 3 outcomes, yet the battery takes 45 minutes and exit calls tie dropout to it. Brief measures that cover the required outcomes keep accountability while restoring a client-centered first visit. It beats B because stopping measurement breaks the funding agreement and leaves the program unable to see whether clients improve; the fix is proportion, not abandonment (Royse, Thyer & Padgett, 2016; NASW Code 5.02(a)).

B **Incorrect** (trap: Good but not best). Pausing measurement removes the burden but breaks the funder requirement and leaves the program without outcome data. Suspension makes sense only if no brief, valid option exists; here 3 outcomes can be measured quickly.

C **Incorrect** (trap: Right action, wrong time). Asking dropouts about their experience is right when the cause of dropout is unknown. Exit calls have already linked it to the long paperwork, so another survey adds burden and delays the fix.

Takeaway: When evaluation demands harm service delivery, streamline to measures that answer the required questions rather than dropping measurement.

Sources: Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, Ch. 12: Selecting the Best Evaluation Measure for Your Project; NASW Code of Ethics (2021), 5.02(a)
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.3 Program evaluation and its effect on services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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39. ASM-1082 (rev 1) · Best answer: C

In a youth residential program, staff find fresh, shallow cuts on a 15-year-old resident's thigh. The resident tells the social worker, 'It's not like that. It just takes the pressure off when I get overwhelmed.' The unit manager wants to start one-to-one suicide watch and remove the resident's privileges for a week. What should the social worker do **FIRST**?

A. Start one-to-one observation until the psychiatrist visits
B. Teach replacement coping skills for overwhelming feelings
C. Assess the self-injury's function and any wish to die

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Close observation is right if assessment shows suicidal intent. Started before any assessment, and paired with lost privileges, it can feel punitive, push self-injury into hiding and still leave intent unknown.

B **Incorrect** (trap: Right action, wrong time). Skills such as distress tolerance are a core treatment for self-injury. They come after the social worker knows whether the cutting relieves emotion or carries intent to die, which changes the immediate plan.

C **Correct.** Nonsuicidal self-injury is self-harm without intent to die, often used to relieve intense emotion, while any intent to die makes an act suicidal (Nock, 2010; DSM-5-TR). The resident's words suggest emotional relief, but only a direct assessment of function, intent and history can tell the two apart and set the response. It beats A because one-to-one watch fits if the assessment finds suicidal intent; imposing it with punishment first can shame the youth and does not replace the assessment.

Takeaway: Self-injury needs assessment of its function and of suicidal intent before deciding on restrictions or treatment.

Sources: Nock, M. K. (2010). Self-injury. Annual Review of Clinical Psychology, 6, 339-363, Definitions; functions of self-injury; DSM-5-TR (APA, 2022), Conditions for Further Study: Nonsuicidal Self-Injury Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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40. ETH-1702 (rev 2) · Best answer: D

At a school team meeting, a teacher proposes testing a 12-year-old refugee student for intellectual disability because the student reads at an early-elementary level in English. Records show about 3 years of schooling in a refugee camp, taught in Arabic. The student solves math puzzles quickly and teaches classmates complex games. What is the **BEST** recommendation for the school social worker to make?

A. Proceed with the school's standard intelligence tests in English
B. Delay any evaluation until the student is fluent in English
C. Place the student in a self-contained class while testing proceeds
D. Seek native-language testing that accounts for lost instruction

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Standard English-language intelligence tests would measure exposure to English and to schooling more than ability. Results could mislabel a student whose gaps reflect lost instruction, which the rules do not allow as a basis for eligibility.

B **Incorrect** (trap: Good but not best). Waiting for English fluency can take years and may leave a real disability unaddressed. The better path is an evaluation in the student's language that accounts for interrupted schooling, alongside strong language instruction.

C **Incorrect** (trap: Skips assessment). Moving the student to a self-contained class before any evaluation assumes a disability and removes the student from peers and grade-level instruction. Placement decisions follow evaluation, not the other way around.

D **Correct.** Federal special education rules require assessments given in the child's native language and bar a disability finding when the determining factor is lack of appropriate instruction or limited English proficiency (34 CFR 300.304(c)(1); 300.306(b)). This student had about 3 years of interrupted camp schooling in Arabic, and quick math reasoning and complex play suggest strong learning ability. An

evaluation in the student's language that weighs this history guards against mislabeling while still checking for a real disability. It beats B because language status alone is no reason to postpone, and postponing can leave a true need unmet.

Takeaway: For refugee students with interrupted schooling, seek evaluations in their language that weigh lost instruction before labeling a disability.

Sources: Individuals with Disabilities Education Act regulations, 34 CFR 300.304(c)(1)(i)-(ii); Individuals with Disabilities Education Act regulations, 34 CFR 300.306(b)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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41. ETH-1525 (rev 1) · Best answer: C

In an integrated primary care clinic, a client who left home last month after a partner pushed them down the stairs says they have decided to move back in. The couple has no children. The client asks the social worker for help 'being smart about it.' What is the **BEST** response?

A. Urge a shelter stay until the partner gets help
B. Explore the client's mixed feelings about going back
C. Develop a safety plan with the client for living at home
D. Contact the partner to set conditions for the return

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Urging a shelter stay substitutes the social worker's judgment for the client's. Shelter is a resource to offer, but pressing it after the client has decided can damage trust and make the client less likely to return when danger rises.

B **Incorrect** (trap: Good but not best). Exploring mixed feelings fits a client who is still weighing whether to go back. This client has made a decision and asked for help carrying it out safely, so more exploration delays the safety work the client requested.

C **Correct.** A competent adult may choose to return to an abusive partner, and the social worker respects that choice while reducing risk (NASW Code 1.02). The client has decided and is asking for practical help, so the most useful step is a safety plan built on the client's own knowledge of the partner: warning signs, an exit route, a packed bag and trusted contacts (Davies, Lyon & Monti-Catania, 1998). It beats B because exploring ambivalence suits a client who is still deciding; this client has decided and asked for something specific.

D **Incorrect** (trap: Ignores risk). Contacting the partner could alert an abusive person to the client's plans and raise the danger. It also shares client information without consent. Any contact with the partner is the client's decision, made with a safety plan in place.

Takeaway: When a competent adult returns to an abusive partner, respect the choice and build a safety plan from the client's own knowledge of the risks.

Sources: NASW Code of Ethics (2021), 1.02; Davies, J., Lyon, E., & Monti-Catania, D. (1998). Safety Planning with Battered Women: Complex Lives/Difficult Choices. Sage, woman-defined advocacy and safety planning
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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42. ASM-1625 (rev 1) · Best answer: D

During a home visit, a parent describes their 13-month-old, who began walking last week. The toddler roams happily from room to room, shrugs off small tumbles and often seems to forget the parent is nearby. Every few minutes the child toddles back, pats the parent's knee and heads off again. Which of Mahler's subphases is this child **MOST** likely in?

A. Differentiation
B. Rapprochement
C. Object constancy
D. Practicing

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Differentiation, or 'hatching' (about 5 to 10 months), is when the infant first turns attention to the world beyond the parent, scanning faces and exploring the parent's body. It comes before independent walking, so it does not fit a toddler roaming the house.

B **Incorrect** (trap: Wrong model or concept). Rapprochement (about 16 to 24 months) also involves coming back to the parent, but the returns are driven by a new awareness of being separate. The toddler shadows and clings, then darts away, often with distress. This child shows elation and little concern, not that ambivalence.

C **Incorrect** (trap: Wrong model or concept). Object constancy develops from about 24 to 36 months, when the child holds a stable inner image of the parent and can tolerate longer separations. A 13-month-old who needs frequent physical check-ins has not reached it.

D **Correct.** In Mahler's theory, the practicing subphase (about 10 to 16 months) begins as the child crawls and then walks. The child explores with elation, seems almost unaware of the parent, and returns only briefly for emotional 'refueling' before setting off again. The 13-month-old's happy roaming, indifference to tumbles and quick touch-and-go returns fit this picture. It beats B because rapprochement, at about 16 to 24 months, brings a new awareness of separateness, with anxious clinging, shadowing and conflict rather than carefree exploring (Mahler, Pine & Bergman, 1975).

Takeaway: Mahler's practicing subphase features joyful exploration with brief check-ins; rapprochement adds awareness of separateness, clinging and ambivalence.

Sources: Mahler, M. S., Pine, F., & Bergman, A. (1975). The Psychological Birth of the Human Infant: Symbiosis and Individuation. Basic Books, separation-individuation subphases: practicing
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

43. ETH-1545 (rev 2) · Best answer: B

A nursing home resident in their 80s recently chaired the residents' council and manages their own finances. The resident has begun a romantic relationship with another resident. The resident's adult children call the social worker, upset, ask what staff know about the relationship, and demand that staff keep the two apart. What should the social worker do **FIRST**?

A. Assure the children that staff will limit the residents' time alone
B. Ask the resident how they want staff to respond to the children
C. Explain to the children that the relationship is the resident's choice

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Limiting a capable resident's time with a partner because relatives object overrides the resident's self-determination and privacy rights. Family concern does not give relatives authority over a competent adult's relationships.

B **Correct.** Nursing home residents have rights to self-determination and personal privacy, including in their visits and relationships (42 CFR 483.10(f), (h)). The resident shows clear capacity, and the relationship is the resident's private matter, so the first step is to learn the resident's wishes: what may be shared with the children and how to handle their concerns (NASW Code 1.02). It beats C because the children may need that explanation, but discussing the resident's private life with family before checking with the resident bypasses the resident's control over their own information (HIPAA, 45 CFR 164.510(b)).

C **Incorrect** (trap: Right action, wrong time). Explaining the resident's rights may help the children accept the relationship, and it could follow. Doing it before asking the resident means discussing the resident's private life, which the children are asking about, without the resident's agreement, and it takes the matter out of the resident's hands.

Takeaway: A capable older adult controls their own relationships and information; ask the resident before responding to family objections.

Sources: 42 CFR 483.10 (Resident rights), (f) self-determination; (h) privacy; HIPAA Privacy Rule, 45 CFR 164.510(b); NASW Code of Ethics (2021), 1.02
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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44. ASM-1155 (rev 1) · Best answer: A

Six weeks after a heart attack, a patient in a hospital cardiac rehabilitation program has missed several exercise sessions. The patient tells the social worker they feel down most days, have lost interest in hobbies, and sleep poorly. The patient adds, 'I don't need a shrink. I just need my strength back.' A nurse calls this a normal adjustment. What is the **MOST** appropriate action?

A. Screen for depression as a routine part of rehab care
B. Focus on exercise goals as the patient prefers
C. Arrange a psychiatric consult to consider antidepressants
D. Reassure the patient that low mood after a heart attack passes

Why each option is right or wrong

A **Correct.** Depression after a heart attack is common and is linked to higher rates of later cardiac events and death (Lichtman et al., 2014). Weeks of low mood, lost interest and poor sleep call for a standardized screen such as the PHQ-9 (Kroenke et al., 2001). Presenting it as routine rehab care eases the stigma the patient voices. It beats B because following the patient's focus is usually sound, but here untreated depression may be what is undermining the exercise goals.

B **Incorrect** (trap: Good but not best). Starting where the client is matters, and exercise goals belong in the plan. Setting the mood concern aside, though, ignores a condition that predicts poorer heart outcomes and may explain the missed sessions.

C **Incorrect** (trap: Skips assessment). Medication may eventually help, but referral comes after screening shows the size of the problem. A psychiatric referral now also runs into the stigma the patient has just expressed.

D **Incorrect** (trap: Good but not best). Some low mood after a heart attack does fade. Weeks of lost interest and poor sleep go beyond that, and reassurance without screening risks missing depression that affects survival.

Takeaway: Depression after a heart attack predicts worse cardiac outcomes; screen persistent symptoms rather than accept them as normal adjustment.

Sources: Lichtman, J. H., Froelicher, E. S., Blumenthal, J. A., et al. (2014). Depression as a risk factor for poor prognosis among patients with acute coronary syndrome: Systematic review and recommendations. A scientific statement from the American Heart Association. Circulation, 129(12), 1350-1369, whole article; Kroenke, K., Spitzer, R. L., & Williams, J. B. W. (2001). The PHQ-9: Validity of a brief depression severity measure. Journal of General Internal Medicine, 16(9), 606-613, whole article

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.6 Comorbid (co-occurring) disorders and conditions · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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45. INT-1585 (rev 1) · Best answer: C

A youth mentoring program runs at four sites, and its funder asks for an outcome study within the year. Site reports show that two sites hold weekly mentor meetings as designed. The other two meet about once a month and have dropped mentor training. The program director asks the social worker to plan the evaluation. What should the social worker do **FIRST**?

A. Start measuring youth outcomes at every site now
B. Compare outcomes between high- and low-fidelity sites
C. Examine and correct how each site delivers the model
D. Ask the funder to drop the outcome requirement

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Measuring outcomes is the right step once delivery is consistent. Starting now would blend sites that deliver the model with sites that do not, so poor results could not be traced to the model or to its delivery.

B **Incorrect** (trap: Misuses the technique). Comparing sites seems to use the variation, but the sites differ in staff, youth and neighborhoods as well as fidelity. The comparison was not designed and cannot isolate the effect of delivering the model well.

C **Correct.** A process or formative evaluation checks whether a program is delivered as designed, and an outcome study of a poorly implemented program cannot tell implementation failure from a weak model (Rossi, Lipsey & Henry, 2019). Two sites have dropped core parts of the model, so documenting delivery and correcting the gaps comes first, with the outcome study to follow within the year. This beats B because a site comparison confounds fidelity with many other differences between sites.

D **Incorrect** (trap: Good but not best). Funder requirements can sometimes be renegotiated, but an outcome study is appropriate once delivery is consistent. Asking to drop it gives up accountability instead of putting the steps in order.

Takeaway: Confirm a program is delivered as designed before judging its outcomes; otherwise poor results cannot be traced to the model or the delivery.

Sources: Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 4: Assessing Program Process and Implementation

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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46. ETH-1117 (rev 2) · Best answer: C

A social worker has counseled adult survivors of sexual assault for 8 years. Over time, the worker has come to assume most strangers have harmful motives, has stopped letting their teenager ride public transit, and tells a colleague that the world now feels unsafe in a way it never did. The worker says these views developed gradually. Which concept **MOST** likely explains these changes?

A. Secondary traumatic stress
B. Burnout
C. Vicarious trauma
D. Countertransference

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Secondary traumatic stress involves symptoms resembling PTSD, such as intrusive images or avoidance, after hearing clients' trauma. The changes here are gradual shifts in core beliefs, which fit a different concept.

B **Incorrect** (trap: Wrong model or concept). Burnout involves exhaustion, cynicism and a reduced sense of accomplishment driven by workplace conditions (Maslach & Leiter, 2016). It does not center on altered beliefs about personal safety.

C **Correct.** Vicarious trauma describes cumulative changes in a helper's beliefs about safety, trust and control that develop through empathic engagement with clients' trauma over time (McCann & Pearlman, 1990; NCTSN, 2011). This worker's changes are in worldview: distrust of strangers, new limits on a child's freedom and a sense that the world is unsafe, and they built up gradually over years. C beats A because secondary traumatic stress centers on PTSD-like symptoms, such as intrusive images, avoidance and arousal; the stem describes gradual changes in beliefs instead.

D **Incorrect** (trap: Wrong model or concept). Countertransference is a reaction to a particular client shaped by the worker's own history. These changes are general shifts in worldview, not responses to one client.

Takeaway: Vicarious trauma means gradual change in a helper's core beliefs about safety and trust; secondary traumatic stress means PTSD-like symptoms.

Sources: McCann, I. L., & Pearlman, L. A. (1990). Vicarious traumatization: A framework for understanding the psychological effects of working with victims. Journal of Traumatic Stress, 3(1), 131-149, whole article; National Child Traumatic Stress Network, Secondary Traumatic Stress Committee (2011). Secondary Traumatic Stress: A Fact Sheet for Child-Serving Professionals, Sidebar: Secondary Traumatic Stress and Related Conditions; Maslach, C., & Leiter, M. P. (2016). Understanding the burnout experience: recent research and its implications for psychiatry. World Psychiatry, 15(2), 103-111, burnout dimensions

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.6 Burnout, compassion fatigue, and secondary traumatic stress · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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47. ETH-1036 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

In outpatient treatment for depression, an adult client says they left their 5-year-old alone overnight twice last month while working night shifts. The social worker determines that a report is mandated by law. The client has no history of violence, and the child is staying with a grandparent this week. What is the **BEST** way to proceed?

A. File the report without telling the client, to avoid conflict
B. Tell the client about the report, then make it
C. Give the client a week to arrange child care first
D. Make the report, then transfer the client to a colleague

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Reporting without notice is appropriate when warning the client could put the child, the worker or the investigation at risk. None of those risks appears here, so silence needlessly damages trust and departs from the Code's preference for advance notice.

B **Correct.** When feasible, social workers tell clients before disclosing confidential information, including disclosures the law requires, and explain the likely consequences (NASW Code 1.07(d)); clients should know when legal duties override other loyalties (NASW Code 1.01). Telling this client is safe: there is no history of violence and the child is with a grandparent. B beats A because a silent report is reserved for cases where notice could endanger the child or the worker, and advance notice keeps the alliance intact.

C **Incorrect** (trap: Misreads law or ethics). The reporting duty arises once the threshold is met and is not suspended while a parent fixes the problem. Delaying a week substitutes the worker's judgment for the law's requirement.

D **Incorrect** (trap: Hands off too soon). A report does not end the helping relationship. Transferring the client right after reporting can feel like punishment, and continued services are often how the worker helps the family through the process.

Takeaway: When it is safe, tell the client before making a mandated report, and keep working with the client afterward.

Sources: NASW Code of Ethics (2021), 1.07(d); NASW Code of Ethics (2021), 1.01
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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48. ASM-1132 (rev 2) · Best answer: A

PRETEST — unscored on this form; do not count it

Police bring a 23-year-old to the emergency department after finding them directing traffic and announcing a plan to buy the city's stadium. A roommate says the client has barely slept for 3 days, has talked nonstop and has spent their savings online. The client is admitted to the psychiatric unit that night. Which type of mood episode is **MOST** likely?

A. Manic episode
B. Hypomanic episode
C. Major depressive episode with mixed features

Why each option is right or wrong

A **Correct.** A manic episode normally lasts at least 1 week, but DSM-5-TR waives the duration when symptoms are severe enough to require hospitalization. Hypomania needs at least 4 days and, by definition, is never severe enough to need admission. After 3 days of little sleep, nonstop talk, reckless spending and grandiose plans, this client needed admission, so the episode is manic. It beats B because the short duration tempts a hypomania label, yet the need for hospitalization rules hypomania out (DSM-5-TR).

B **Incorrect** (trap: Near-miss diagnosis). Hypomania requires at least 4 consecutive days and must not cause marked impairment or need hospitalization. Three days of symptoms ending in psychiatric admission fails both conditions.

C **Incorrect** (trap: Near-miss diagnosis). Mixed features describe a full major depressive episode with some manic symptoms alongside. This client shows elevated, grandiose mood and increased activity, with no depressive episode described.

Takeaway: Hospitalization makes an elevated-mood episode manic regardless of duration; hypomania needs 4 days and never requires admission.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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49. INT-1093 (rev 1) · Best answer: D

A community outreach client uses cocaine at weekend parties and says they have no plans to stop. After two friends overdosed on cocaine that contained fentanyl, the client asks the social worker, 'How do I keep this from happening to me?' What is the **BEST** response?

A. Advise the client that quitting is the safest choice
B. Explore what the client might gain from quitting cocaine
C. Urge the client to enroll in an intensive treatment program
D. Go over practical ways to make each use less risky

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Quitting would remove the risk, but the client has said they will keep using. Advice to stop answers a question the client did not ask and may end the conversation before any safety steps are shared.

B **Incorrect** (trap: Good but not best). Exploring reasons for change fits a client who is weighing it. This client asked a direct safety question, and pivoting to quitting can feel like a lecture; it leaves the immediate overdose risk unaddressed.

C **Incorrect** (trap: Hands off too soon). Intensive treatment fits people who want structured help with a substance use disorder. The client has not asked for treatment, and pushing it ignores the stated goal and may cost the relationship.

D **Correct.** Harm reduction meets people where they are and works to reduce the harms of use without requiring abstinence (Hawk et al., 2017). The client has a clear safety question and has said they will keep using. Discussing concrete steps, such as having naloxone on hand and someone nearby who can respond, answers that question and builds trust that keeps the door open to later change (SAMHSA, 2023). It beats B because exploring change is useful, but turning to quitting after the client asked how to stay alive sidesteps the request.

Takeaway: When a client who plans to keep using asks how to stay safer, answer with concrete harm-reduction steps rather than redirecting to abstinence.

Sources: Hawk, M., et al. (2017). Harm reduction principles for healthcare settings. Harm Reduction Journal, 14, 70, Table 1: harm reduction principles (pragmatism, autonomy); SAMHSA (2023) Harm Reduction Framework, pillars and principles; overdose prevention education

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.12 Harm-reduction approaches · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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50. ETH-1049 (rev 2) · Best answer: B

PRETEST — unscored on this form; do not count it

During an internal records audit at a community agency, a social worker discovers that notes for two counseling sessions held last month were never written. The social worker has detailed appointment logs and remembers both sessions clearly. What is the **BEST** way to handle the missing notes?

A. Write the notes now and date them on the session days

B. Write them now, dated today and labeled as late entries

C. Leave the gap rather than change the record afterward

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Dating the notes as if written on the session days misrepresents when they were made. Backdating is falsification even when the content is true, and it can destroy the record's credibility in an audit or court.

B **Correct.** Records must be accurate and timely enough to support continuity of care (NASW Code 3.04(a), 3.04(b)). When documentation was missed, the accepted fix is a late entry: written now, dated with the actual date of writing, and clearly labeled as late, so the record is complete without misrepresenting when it was made. B beats C because leaving the gap protects nothing; two sessions of care go undocumented, which undermines continuity and the record's accuracy.

C **Incorrect** (trap: Good but not best). Avoiding changes is sound for completed notes, which are corrected only by dated addenda. Missing notes are different: the services happened, and leaving them undocumented makes the record inaccurate and incomplete.

Takeaway: Document missed services as clearly labeled late entries dated when written; never backdate, and never leave delivered services unrecorded.

Sources: NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 3.04(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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51. ASM-1556 (rev 1) · Best answer: B

What is the status of internet gaming disorder in the DSM-5-TR?

A. An official diagnosis among the addictive disorders
B. A condition for further study in Section III
C. A specifier used with gambling disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Only gambling disorder appears as a non-substance diagnosis in the substance-related and addictive disorders chapter. Internet gaming disorder was proposed but placed in Section III pending more research.

B **Correct.** DSM-5-TR places internet gaming disorder in Section III, Conditions for Further Study, meaning the evidence is not yet strong enough for official clinical use (DSM-5-TR). Gambling disorder remains the only behavioral addiction among the official diagnoses. Social workers can still assess how gaming affects sleep, school, work and relationships, and diagnose co-occurring conditions that meet criteria. It beats A because many people assume gaming joined gambling as an official diagnosis, and it has not.

C **Incorrect** (trap: Near-miss diagnosis). Gambling disorder has course and severity specifiers, none of which concern gaming. Gaming problems are described separately, as a proposed condition in Section III rather than a feature of gambling disorder.

Takeaway: Internet gaming disorder is a DSM-5-TR condition for further study, not an official diagnosis; assess gaming's effects and co-occurring disorders.

Sources: DSM-5-TR (APA, 2022), Section III, Conditions for Further Study: Internet Gaming Disorder; DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Gambling Disorder
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.21 Technology and social media effects on clients · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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52. ETH-1505 (rev 2) · Best answer: A

A social worker is providing family therapy for a 19-year-old client. The client's 27-year-old sibling attended 3 of the sessions. Between sessions, the sibling messages the social worker to ask for a dinner date, adding, 'I'm technically outside the treatment.' What is the **MOST** ethical response?

A. Decline the invitation and keep the professional role
B. Accept, since the sibling is outside the client role
C. Accept after the family's treatment has ended
D. Decline and transfer the family to a colleague

Why each option is right or wrong

A **Correct.** Social workers avoid romantic or sexual involvement with clients' relatives or others close to them when there is a risk of exploitation or harm, and the worker, not the other person, sets the boundary (NASW Code 1.09(b)). The sibling took part in sessions, so the worker holds information and influence that could harm the client and family. A clear decline that keeps the professional role protects the treatment. It beats D because the invitation can be handled without disrupting the family's care; transfer is needed only if the boundary cannot be held.

B **Incorrect** (trap: Misreads law or ethics). Standing outside the identified-client role does not remove the risk. The sibling took part in therapy, and a relationship would affect the client and the family's treatment. The Code extends this caution to clients' relatives and close others.

C **Incorrect** (trap: Boundary slip or collusion). Waiting until treatment ends does not remove the risk, because the worker would still hold what was learned in family sessions. The family might also need to return for services, which the relationship would make difficult (Reamer, 2020).

D **Incorrect** (trap: Hands off too soon). Declining is right, but transfer is an overreaction here. The family's work would be disrupted over a boundary the worker can simply hold. Transfer would fit if the worker's objectivity were compromised or the sibling kept pressing.

Takeaway: Avoid romantic involvement with clients' relatives or close others; the social worker, not the other person, holds the boundary.

Sources: NASW Code of Ethics (2021), 1.09(b); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, intimate relationships with clients' relatives
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

53. INT-1643 (rev 1) · Best answer: A

A social worker recommends that a client with schizophrenia join an intensive outpatient program after a recent rise in hearing voices. The client declines because the program hours conflict with a valued job. After going over the likely benefits and dangers, the social worker judges that the client understands them. Which entry **BEST** documents this decision?

A. The recommendation, risks discussed and the client's reasons
B. That the client was noncompliant with the treatment plan
C. A signed statement releasing the agency from responsibility
D. That the client declined the referral

Why each option is right or wrong

A **Correct.** Documenting an informed refusal means recording what was recommended, the risks and benefits discussed, the client's understanding, and the client's own reasons. Such a record shows that the client's self-determination was respected (NASW Code 1.02) and gives sufficient documentation for continuity of care (3.04(b)). It beats D because a bare note that the client declined leaves out what the next worker, or a reviewer, needs to understand the decision and revisit it if symptoms worsen.

B **Incorrect** (trap: Misuses the technique). 'Noncompliant' is a judgmental label that misdescribes a capable client's informed choice. It can bias future providers and says nothing about the reasons or the risks discussed.

C **Incorrect** (trap: Misreads law or ethics). A release form does not replace clear documentation or remove professional duties. Asking for one can feel punitive and damage trust with a client making a legitimate choice.

D **Incorrect** (trap: Good but not best). This is accurate but too thin. Without the recommendation, the risks discussed and the client's reasons, later readers cannot tell whether the refusal was informed or know what to watch for.

Takeaway: Document informed refusal fully: the recommendation, the risks and benefits discussed, the client's understanding and the client's own reasons for declining.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 3.04(b)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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54. ASM-1136 (rev 2) · Best answer: B

A mobile crisis team brings an adult to a crisis stabilization unit. The adult is agitated, irritable and tearful, with rapid speech, and cannot give a history. No records or family are available. The symptoms clearly involve mood, but it is unclear whether they belong to a bipolar or a depressive disorder. Which DSM-5-TR diagnosis fits this situation?

A. Unspecified bipolar and related disorder
B. Unspecified mood disorder
C. Unspecified mental disorder
D. Unspecified depressive disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Unspecified bipolar and related disorder fits when symptoms are clearly bipolar-type but criteria or information are incomplete. Here even the bipolar-versus-depressive question cannot be answered.

B **Correct.** DSM-5-TR restored unspecified mood disorder for presentations with clear mood symptoms that do not meet full criteria for any bipolar or depressive disorder, when choosing between unspecified bipolar and unspecified depressive disorder is difficult. Acute agitation in a crisis or emergency setting with no available history is the classic example. It beats C because unspecified mental disorder is for symptoms that cannot be tied to any diagnostic class, and these clearly involve mood. A and D each commit to one chapter, which the limited information cannot support. The diagnosis can be revised as history becomes available (DSM-5-TR).

C **Incorrect** (trap: Near-miss diagnosis). Unspecified mental disorder fits when symptoms cause distress or impairment but cannot be placed in any diagnostic class, sometimes because an emergency setting allows too little information. Here the symptoms clearly involve mood, so the narrower mood category applies.

D **Incorrect** (trap: Near-miss diagnosis). Unspecified depressive disorder fits when the symptoms are clearly depressive but the clinician cannot or does not specify more. Rapid speech and agitation leave open a bipolar picture, so this label commits too early.

Takeaway: When mood symptoms are clear but bipolar versus depressive cannot be determined, DSM-5-TR offers unspecified mood disorder.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Unspecified Mood Disorder; DSM-5-TR (APA, 2022), Other Mental Disorders and Additional Codes: Unspecified Mental Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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55. ETH-1020 (rev 1) · Best answer: C

A supervisor at a family service agency asks a social worker to video-record sessions with an adult client for use in supervision. The client has not yet heard about the request. Before the next session, what is the **MOST** appropriate way for the social worker to proceed?

A. Tell the client the agency requires recording for supervision
B. Start recording and tell the client they may ask to stop
C. Explain the recording's purpose and use and ask for consent

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Presenting recording as an agency requirement pressures the client and suggests they have no choice. Recording for supervision depends on the client's informed agreement, and services should continue if the client declines.

B **Incorrect** (trap: Misuses the technique). Letting the client stop once recording has started treats silence as agreement. Consent must be informed and given before recording begins, after the client understands its purpose and use.

C **Correct.** Social workers obtain clients' informed consent before audio or video recording, or before observation by a third party (NASW Code 1.03(h)). Valid consent means explaining the purpose, who will see the recording, how it will be stored and destroyed, and that the client may decline without losing services (NASW Code 1.03(a)). C beats B because an opt-out after recording starts is not informed consent; the client must agree before any recording begins.

Takeaway: Get informed consent before recording a session, explaining purpose, viewers, storage and the right to decline.

Sources: NASW Code of Ethics (2021), 1.03(h); NASW Code of Ethics (2021), 1.03(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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56. ASM-1084 (rev 2) · Best answer: A

Police bring an adult to the emergency department after a neighbor heard the person shouting that they would 'end it tonight.' The patient smells of alcohol, slurs words and walks unsteadily. Now the patient laughs it off, says, 'I was just drunk,' and asks to leave. What should the social worker do **FIRST**?

A. Observe until sober, then do a full assessment
B. Complete the full suicide risk assessment right now
C. Accept the retraction and discharge with a crisis line

Why each option is right or wrong

A **Correct.** Intoxication impairs judgment and makes both suicidal statements and retractions unreliable. The conservative approach is to keep the patient safe and observed until sober enough to take part in a full risk assessment, and not to let the patient leave before it is done (Betz & Boudreaux, 2016). A public threat to 'end it tonight' is not cancelled by a drunken retraction. It beats B because answers given while the patient slurs and staggers cannot be trusted and may falsely reassure; the full assessment belongs after sobering.

B **Incorrect** (trap: Right action, wrong time). A thorough assessment is the goal, and basic safety questions can be asked now. A full assessment while the patient is visibly intoxicated, though, yields unreliable answers. It should be completed once the patient can take part meaningfully.

C **Incorrect** (trap: Ignores risk). A retraction may be sincere, but one made while intoxicated, hours after a threat heard by a neighbor, cannot be relied on. Discharge before a sober assessment risks releasing someone at real risk.

Takeaway: Suicidal statements or retractions made while intoxicated are unreliable: keep the person safe and complete the assessment once sober.

Sources: Betz, M. E., & Boudreaux, E. D. (2016). Managing suicidal patients in the emergency department. Annals of Emergency Medicine, 67(2), 276-282, Intoxicated patients
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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57. ASM-1047 (rev 2) · Best answer: B

A client entered residential treatment for severe alcohol use disorder 6 days ago after years of heavy daily drinking. The client reports low mood, poor sleep and guilt, and says the low mood began after the drinking became heavy 3 years ago. The client answers no to direct questions about suicide. The staff psychiatrist asks for the social worker's diagnostic input. What is the social worker's **BEST** recommendation?

A. Recommend diagnosing major depressive disorder now
B. Reassess the mood after about a month of abstinence
C. Treat the low mood as alcohol-related and stop assessing it
D. Suggest starting an antidepressant during detox

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Major depressive disorder may prove correct, but the low mood began with heavy drinking and is being judged in the first week of abstinence. DSM-5-TR asks clinicians to consider a substance-induced cause first.

B **Correct.** Depressive symptoms are common during heavy drinking and early abstinence and often lift within weeks. DSM-5-TR points to an independent depressive disorder when depression began before the substance use or persists for a substantial period, about a month, after acute withdrawal or intoxication ends. This client's low mood began with heavy drinking and is being assessed on day 6, so reassessing after about a month of abstinence, while monitoring mood and safety, gives the clearest answer. It beats A because diagnosing major depression now cannot separate alcohol's effects from an independent disorder.

C **Incorrect** (trap: Ignores risk). Alcohol may well explain the mood, but closing the question risks missing an independent depression that persists after abstinence. Co-occurring disorders are common and need continued assessment.

D **Incorrect** (trap: Skips assessment). Medication decisions belong to the prescriber, and starting an antidepressant during early withdrawal assumes an independent depression that the timeline does not yet support. It may fit if low mood persists.

Takeaway: Depression that starts with heavy substance use is reassessed after about a month of abstinence before diagnosing an independent depressive disorder.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Substance/Medication-Induced Depressive Disorder
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.6 Comorbid (co-occurring) disorders and conditions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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58. INT-1134 (rev 1) · Best answer: A

A 15-year-old in a school-based DBT program uses emotion regulation skills well in sessions. Diary cards show that the teen's worst outbursts follow times when a parent says things like 'You're overreacting, just get over it.' The parent attends sessions and wants to help. What is the **MOST** appropriate next step?

A. Coach the parent in validating the teen's feelings
B. Add home distress tolerance practice the parent can prompt
C. Refer the teen to a residential DBT program

Why each option is right or wrong

A **Correct.** DBT's biosocial theory holds that emotional sensitivity is intensified by an invalidating environment (Linehan, 1993), so adolescent DBT teaches caregivers validation alongside the teen (Rathus & Miller, 2015). The diary cards tie the worst outbursts to the parent's dismissive comments, and the parent is willing to help. Coaching the parent to acknowledge what the teen feels changes the trigger itself. It beats B because more skill practice asks the teen to cope with invalidation that can be reduced at its source.

B **Incorrect** (trap: Good but not best). Extra practice is useful, and the teen already uses skills well in session. It leaves the main trigger, the parent's dismissive comments, in place, so outbursts at home are likely to continue.

C **Incorrect** (trap: Hands off too soon). Residential care is for youth who cannot be kept safe or treated in outpatient care. Nothing here suggests that level of need, and the problem has a clear trigger at home that outpatient work can address.

Takeaway: When invalidation at home drives a teen's dysregulation, teach caregivers validation rather than only adding skills for the teen.

Sources: Linehan, M. M. (1993). Cognitive-Behavioral Treatment of Borderline Personality Disorder. Guilford Press, biosocial theory: invalidating environment; Rathus, J. H., & Miller, A. L. (2015). DBT Skills Manual for Adolescents. Guilford Press, Walking the Middle Path: validation

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.6 Teaching emotion-regulation skills · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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59. ETH-0008 (rev 1) · Best answer: A

A social worker provides weekly video sessions to an adult client with depression. At the start of a session, the client mentions that they are calling from a vacation rental in another state where the social worker is not licensed. What should the social worker do **FIRST**?

A. Determine whether practice is permitted in that state

B. Continue the session because the client is already established

C. Identify emergency resources in the client's current state

D. End the session and terminate services with the client

Why each option is right or wrong

A **Correct.** Telehealth is generally considered to take place where the client is located. The social worker must follow the laws of that jurisdiction as well as their own (Technology Standards 2.02; NASW Code 1.04(e)). Before going on, the social worker determines whether practice there is allowed, for example through a temporary permit or compact privilege, and then decides how to proceed. It beats C because emergency planning for the client's location matters only if the session can lawfully continue, so it follows once that is settled.

B **Incorrect** (trap: Misreads law or ethics). Being an established client does not change where the service takes place. Continuing without checking risks practicing without authorization in the client's current jurisdiction.

C **Incorrect** (trap: Right action, wrong time). Knowing emergency resources near a remote client is required (Technology Standards 2.13). It comes after the social worker confirms that serving the client in that state is permitted.

D **Incorrect** (trap: Hands off too soon). Ending services is premature, because the social worker does not yet know that state's rules. Even if this session must stop, the social worker would plan for continuity rather than terminate an ongoing client.

Takeaway: Telehealth happens where the client is: confirm you may practice in the client's location before continuing.

Sources: NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.02; NASW Code of Ethics (2021), 1.04(e)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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60. INT-1601 (rev 1) · Best answer: D

A nursing home's charge nurse asks a social work consultant for help with a resident who refuses meals. The consultant learns the nurse has managed similar residents well, but this resident reminds the nurse of a parent, and the nurse now pleads and argues at mealtimes. The consultant works on the nurse's reactions to this resident. Which type of consultation is this **MOST** likely?

A. Client-centered case consultation
B. Program-centered administrative consultation
C. Consultee-centered administrative consultation
D. Consultee-centered case consultation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Client-centered case consultation centers on the client: the consultant assesses the resident and recommends a care plan, and the consultee's own functioning is not the focus. It would fit if the nurse simply lacked information about meal refusal.

B **Incorrect** (trap: Wrong model or concept). Program-centered administrative consultation helps plan or run a program, such as designing a facility-wide nutrition protocol. This request concerns one resident and one staff member's handling of that resident.

C **Incorrect** (trap: Wrong model or concept). Consultee-centered administrative consultation addresses an administrator's difficulties with organizational or program tasks. The nurse's difficulty here involves the care of one resident, not a program or management task.

D **Correct.** Caplan (1970) classified consultation by its focus. In consultee-centered case consultation, the consultant addresses the consultee's difficulty with a particular case, such as a loss of objectivity, rather than the client's problem itself. The nurse has the needed skill, but a personal resemblance is distorting the work, and the consultant targets that reaction. It beats A because client-centered case consultation would center on assessing the resident and advising a plan for the resident, which is not what this consultant is doing.

Takeaway: When a consultant works on the consultee's difficulty with a case, such as lost objectivity, it is consultee-centered case consultation.

Sources: Caplan, G. (1970). The Theory and Practice of Mental Health Consultation. Basic Books, Types of mental health consultation

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.2 Supervision and consultation models: individual, peer, group · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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61. INT-1088 (rev 1) · Best answer: C

In a hospice program, a patient is expected to die within days. The patient's partner asks the social worker how to tell their 5-year-old and plans to say the patient is 'going to sleep for a long time' because it sounds gentle. What is the **BEST** guidance?

A. Use the gentle phrase, since young children cannot grasp death

B. Wait to explain the death until the child starts asking

C. Use the words 'dying' and 'died,' explained in simple terms

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Children around age 5 are beginning to understand that death is permanent, universal and has a cause. The 'sleep' phrase can lead to fear of bedtime or hope that the parent will wake up. A gentle tone with accurate words works better.

B **Incorrect** (trap: Good but not best). Many young children do not ask directly even when they sense a crisis, and silence can increase anxiety. Prepared, honest explanation before and after the death supports grieving; later questions can be answered as they come.

C **Correct.** Young children take words literally, so euphemisms such as sleep can confuse them or make them afraid to go to sleep; using 'dead' or 'died' is recommended (Schonfeld et al., 2024). Pairing those words with a simple explanation, such as the body stopping working and not being able to come back, helps a 5-year-old begin to grasp that death is permanent. It beats B because waiting leaves the child sensing that something is wrong without explanation; children do better with honest, early, age-suited information.

Takeaway: Talk with young children about death using concrete words such as 'died'; euphemisms like sleep can confuse or frighten them.

Sources: Schonfeld, D. J., Demaria, T., Nasir, A., Kumar, S., & AAP Committee on Psychosocial Aspects of Child and Family Health, Council on Children and Disasters (2024). Supporting the grieving child and family. Pediatrics, 154(1), e2024067212, communicating with children about death: avoid euphemisms

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.10 Working with loss, separation, and grief · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

Section 2

62. INT-1032 (rev 1) · Best answer: B

At a community health center, a client asks the social worker for help 'being more assertive' with their partner. The client says the partner checks their phone, gets 'scary' when they disagree, and last month broke the client's phone during an argument. The client wants to practice speaking up at home. What should the social worker do **FIRST**?

A. Role-play a calm I-statement for the next disagreement
B. Assess the danger posed by the partner and plan for safety
C. Suggest joint sessions so both partners can practice

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Role-playing I-statements is a standard assertiveness step and may come later. Done first, it prepares the client to confront a partner who already becomes intimidating, before anyone knows how dangerous that could be.

B **Correct.** When a client describes possible partner abuse, safety comes before skill-building (SAMHSA TIP 25). Monitoring, intimidation when the client disagrees, and breaking the client's phone are warning signs. Speaking up to a controlling partner can raise the risk of retaliation, and better communication cannot make the partner stop. Assessing danger and planning for safety lets the client decide how, and whether, to assert themselves. It beats A because assertiveness practice may still help, but only after the risk is understood and the plan accounts for it.

C **Incorrect** (trap: Ignores risk). Joint sessions can put a person at risk when there are signs of abuse, because what they say in session may be punished at home (SAMHSA TIP 25). Individual safety assessment comes first.

Takeaway: Before teaching assertiveness with a partner, screen for abuse and plan for safety; speaking up to an abusive partner can raise the danger.

Sources: SAMHSA TIP 25 (1997), Ch. 2: Survivors of Domestic Violence: An Overview (safety; cautions on couples therapy)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.25 Teaching assertiveness skills · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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63. ASM-1501 (rev 1) · Best answer: A

During a home visit, the parent of a 20-month-old tells an early childhood social worker that the toddler used to say 'ball,' 'milk' and 'up' but stopped using any words about 3 months ago. The toddler does not point to show objects and seldom turns when called by name. Relatives say the child will grow out of it. What should the social worker do **FIRST**?

A. Refer the child for a prompt developmental evaluation
B. Plan to recheck the child's language at the next visit
C. Ask the parent how the family feels about the changes
D. Coach the parent in daily language-building play

Why each option is right or wrong

A **Correct.** Loss of words or social skills at any age is a developmental red flag that calls for prompt evaluation rather than watchful waiting (Centers for Disease Control and Prevention, Developmental Milestones). This toddler has lost all previously used words and also shows missing pointing and weak response to name, early signs linked to autism spectrum disorder (Hyman et al., 2020). It beats C because the family's feelings matter but do not change the need for a timely referral; they can be explored while the referral moves forward. Early evaluation opens the door to early intervention, which works best when started young.

B **Incorrect** (trap: Ignores risk). Waiting to recheck delays evaluation for a child who has already lost skills. Rescreening at a later visit can suit a borderline screening result with no regression; it is wrong once words have been lost.

C **Incorrect** (trap: Good but not best). Understanding the family's reactions supports engagement and can happen alongside the referral. Making it the first step, when the toddler has lost words and shows other red flags, puts a feelings conversation ahead of a time-sensitive evaluation.

D **Incorrect** (trap: Right action, wrong time). Language-building play is useful and may become part of an early intervention plan. Starting it in place of an evaluation treats the problem as simple delay before the cause of the lost words is known.

Takeaway: Loss of previously gained language or social skills at any age is a red flag that warrants prompt developmental evaluation, not waiting.

Sources: Centers for Disease Control and Prevention. Learn the Signs. Act Early.: CDC's Developmental Milestones, when to act early: lost skills; Hyman, S. L., Levy, S. E., Myers, S. M., & AAP Council on Children with Disabilities (2020). Identification, evaluation, and management of children with autism spectrum disorder. Pediatrics, 145(1), e20193447, early signs and referral for evaluation

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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64. ETH-0009 (rev 1) · Best answer: C

An agency begins requiring government-issued photo identification at intake. Over the next month, a social worker notices that several unhoused clients and clients who recently immigrated leave without being served. What should the social worker do **FIRST**?

A. Waive the requirement for affected clients without asking approval

B. Refer clients without identification to another agency

C. Document who is turned away and raise it with agency leaders

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Quietly ignoring the policy may feel just, but it bypasses the agency's channels and leaves the barrier in place for everyone else. Social workers first try to change harmful policies through appropriate processes.

B **Incorrect** (trap: Hands off too soon). Referring people elsewhere accepts the barrier and pushes its cost onto the clients it excludes. It treats a structural problem as an individual one and leaves the policy unexamined.

C **Correct.** Anti-oppressive practice looks past individual cases to policies that block access for marginalized groups, and it holds institutions accountable (NASW Code

1.05(c)). The first step is to gather and document evidence of the policy's effect, then use agency channels to seek change (NASW Code 3.09(b)). It beats A because breaking the rule on one's own helps a few clients while leaving the policy in place, and it exposes clients and the social worker to risk. Documented cases make a stronger case to leadership than anecdotes.

Takeaway: When a policy blocks access for marginalized groups, document its impact and advocate for change through organizational channels.

Sources: NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 3.09(b); NASW Code of Ethics (2021), 6.04(b)

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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65. ASM-1191 (rev 1) · Best answer: A

An adult client with bipolar I disorder has kept a daily mood chart for a year. Reviewing it with the social worker, the client sees that the chart rated their mood as 'normal' during the 2 weeks before last spring's manic episode, while the client's partner noticed clear changes then. The client wants to use the chart alone to decide when to call the prescriber. What is the **BEST** response?

A. Plan with the client to add the partner's observations

B. Support the client's plan to rely on the mood chart

C. Have the partner report mood changes to the prescriber directly

D. Switch the client to a more detailed daily mood scale

Why each option is right or wrong

A **Correct.** Learning to spot early relapse signs and act quickly helps prevent mania (Perry et al., 1999). Insight often fades as mania begins, though, so family members can help recognize early warning signs (Miklowitz, 2019). The chart missed the last episode while the partner saw it, so planning with the client to add the partner's observations keeps the client in charge while covering the blind spot. It beats D because a more detailed scale still depends on the same self-ratings that failed.

B **Incorrect** (trap: Ignores risk). Self-monitoring is valuable and the client's wish matters, but the chart has already missed the run-up to a manic episode. Relying on it alone ignores evidence that it misses the signs that matter most.

C **Incorrect** (trap: Overrides the client's choice). Having the partner go straight to the prescriber cuts the client out of their own monitoring and may breach trust in the relationship. The partner's observations help most when the client agrees to them and helps decide how they are used.

D **Incorrect** (trap: Misuses the technique). A finer scale feels like a fix, but the problem is reduced awareness during early mania, not the scale. More detailed self-ratings would likely miss the same changes.

Takeaway: When self-monitoring misses early relapse signs, keep the client in charge but add a trusted observer's input rather than rely on self-report alone.

Sources: Perry, A., Tarrier, N., Morriss, R., McCarthy, E., & Limb, K. (1999). Randomised controlled trial of efficacy of teaching patients with bipolar disorder to identify early symptoms of relapse and obtain treatment. BMJ, 318, 149-153, whole article; Miklowitz, D. J. (2019). The Bipolar Disorder Survival Guide (3rd ed.). Guilford Press, recognizing early warning signs; family involvement
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.23 Client capacity, self-determination, self-monitoring (consent age, emancipation, permanency) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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66. INT-1512 (rev 1) · Best answer: C

Last week, a client was released from prison after 5 years and enrolled in a reentry program. The social worker plans to open the first meeting with a strengths-based assessment. Which question is **BEST** to ask?

A. What led to the offense that sent you to prison?
B. What barriers do you expect when you look for work?
C. What helped you get through your time inside?

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Understanding the offense may matter for risk planning, but opening with it frames the client by their worst act. A strengths-based assessment starts with what the client brings.

B **Incorrect** (trap: Good but not best). Expected barriers are real and will be addressed in planning. Starting with them makes the first conversation about problems rather than resources.

C **Correct.** A strengths-based assessment starts by asking about the capacities, resources, and survival skills a person already has, on the view that struggle can also build strength (Saleebey, 2013). Asking how the client coped with incarceration invites skills, relationships, and achievements, such as classes or work assignments, that can anchor reentry goals. It beats B because barriers matter for planning, but leading with them frames the client by deficits; they are explored after strengths are on the table.

Takeaway: A strengths-based assessment opens by asking how the client has coped and what resources they have, before turning to problems.

Sources: Saleebey, D. (2013). The Strengths Perspective in Social Work Practice (6th ed.). Pearson, principles of the strengths perspective
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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67. ETH-1510 (rev 1) · Best answer: D

At a community mental health center, a client tells the social worker that their prescription copay went up and they are $40 short this month. The client asks to borrow the money until payday, promising to repay it in 2 weeks. The center has an emergency assistance fund. What is the **BEST** response?

A. Lend the money with a written repayment date
B. Give the client $40 as a one-time gift
C. Suggest the client ask relatives for emergency help
D. Help the client apply to the emergency fund

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). A written repayment date makes the loan look businesslike, but it turns the worker into the client's creditor. If repayment fails, the debt can strain or end the helping relationship.

B **Incorrect** (trap: Boundary slip or collusion). A gift seems kind, but money from the worker blurs the role and can create dependence or a sense of obligation. It also sets a precedent the worker cannot sustain with this client or others.

C **Incorrect** (trap: Good but not best). Asking family can be part of problem solving, but it may be impossible and leaves the medication need unmet. The center's own fund is a more reliable resource the worker can help the client use today.

D **Correct.** Personal financial ties with clients create a conflict of interest and a dual relationship that can leave the client feeling indebted to the worker (NASW Code 1.06(a)). The client's need is real, and the center's emergency fund meets it through the agency rather than the worker's own money. Helping the client apply keeps the relationship professional and solves the problem now. It beats C because relatives may be unable or unwilling to help, while the fund is a resource the social worker can help the client reach today (Reamer, 2020).

Takeaway: Meet clients' financial needs through agency and community resources, not through personal loans or gifts from the worker.

Sources: NASW Code of Ethics (2021), 1.06(a); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, altruistic gestures
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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68. ASM-1610 (rev 1) · Best answer: C

A 28-year-old client with mild intellectual developmental disorder lives in a group home and feels anxious about riding the bus to a new job. The client reads at about a second-grade level, learns best by practicing tasks with staff and asks for help 'feeling less scared.' Which treatment approach is **MOST** appropriate?

A. Standard CBT using the agency's written workbook
B. Insight-oriented therapy exploring the fear's origins
C. CBT adapted with pictures, repetition and a support person

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). The model fits anxiety, but a written workbook assumes reading skills the client does not have. Using it unchanged is likely to frustrate the client and leave the coping skills unpracticed.

B **Incorrect** (trap: Wrong model or concept). Insight-oriented work relies on abstract reflection and extended verbal exploration, a poor fit for this client's learning style. The client's goal is practical, riding the bus with less fear, which calls for concrete, rehearsed skills.

C **Correct.** Modality is matched to the client's cognitive ability. Reviews of therapy for people with intellectual disabilities describe common adaptations: simpler language, repetition, visual materials and involvement of carers (Poku et al., 2026). This client's low reading level and preference for practicing with staff point to a pictured, rehearsal-based approach with a support person who can coach between sessions. It beats A because a text-heavy workbook assumes reading skills the client lacks, so the right model would fail in delivery.

Takeaway: Choose and adapt modalities to cognitive ability: concrete, visual, repeated and supported formats make evidence-based methods usable.

Sources: Poku, A. C., Gray, K. M., Hewitt, O., & Langdon, P. E. (2026). Adapting psychological therapies for individuals with intellectual disabilities: A systematic review. Clinical Psychology & Psychotherapy, 33(1), e70202, whole article

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.3 Choosing intervention modality by ability, development, culture, life stage · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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69. INT-1179 (rev 2) · Best answer: A

PRETEST — unscored on this form; do not count it

A client has attended an opioid treatment program for 8 months, taking methadone daily and holding a steady job. Recent urine tests show benzodiazepines that were not prescribed. The client says the pills help with sleep and fears losing methadone. At a team meeting, a staff member proposes discharge under the program's 'no illicit use' rule. What is the **BEST** response by the social worker?

A. Urge keeping the client enrolled while the team addresses the pill use

B. Support discharge with a referral to an abstinence-based residential program

C. Tell the client that staying enrolled requires benzodiazepine-free tests

Why each option is right or wrong

A **Correct.** Harm reduction keeps people connected to protective care while risks are addressed (SAMHSA Harm Reduction Framework). In 2017 the FDA advised that methadone and buprenorphine should not be withheld from patients who use benzodiazepines, because untreated opioid addiction can outweigh the risks of the combination, and discharge is unlikely to stop the use (FDA, 2017). This client is stable and engaged, so the team can assess the sleep problem, involve the prescriber and add overdose education. It beats C because an abstinence ultimatum is likely to end in the same discharge if use continues.

B **Incorrect** (trap: Hands off too soon). Residential care may suit some clients, but discharge here ends a treatment that is working and leaves an opioid-dependent person without methadone. The FDA has cautioned that excluding patients for benzodiazepine use is unlikely to stop it.

C **Incorrect** (trap: Ignores risk). Clear expectations matter, but an ultimatum tied to test results usually ends in discharge if use continues. It repeats the harm of B on a delay and skips assessing why the client takes the pills for sleep.

Takeaway: Ongoing use of another substance is a reason to adjust care, not to withdraw medication for opioid use disorder.

Sources: FDA (2017). Drug Safety Communication: FDA urges caution about withholding opioid addiction medications from patients taking benzodiazepines or CNS depressants, September 20, 2017; SAMHSA (2023) Harm Reduction Framework, Pillar 2: embraces the inherent value of people; Pillar 5: offers most accessible and noncoercive support

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.12 Harm-reduction approaches · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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70. ETH-1091 (rev 1) · Best answer: C

A parent working toward reunification tells the child welfare social worker about drinking heavily one night last weekend while the children were at the foster home. The parent has met every other case plan goal and asks the worker to leave the drinking out of next week's court report. What is the **MOST** appropriate response?

A. Omit it, since the children were elsewhere

B. Include it in the report without telling the parent

C. Tell the parent the report will include it, with the progress

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). The children's absence lowers the immediate risk but does not make the drinking irrelevant to a case plan the court is monitoring. Leaving it out would mislead the court.

B **Incorrect** (trap: Good but not best). Accurate reporting is right, but clients should be told when the worker's obligations override their wishes (1.01). Surprising the parent in court harms trust and engagement.

C **Correct.** Loyalty to clients is real but limited: when legal obligations supersede it, clients should be told (NASW Code 1.01). A court report must be honest, and leaving out a relapse the court would consider material is deception (4.04). Telling the parent first, and presenting the lapse alongside the strong progress, keeps the report accurate and preserves trust. C beats B because including the information without warning blindsides the parent and damages the working relationship reunification depends on.

Takeaway: Report material facts honestly to the court, and tell the client beforehand when obligations override their wishes.

Sources: NASW Code of Ethics (2021), 1.01; NASW Code of Ethics (2021), 4.04

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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71. ETH-1704 (rev 2) · Best answer: A

At a family services agency, three bilingual case managers who completed the agency's interpreter training are often pulled from their own caseloads to interpret for colleagues. They carry full caseloads with no added pay, and one was marked down for late notes. They say they want to keep interpreting for families they know but need the work recognized. The social worker chairs the equity committee. What is the **BEST** action?

A. Propose crediting interpreting time in workload and pay
B. Ask the supervisor to revise the late-notes rating
C. Arrange contract interpreters so bilingual staff stop interpreting
D. Encourage the case managers to file a grievance themselves

Why each option is right or wrong

A **Correct.** Social workers act to prevent and eliminate discrimination in an employing organization's work assignments (NASW Code 3.09(e)) and challenge social injustice, including discrimination (NASW Code, Ethical Principles: Social Justice). Here bilingual staff carry unpaid extra duties and are penalized for the delays those duties cause, an inequity built into the workload system. Because the case managers want to keep interpreting, crediting that time in caseload size and pay corrects the inequity while keeping families' access to trusted interpreters. It beats C because contract interpreters would end the burden only by overriding the staff's stated wish.

B **Incorrect** (trap: Good but not best). The rating is unfair and should be revisited, but changing one evaluation fixes a symptom. The workload system that keeps adding unpaid interpreting would produce the same result again.

C **Incorrect** (trap: Good but not best). Contract interpreters are a sound choice when staff do not want the role or lack interpreter training. These case managers are trained and want to keep interpreting, so removing the duty sidesteps their preference rather than correcting the pay and workload gap.

D **Incorrect** (trap: Hands off too soon). Staff can file grievances, but the social worker chairs the committee charged with equity issues. Sending the problem back to the affected staff shifts the burden onto them when the worker has a direct channel to propose a fix.

Takeaway: When some staff carry unpaid extra duties because of their language or identity, advocate to recognize that work in workload and pay.

Sources: NASW Code of Ethics (2021), 3.09(e); NASW Code of Ethics (2021), Ethical Principles: Social Justice
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

72. INT-1112 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

In a senior housing building, a resident tells the social worker, 'My kids keep pushing me to move to assisted living. I've lived in this apartment for 40 years.' Which response **BEST** shows accurate reflective listening?

A. 'Your children are worried and just want you to be safe.'
B. 'Why don't you want to consider assisted living?'
C. 'Leaving this home would mean losing a big part of your life.'
D. 'Many people your age feel that same way about moving.'

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). The children's concern may be real, but restating it moves the focus from the resident to the family and can sound like the worker agrees with them. Their worries can be discussed later, once the resident feels heard.

B **Incorrect** (trap: Misuses the technique). A 'why' question asks the resident to defend a position and tends to raise defensiveness. It gathers reasons but shows no understanding of what the apartment means. A reflection invites the same information more safely.

C **Correct.** A reflection captures the feeling and meaning beneath the client's words and offers it back for the client to confirm or correct (Miller & Rollnick, 2023). The resident's mention of 40 years in the apartment signals attachment and loss, not just a housing question. Reflecting that meaning shows the worker understood what the move would cost and invites the resident to say more. It beats A because voicing the children's worries shifts attention away from the resident's experience and can sound like taking the family's side.

D **Incorrect** (trap: Good but not best). Normalizing can reduce shame, but offered first it brushes past this resident's particular attachment to the home and can feel dismissive. It works better after the resident's own feelings have been reflected and understood.

Takeaway: Reflective listening returns the feeling and meaning behind the client's words; avoid 'why' questions, taking sides, or early normalizing.

Sources: Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Engaging: reflective listening **Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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73. ASM-0102 (rev 1) · Best answer: A

In an emergency department, a social worker is asked to see a patient with a broken wrist. The patient says the injury came from a fall, but the physician notes bruises of different ages on both upper arms. The patient's partner stays in the room and answers most questions. What should the social worker do **FIRST**?

A. Arrange to speak with the patient alone
B. Ask about safety at home with the partner present
C. Give the patient domestic violence hotline information

Why each option is right or wrong

A **Correct.** Possible intimate partner violence should be explored in private, because asking in front of a suspected abuser can put the patient at risk and makes honest disclosure unlikely (WHO, 2013). The inconsistent history, bruises of different ages and a partner who answers for the patient are warning signs. It beats C because handing over hotline materials before a private conversation skips assessment, and the partner may see them, which can raise danger. Once alone, the social worker can ask directly and assess safety.

B **Incorrect** (trap: Ignores risk). Asking about violence while the partner listens can expose the patient to retaliation and almost guarantees a guarded answer. Direct questions about safety are right, but only once the patient can talk privately.

C **Incorrect** (trap: Right action, wrong time). Resource information belongs after a private conversation and safety assessment. Given now, it skips the assessment and could be noticed by the partner, which may increase risk (WHO, 2013).

Takeaway: When abuse indicators appear and a possible abuser is present, first create a private space before asking about violence or offering resources.

Sources: World Health Organization (2013). Responding to Intimate Partner Violence and Sexual Violence Against Women: WHO Clinical and Policy Guidelines, Recommendations 1 and 3: first-line support and clinical enquiry
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.4 Physical, sexual, psychological abuse and neglect: signs, dynamics, impact · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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74. ASM-1593 (rev 2) · Best answer: B

At a senior center, a 71-year-old client reports forgetting appointments over the past 2 months. The social worker gives a brief cognitive screen, and the score falls in the impaired range. The client mentions starting a new bladder medication about 10 weeks ago and has not seen a doctor since. What should the social worker do **NEXT**?

A. Tell the client the score points to early dementia
B. Arrange a prompt medical evaluation of the memory change
C. Repeat the screen in 6 months to track any change
D. Give the client a second cognitive screen to confirm

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). A screening score does not diagnose dementia, and diagnosing a medical condition is outside the social work role. Labeling the problem could alarm the client when a treatable cause may exist.

B **Correct.** A brief cognitive screen such as the MoCA flags possible impairment but does not diagnose (Nasreddine et al., 2005). The memory problems began around when the client started a bladder medication, and some bladder drugs have anticholinergic effects that can cause confusion in older adults (American Geriatrics Society Beers Criteria, 2023). A medical evaluation can review medications and look for reversible causes. It beats D because a second screen only repeats the finding; the next need is a medical workup.

C **Incorrect** (trap: Ignores risk). Waiting 6 months leaves a possibly reversible, medication-related cause untreated while the client keeps missing appointments. Monitoring fits after a medical evaluation, not instead of one.

D **Incorrect** (trap: Misuses the technique). A second screening tool adds little once one screen is positive and does not explain why. The finding needs evaluation by a medical provider, who can examine medications and other causes.

Takeaway: A positive cognitive screen calls for medical evaluation, including a medication review, not a diagnosis or a wait-and-see plan.

Sources: Nasreddine, Z. S., et al. (2005). The Montreal Cognitive Assessment, MoCA: A brief screening tool for mild cognitive impairment. Journal of the American Geriatrics Society, 53(4), 695-699, whole article; American Geriatrics Society Beers Criteria Update Expert Panel (2023). American Geriatrics Society 2023 updated AGS Beers Criteria for potentially inappropriate medication use in older adults. Journal of the American Geriatrics Society, 71(7), 2052-2081, strongly anticholinergic medications

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.6 Tools for assessing strengths, resources, needs, challenges · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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75. INT-1116 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

During home-based services, a client has made comments about the social worker's body. At the last visit, the social worker clearly stated that such comments must stop. Today the client repeats them and briefly stands between the social worker and the door. What should the social worker do **FIRST**?

A. Restate the limit firmly and continue with the visit

B. End the visit and review safety with the supervisor

C. Explore what the comments mean to the client

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Restating the limit was the right step the first time. The comments have continued and now include blocking the door, so another warning while staying in the home does not address the risk.

B **Correct.** Worker safety and a clear response to repeated harassment come first. The limit was already stated and the behavior has escalated to blocking the exit, so the social worker ends the visit and brings the incident to the supervisor for reporting and a plan, such as office-based or paired visits (NASW Guidelines for Social Worker Safety, 2013). Ending this visit is not ending services, and the plan protects the client's care as well (NASW Code 1.17(b)). It beats A because repeating a limit that has already failed, while the client controls the exit, leaves the worker at risk.

C **Incorrect** (trap: Right action, wrong time). Understanding what drives the comments may matter clinically, but exploring now, in the home with the exit blocked, puts the worker at risk and treats harassment as ordinary material. It can be addressed later in a safer setting.

Takeaway: When a stated limit fails and behavior escalates on a home visit, leave and involve the supervisor in a safety plan; don't abandon services.

Sources: NASW (2013). Guidelines for Social Worker Safety in the Workplace, Standard 6: risk assessment for field visits; Standard 9: post-incident reporting and response; NASW Code of Ethics (2021), 1.17(b)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.4 Setting limits and boundaries with clients · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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76. INT-1605 (rev 2) · Best answer: A

An associate social worker sees an adult client by telehealth. The client, angry after losing a custody hearing, says the former partner 'will regret this' and mentions keeping a handgun at home. When asked directly, the client says the comment meant an appeal and denies intent to harm anyone. The associate remains unsure whether protective action is needed, and supervision is scheduled in 6 days. What should the associate do **NEXT**?

A. Contact the supervisor today to review the risk together
B. Bring detailed notes to the scheduled supervision
C. Revisit intent and firearm access at the next session

Why each option is right or wrong

A **Correct.** Part of the supervisee's role is disclosing risk situations promptly and getting direction when unsure of a protective threshold, rather than waiting for a routine meeting. Supervisors share responsibility for the services their supervisees provide (NASW & ASWB supervision standards, 2013), so the supervisor may need to act. A threat-like statement, a firearm and unresolved doubt call for same-day consultation, a core risk-management step (Reamer, 2015). It beats C because the direct questions have already been asked; another round days later delays a decision the associate should not make alone.

B **Incorrect** (trap: Ignores risk). Waiting 6 days leaves a possible threat with firearm access unreviewed. Bringing notes to routine supervision fits stable cases and questions that can safely wait.

C **Incorrect** (trap: Right action, wrong time). Discussing intent and firearm storage again can be useful, but the associate has already asked directly and is still unsure. The decision about protective action needs the supervisor's input now, not at a later session.

Takeaway: A supervisee unsure whether a client's statement requires protective action contacts the supervisor the same day instead of waiting for scheduled supervision.

Sources: NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 3 (Legal and Regulatory Issues): supervisors share responsibility for services to clients; Reamer, F. G. (2015). Risk Management in Social Work: Preventing Professional Malpractice, Liability, and Disciplinary Action. Columbia University Press, Ch. 6: Consultation, Referral, Documentation, and Records
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.1 Supervisee's role: learning needs, self-assessment, prioritizing · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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77. INT-1058 (rev 2) · Best answer: A

In a supportive housing program, two roommates have argued for weeks over the shared kitchen, and each now demands that the other move out. The social worker meets with both, sets ground rules for the talk, and has each describe the conflict. What should the social worker do **NEXT**?

A. Ask each roommate what they need from the kitchen
B. Propose a schedule that splits kitchen hours evenly
C. Ask each roommate to propose a fair solution

Why each option is right or wrong

A **Correct.** Principled negotiation separates positions, the demands people arrive with, from interests, the needs beneath them. 'Move out' is a position; needs such as cooking without waiting or finding a clean counter are interests, and interests open up solutions both people can accept (Fisher et al., 2011). With ground rules set and both sides heard, uncovering interests is the next step. It beats B because an even split is a compromise offered before anyone knows what each roommate needs, so it may satisfy neither; options come after interests are clear.

B **Incorrect** (trap: Right action, wrong time). Generating options is part of resolving the conflict, but proposing one now skips learning what each roommate needs. An even split may miss the real issue, such as cleanliness, and a plan from the worker may not be owned by either party.

C **Incorrect** (trap: Right action, wrong time). Inviting each roommate's ideas builds ownership, and generating options is a real later step. Asked for solutions now, though, each is likely to restate a position ('the other one moves out'). Options work better once the needs beneath those demands are on the table.

Takeaway: In resolving conflict, move the parties from positions to underlying interests before generating or judging options.

Sources: Fisher, R., Ury, W., & Patton, B. (2011). Getting to Yes: Negotiating Agreement Without Giving In (3rd ed.). Penguin Books, focus on interests, not positions
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.19 Resolving conflict · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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78. ETH-1663 (rev 1) · Best answer: A

In a group practice, a social worker learns at the second session that a new client manages the department where the social worker's adult child works. The client is deciding on layoffs in that department. After the social worker explains the conflict, the client says, 'It doesn't bother me. I'd rather stay with you.' What should the social worker do **NEXT**?

A. Arrange for a colleague to take over the client's care
B. Continue, with a written agreement on outside contact
C. Ask how the client feels about the overlap

Why each option is right or wrong

A **Correct.** When a conflict of interest could impair impartial judgment, the social worker resolves it in the client's favor, which can mean ending the relationship with a proper referral (NASW Code 1.06(a)) and an orderly transfer (1.16(b)). The client's layoff choices could affect the worker's own child, so the worker cannot respond neutrally to the client's work stress, whatever the client prefers. A colleague in the same practice makes transfer easy. It beats B because written boundaries handle incidental contact, not a personal stake in the client's decisions.

B **Incorrect** (trap: Good but not best). A boundary agreement fits an unavoidable overlap, such as living on the same block. It cannot remove the worker's personal stake in the client's layoff decisions, which affects judgment inside sessions, not just contact outside them.

C **Incorrect** (trap: Right action, wrong time). Checking the client's comfort is part of disclosing a conflict, and the client has already answered. Asking again delays a decision that rests on the worker's impartiality, which the client's comfort cannot restore.

Takeaway: A conflict that compromises the worker's impartial judgment calls for transfer, even when the client prefers to stay.

Sources: NASW Code of Ethics (2021), 1.06(a); NASW Code of Ethics (2021), 1.16(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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79. ETH-1652 (rev 2) · Best answer: B

On a pediatric intensive care unit, a patient's parent has just learned that the child died. The hospital social worker has supported the family through the 2-week stay. Sobbing, the parent turns and reaches out to hug the social worker. What is the **BEST** response?

A. Offer the parent a hand on the shoulder instead
B. Accept the brief hug and stay with the parent
C. Step back gently and offer a chair and tissues

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A hand on the shoulder is a respectful form of contact and would fit if the parent seemed unsure about touch. Redirecting a hug the parent has already begun, at the moment of the death, signals rejection without reducing any real risk.

B **Correct.** The Code does not ban touch. It warns against contact that could cause psychological harm and makes the social worker responsible for clear, culturally sensitive boundaries when contact is appropriate (NASW Code 1.10). A brief hug, started by a grieving parent at the moment of a child's death on an open unit, is a boundary crossing that meets the parent's need, not the worker's (Reamer, 2020). It beats A because swapping in a different touch at that moment reads as rejection while adding no protection that the brief hug lacks.

C **Incorrect** (trap: Misreads law or ethics). Stepping back treats all physical contact as forbidden, which the Code does not require, and it can feel like abandonment at the

worst moment. Declining touch is right when contact could harm the client or confuse the relationship.

Takeaway: Brief, client-initiated, comforting touch that serves the client's needs can be appropriate; the social worker remains responsible for the boundary.

Sources: NASW Code of Ethics (2021), 1.10; Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, boundary crossings and physical contact
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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80. ASM-1192 (rev 1) · Best answer: C

Five days after surgery that created a permanent colostomy for rectal cancer, a 62-year-old hospital patient will not look at the stoma and asks nurses to handle all of its care. On the social worker's first visit, the patient says, 'I'm disgusting now. Who would want to be near me?' What is the **MOST** appropriate response?

A. Screen the patient for depression with a standard tool
B. Offer a referral to an ostomy peer visitor program
C. Invite the patient to talk about what the change means to them

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Depression screening is reasonable and may follow, especially if hopelessness persists. Opening a first visit with a questionnaire, right after a raw statement of shame, skips past the specific concern the patient just raised.

B **Incorrect** (trap: Right action, wrong time). Peer visitors who live with an ostomy can ease shame and teach practical coping. The referral fits later in the stay, once the social worker understands the patient's concerns; offered now, it can feel like a hand-off.

C **Correct.** Cancer surgery that changes appearance or body function often disrupts body image, and assessment starts with how the change affects the person (White, 2000). At a first visit, empathic exploration of the patient's words builds the relationship and shows what the stoma means for intimacy, identity and closeness (Hepworth et al.). It beats A because the patient has named a specific body-image fear; screening can follow once that meaning is understood.

Takeaway: When illness changes the body, explore what the change means to the person before screening or referring.

Sources: White, C. A. (2000). Body image dimensions and cancer: A heuristic cognitive behavioural model. Psycho-Oncology, 9(3), 183-192, whole article; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Empathic communication
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.10 Influences on self-image and body image · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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81. INT-1544 (rev 1) · Best answer: A

A social worker from a community aging services agency visits a client who lives in a nursing facility. Soon after the client complained about slow call-light response, the facility issued a 30-day discharge notice. The client and social worker met with the administrator, who refused to reconsider. The client wants to stay. What should the social worker do **NEXT**?

A. Help the client contact the long-term care ombudsman program
B. Help the client tour other facilities before the notice period ends
C. Report the facility to adult protective services for neglect

Why each option is right or wrong

A **Correct.** Advocacy moves from informal to formal and outside channels when internal efforts fail. The client already raised the issue with the administrator without success. The long-term care ombudsman program helps residents resolve complaints about their rights, and every discharge notice must list the ombudsman's contact and the resident's appeal rights (42 CFR 483.15(c)(5); 45 CFR 1324.19). The ombudsman can help the client appeal what may be a retaliatory discharge. It beats C because slow call lights alone do not establish neglect, and a protective report would not address the discharge itself.

B **Incorrect** (trap: Good but not best). Touring other facilities accepts a discharge the client wants to fight. It may be a sensible backup later, but the client has appeal rights and has not yet used outside help.

C **Incorrect** (trap: Skips assessment). Slow call-light response is a care-quality complaint, not clear evidence of abuse or neglect, and a protective report would not stop the discharge. A report fits if the visit shows signs of neglect or mistreatment.

Takeaway: After internal efforts fail, link a nursing facility resident to the long-term care ombudsman to contest a discharge or rights problem.

Sources: Requirements for long-term care facilities, 42 CFR 483.15, 42 CFR 483.15(c)(5); State Long-Term Care Ombudsman Program rule, 45 CFR 1324.19, 45 CFR 1324.19(a)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.13 Advocating for needed services and resources · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 1 of 3

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82. ETH-1164 (rev 2) · Best answer: B

A client arrives 25 minutes late for a scheduled 50-minute psychotherapy session at a group practice, and the session lasts 25 minutes. The office manager tells the social worker to bill the client's insurer for the full 50 minutes, since the client chose to come late. How should the social worker bill the session?

A. Bill the full scheduled session, as directed
B. Bill for the time actually provided
C. Submit no claim, since the session was incomplete

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). An office manager's direction does not make an inaccurate claim acceptable. Billing for 25 minutes that were not provided misrepresents the extent of service to the insurer, even though the lateness was the client's choice.

B **Correct.** Billing must accurately reflect the nature and extent of services provided and who provided them (NASW Code 3.05). The client received 25 minutes of psychotherapy, so the claim should reflect that shorter service, however the lateness came about. Billing the scheduled 50 minutes would claim service that never occurred, which is fraud even when the office directs it (4.04). Any late policy agreed with the client must follow payer rules and never justifies overbilling the insurer. It beats C because the 25 minutes of care were real and can be billed accurately.

C **Incorrect** (trap: Good but not best). Skipping the claim is not required: 25 minutes of therapy were delivered and can be billed accurately. Accuracy cuts both ways, and the record and claim should match the service that occurred.

Takeaway: Bills must match the service actually delivered; lateness may be handled by an agreed client policy, never by overbilling insurers.

Sources: NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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83. INT-1519 (rev 1) · Best answer: A

A school requires a 15-year-old to meet with the social worker after a suspension. When the social worker lists the sessions the student must attend and the topics they will cover, the student refuses to talk. At the next meeting, the worker names what is required and lets the student pick the topics and meeting times, and the student begins to participate. Which concept **MOST** likely explains the student's first response?

A. Psychological reactance
B. Learned helplessness
C. Cognitive dissonance
D. Transference

Why each option is right or wrong

A **Correct.** Reactance theory holds that when people sense a threat to their freedom, they push back to restore it (Brehm, 1966). The student shut down when every part of the work was dictated, then engaged once real choices were offered within the school's requirement. That shift with choice is the signature of reactance, and it is why workers with nonvoluntary clients maximize choice (Rooney, 2009). It beats D because transference would follow the student into the relationship whatever the session structure; here, offering choices alone changed the response.

B **Incorrect** (trap: Wrong model or concept). Learned helplessness is passivity after repeated experiences of having no control. The student's first response was active refusal of imposed terms, not resignation or giving up.

C **Incorrect** (trap: Wrong model or concept). Cognitive dissonance is discomfort from holding conflicting beliefs or acting against one's beliefs. Nothing in the stem shows a clash between the student's beliefs and actions.

D **Incorrect** (trap: Wrong model or concept). Transference redirects feelings from earlier relationships, such as with strict caregivers, onto the worker. It would not be expected to fade simply because the meeting structure changed, as happened here.

Takeaway: Threats to freedom provoke reactance; offering real choices within fixed requirements reduces it and helps engage nonvoluntary clients.

Sources: Brehm, J. W. (1966). A Theory of Psychological Reactance. Academic Press, reactance to threatened freedom; Rooney, R. H. (Ed.). (2009). Strategies for Work with Involuntary Clients (2nd ed.). Columbia University Press, reactance and involuntary clients
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.11 Engaging and motivating voluntary and mandated clients · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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84. ASM-1001 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

A 45-year-old client in private practice says their partner of 20 years is having an affair. For 8 months the client has checked the partner's phone, followed the partner to work and read meaning into a moved car seat. No proof has turned up, and the client's certainty has not changed. The client still runs a small business well and speaks clearly. What is the **MOST** likely diagnosis?

A. Delusional disorder
B. Paranoid personality disorder
C. Schizophrenia

Why each option is right or wrong

A **Correct.** Delusional disorder involves one or more fixed false beliefs lasting at least 1 month, without the broader picture of schizophrenia, and with functioning largely intact apart from the belief's effects (DSM-5-TR). The jealous type centers on a partner's supposed unfaithfulness. Eight months of unshakable certainty despite no evidence, with organized speech and a well-run business, fits. It beats B because paranoid personality disorder is a pervasive distrust of many people that starts by early adulthood, not a single fixed belief that began in midlife.

B **Incorrect** (trap: Near-miss diagnosis). Paranoid personality disorder is an enduring pattern of suspicion toward many people, traceable to adolescence or early adulthood. This belief began 8 months ago and focuses on one person, which points to a delusion rather than a personality pattern.

C **Incorrect** (trap: Near-miss diagnosis). Schizophrenia requires two or more core symptoms, such as hallucinations or disorganized speech, with a decline in functioning and signs lasting at least 6 months. A single belief with clear speech and preserved work does not fit that picture.

Takeaway: One fixed false belief for at least a month, with organized thinking and otherwise intact functioning, suggests delusional disorder.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Delusional Disorder; DSM-5-TR (APA, 2022), Personality Disorders: Paranoid Personality Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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85. ASM-1679 (rev 1) · Best answer: D

Which tool has a client list the people in their life and rate each one on concrete help, emotional support, closeness, and how often they are in contact?

A. Ecomap
B. Genogram
C. Culturagram
D. Social network map

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). An ecomap diagrams the client or family and their ties to outside systems such as work, school and health care (Hartman, 1978). It shows the nature of those links, not ratings of each person's support.

B **Incorrect** (trap: Wrong model or concept). A genogram maps family members across at least three generations to show patterns such as illness, loss and relationship conflict (McGoldrick, Gerson & Petry, 2020). It does not rate current support from each person.

C **Incorrect** (trap: Wrong model or concept). A culturagram assesses an immigrant family's cultural context, such as reasons for immigrating, language, legal status and health beliefs (Congress, 1994). It addresses culture and migration rather than support from network members.

D **Correct.** The social network map assesses both the structure and the function of a client's personal network (Tracy & Whittaker, 1990). The client names network members and rates each on the kinds of support given, closeness and frequency of contact, which shows who can actually help. It beats A, the strongest competitor, because an ecomap diagrams the family's connections to outside systems such as school, work and agencies, with lines showing strong or stressful ties, rather than rating individual people on specific types of support.

Takeaway: Use a social network map to rate who in a client's network gives concrete and emotional help; an ecomap maps ties to outside systems.

Sources: Tracy, E. M., & Whittaker, J. K. (1990). The social network map: Assessing social support in clinical practice. Families in Society, 71(8), 461-470, whole article; Hartman, A. (1978). Diagrammatic assessment of family relationships. Social Casework, 59(8), 465-476, the ecomap; McGoldrick, Gerson & Petry (2020). Genograms: Assessment and Treatment (4th ed.). W. W. Norton, purpose of the genogram; Congress, E. P. (1994). The use of culturagrams to assess and empower culturally diverse families. Families in Society, 75(9), 531-540, culturagram components

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.6 Tools for assessing strengths, resources, needs, challenges · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

86. ASM-0106 (rev 1) · Best answer: D

A client who was passed over for a promotion says they feel fine about it. Over the next few sessions, the client often describes the coworker who got the job as bitter, envious and out to undermine the client. Other staff describe the coworker as friendly and supportive. Which defense mechanism **MOST** likely explains the client's view of the coworker?

A. Displacement
B. Reaction formation
C. Rationalization
D. Projection

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Displacement shifts a feeling from its real source to a safer target, such as snapping at a family member after a work setback. The client is not redirecting their own anger; they are seeing their feelings in the coworker.

B **Incorrect** (trap: Wrong model or concept). Reaction formation is acting the opposite of an unacceptable feeling, such as lavishly praising the coworker. The client criticizes the coworker instead, so the behavior is not the opposite of the hidden feeling.

C **Incorrect** (trap: Wrong model or concept). Rationalization offers a reasonable-sounding excuse, such as 'I never wanted that job anyway.' The client is not explaining away the outcome but assigning their own envy to someone else.

D **Correct.** Projection is attributing one's own unacceptable feelings to someone else (Vaillant, 1992). The client denies any hurt about the promotion yet sees the coworker as bitter and envious, the very feelings the client would be expected to have, and others see no such behavior. It beats A because displacement redirects a feeling toward a safer target, whereas projection changes who seems to own the feeling. Recognizing the pattern helps the social worker gently explore the client's disappointment.

Takeaway: Projection assigns one's own unacceptable feelings to another person; displacement redirects the feeling to a safer target.

Sources: Vaillant, G. E. (1992). Ego Mechanisms of Defense: A Guide for Clinicians and Researchers. American Psychiatric Press, projection
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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87. INT-1004 (rev 1) · Best answer: B

A client in a hospital's outpatient cardiac rehabilitation program has medical clearance to exercise but avoids the treadmill. In a CBT session, the social worker helps the client identify the thought 'If I push myself, my heart will stop,' and the client rates belief in it at 90%. What should the social worker do **NEXT**?

A. Remind the client that the cardiologist approved the exercise plan
B. Use guided questions to test how accurate the prediction is
C. Begin graded treadmill sessions starting at a slow speed
D. Explore early experiences that shaped the client's fear of illness

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). The cardiologist's approval is true and relevant, but reassurance rarely shifts a belief held at 90%. CBT helps the client examine the thought directly rather than relying on the authority of the worker or the doctor.

B **Correct.** In cognitive therapy, once an automatic thought is identified and rated, the next step is to evaluate it through Socratic questioning: what supports it, what does not, and what a more balanced view would be (Beck, 2021). The client holds a catastrophic prediction at 90%, so testing it together comes next. It beats C because graded activity works best once the client has examined the prediction and agreed to test it; starting on the treadmill first skips the cognitive step.

C **Incorrect** (trap: Right action, wrong time). Graded activity is a good later step, often set up as a behavioral experiment to test the prediction. Starting it before the thought is examined skips the collaborative evaluation that helps the client agree to try.

D **Incorrect** (trap: Wrong model or concept). Tracing the early roots of a fear belongs to psychodynamic work. CBT stays with the present thought and the evidence for and against it.

Takeaway: In CBT, after identifying and rating an automatic thought, evaluate it with guided Socratic questions before moving to behavioral tests.

Sources: Beck, J. S. (2021). Cognitive Behavior Therapy: Basics and Beyond (3rd ed.). Guilford Press, evaluating automatic thoughts

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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88. INT-1068 (rev 1) · Best answer: B

A social worker is asked to start a daily group on an acute inpatient psychiatric unit. Most patients stay 4 to 7 days, and new patients arrive every day. Which group format fits this unit?

A. Closed group meeting weekly for 8 sessions

B. Open group with each session complete in itself

C. Open group whose themes build week to week

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A closed 8-week group needs stable members who attend every session (Toseland & Rivas, 2017). Patients here leave within a week, so most would miss the majority of sessions. Closed groups fit longer-stay or outpatient programs.

B **Correct.** With short stays and daily admissions, membership changes almost every meeting. Yalom's model for acute inpatient groups uses a single-session time frame: each meeting is self-contained, with its own goals, so every patient can benefit even after attending only once or twice (Yalom, 1983). It beats C because an open format is right for this unit, but themes that build across weeks assume a stable membership that rapid turnover rules out.

C **Incorrect** (trap: Misuses the technique). Open membership suits this unit, but linking themes from week to week assumes members carry continuity between sessions. With stays of 4 to 7 days, most patients would arrive mid-thread and leave before it resolves.

Takeaway: When group membership turns over rapidly, use an open group in which each session is complete in itself.

Sources: Yalom, I. D. (1983). Inpatient Group Psychotherapy. Basic Books, single-session time frame for acute inpatient groups; Toseland, R. W., & Rivas, R. F. (2017). An Introduction to Group Work Practice (8th ed.). Pearson, planning the group: open and closed membership

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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89. ASM-1189 (rev 1) · Best answer: C

A refugee resettlement social worker completes an assessment with an adult client who fled armed conflict 8 months ago. The client reports poor sleep, irritability and occasional nightmares about the war. The client says that what keeps them awake most nights is an eviction notice, a job search with no results, and no word from a sibling still in a camp. What should the social worker do **NEXT**?

A. Refer the client for trauma-focused therapy for war memories

B. Teach sleep hygiene and relaxation skills for the insomnia

C. Help the client address housing, work and family tracing

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Trauma-focused therapy is appropriate if war-related symptoms persist or dominate once current stressors ease. Here the client links most distress to present problems, so a trauma referral first would leave the main drivers untouched.

B **Incorrect** (trap: Good but not best). Sleep skills may ease some symptoms and can be added. They treat the result rather than the cause, though; the client says worry about eviction, work and a missing sibling is what keeps them awake.

C **Correct.** Among people displaced by war, ongoing daily stressors, such as housing insecurity, unemployment and separation from family, explain much of their current distress and partly carry the effect of war exposure (Miller & Rasmussen, 2010). This client names those stressors as the main cause of sleeplessness, so addressing them comes next. It beats A because trauma-focused therapy can follow if war-related symptoms persist once the daily stressors are reduced.

Takeaway: For displaced clients, address current daily stressors such as housing, income and family separation before assuming distress is mainly past trauma.

Sources: Miller, K. E., & Rasmussen, A. (2010). War exposure, daily stressors, and mental health in conflict and post-conflict settings. Social Science & Medicine, 70(1), 7-16, whole article

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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90. ASM-1602 (rev 1) · Best answer: B

A 15-year-old enters a residential program after five foster placements. Records from prior providers list ADHD at age 8, oppositional defiant disorder at age 11 and bipolar disorder at age 13. School and child welfare records show a placement change or a new school shortly before each new diagnosis. Staff observe irritability and poor sleep. What is the **MOST** appropriate way for the social worker to use these records?

A. Adopt the most recent diagnosis as the working one
B. Build a timeline linking symptoms, moves and diagnoses
C. Set the records aside and rely on current observation

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). The latest label is not necessarily the most accurate; it was made with the same partial history as the others. Accepting it without reviewing the pattern could lock in a diagnosis that the records themselves call into question.

B **Correct.** Assessment draws on multiple sources of information, and integrating records means comparing them rather than adopting one label (Hepworth et al.). Here each new diagnosis came soon after a move or school change, a pattern visible only when records are arranged over time. A timeline lets the team see whether symptoms track losses and stress, which may point to trauma responses rather than three separate disorders. It beats C because setting the records aside discards the history that reveals this pattern, which current observation cannot show.

C **Incorrect** (trap: Good but not best). Fresh observation matters, but it covers only a few days in a new setting. Ignoring years of records loses the link between moves and new symptoms, the history most needed to understand the teen's irritability and poor sleep.

Takeaway: Integrate records by comparing them over time with current findings; don't adopt or discard prior labels wholesale.

Sources: Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, assessment: sources of information
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.11 Integrating records and evaluations (medical, school, psychological, employment) · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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91. ASM-1106 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

In a video session, a 52-year-old client mentions plans to wire $6,000 to an online partner they have never met, who needs money for a 'customs fee' abroad. The client has already sent $9,000 in gift cards for earlier emergencies. The client otherwise manages money well and says, 'It's my choice, and I trust them.' What response is **MOST** helpful?

A. Describe common scam signs and offer to check the story together
B. Accept the client's choice and return to the session agenda
C. Urge the client to end contact with the online partner

Why each option is right or wrong

A **Correct.** Self-determination is strongest when the client decides with full information (NASW Code 1.02). A partner never met in person, repeated emergencies, and requests for gift cards and wire transfers are the romance scam signs the Federal Trade Commission describes. Naming these signs and offering to check the story together protects the client's money without taking over the decision. It beats B because accepting the plan without sharing what the social worker recognizes leaves the client open to further loss.

B **Incorrect** (trap: Good but not best). Respecting the decision matters, and the money is the client's. Staying silent about strong scam signs is not neutral, though; it leaves the client choosing without information the social worker has.

C **Incorrect** (trap: Too directive). Telling an adult to cut off the relationship is directive and often backfires, pushing the client to defend the partner. Sharing specific warning signs invites the client to weigh the evidence.

Takeaway: When a competent adult faces likely exploitation, share specific warning signs and offer help while leaving the decision with the client.

Sources: NASW Code of Ethics (2021), 1.02; Federal Trade Commission. What To Know About Romance Scams (consumer advice), Signs of a romance scam
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.15 Exploitation across the lifespan: financial, immigration-related, trafficking · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 2 of 3

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92. ETH-1577 (rev 1) · Best answer: D

A hospital social worker meets with a Black adult patient admitted during a sickle cell pain crisis. The patient says pain medicine has been delayed and that a nurse called the patient 'drug-seeking.' The chart shows pain scores at the top of the scale for 6 hours. What should the social worker do **FIRST**?

A. Help the patient file a grievance with patient relations
B. Explain how the unit decides when to give pain medicine
C. Coach the patient to describe the pain calmly to staff
D. Bring the untreated pain to the care team now

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A grievance is a valid option if the patient wants one, and the social worker can help with it later. Filing first leaves severe pain untreated while the complaint is reviewed.

B **Incorrect** (trap: Good but not best). Explaining unit procedures defends the delay rather than addressing it. The patient's report and the chart point to a treatment gap that needs action, not an explanation of policy.

C **Incorrect** (trap: Too directive). Coaching the patient to stay calm shifts responsibility for the problem onto the patient. Self-advocacy support can come later, but the care team, not the patient, needs to act on the untreated pain.

D **Correct.** Implicit bias can lead clinicians to underrate and undertreat pain in Black patients; in one study, medical trainees who held false beliefs about biological differences rated a Black patient's pain lower (Hoffman et al., 2016). With top-of-scale pain for hours and a stigmatizing label in play, the first step is to advocate with the

team so that pain is reassessed and treated (NASW Code 1.05(b)). It beats A because a grievance documents the problem but leaves the patient in pain during review; it can follow if the patient wants it.

Takeaway: When bias may be driving undertreatment, advocate within the care team for prompt reassessment before pursuing formal complaints.

Sources: Hoffman, K. M., Trawalter, S., Axt, J. R., & Oliver, M. N. (2016). Racial bias in pain assessment and treatment recommendations, and false beliefs about biological differences between blacks and whites. PNAS, 113(16), 4296-4301, whole article; NASW Code of Ethics (2021), 1.05(b)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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93. INT-1580 (rev 1) · Best answer: C

An agency director shares a study of a new parenting program that randomly assigned 3,000 families. Program families scored 1 point lower than controls on a 100-point child stress scale, and the difference was statistically significant. The director proposes replacing the agency's current parenting program with the new one. How should the social worker respond?

A. Support the change, since the result was statistically significant
B. Ask for the study to be repeated with a larger sample
C. Ask whether a change that small matters in practice

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Statistical significance means the difference is probably not due to chance. With 3,000 families, even a trivial difference can reach significance, so it is not by itself a reason to replace an existing program.

B **Incorrect** (trap: Wrong model or concept). A larger sample makes small differences more likely to be significant but does not make them any bigger. The concern here is the size of the effect, not whether it is real.

C **Correct.** Statistical significance says a difference is unlikely to be chance; it says nothing about how large or useful the difference is. With very large samples, tiny differences become significant. Clinical or practical significance asks whether the change makes a real difference in people's lives (Jacobson & Truax, 1991; Rossi, Lipsey & Henry, 2019). A 1-point change on a 100-point scale is unlikely to matter, and switching programs has real costs. This beats A because significance alone does not show the program is worth adopting.

Takeaway: Statistical significance shows a difference is probably real, not that it is large enough to matter; check effect size before changing practice.

Sources: Jacobson, N. S., & Truax, P. (1991). Clinical significance: A statistical approach to defining meaningful change in psychotherapy research. Journal of Consulting and Clinical Psychology, 59(1), 12-19, whole article; Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 9: Detecting, Interpreting, and Exploring Program Effects
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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94. ASM-1108 (rev 1) · Best answer: C

A client's partner died 5 months ago. The client kept working, cared for two children, and handled the funeral and finances. Last week the client's car was towed, and since then the client cannot sleep, cries at work, and says, 'I can't handle one more thing.' In Golan's crisis model, the towing is **MOST** likely which element?

A. Hazardous event
B. Vulnerable state
C. Precipitating factor
D. Active crisis state

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The hazardous event is the initial blow that upsets a person's balance, here the partner's death 5 months ago. The towing came much later and was minor on its own.

B **Incorrect** (trap: Wrong model or concept). The vulnerable state is the person's strained reaction after the hazardous event, such as the months of coping with grief. It describes the client's condition, not the towing.

C **Correct.** Golan (1978) describes a crisis as unfolding from a hazardous event, through a vulnerable state, to a precipitating factor that tips the person into active crisis. The towing is minor by itself, but it arrives while the client is still strained by the loss and turns that strain into acute disorganization. It beats A because the hazardous event is the original blow, the partner's death, which the client coped with for months. Spotting the precipitating factor explains why a small event produces a large reaction.

D **Incorrect** (trap: Wrong model or concept). Active crisis is the acute disorganization itself, such as the sleeplessness and crying at work. The towing set off that state; it is not the state.

Takeaway: In Golan's model, a minor precipitating factor can tip a person already in a vulnerable state into active crisis.

Sources: Golan, N. (1978). Treatment in Crisis Situations. Free Press, Components of the crisis situation
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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95. ETH-0119 (rev 2) · Best answer: A

A client who arrived in the United States as a refugee 4 months ago comes to a community agency for help with sleep problems. At intake, the client hesitates to sign forms and asks whether the information will go to immigration officials. What should the social worker do **FIRST**?

A. Explain what stays confidential and the limits that apply
B. Promise the client that their records will be kept private
C. Complete the trauma history while the client is present

Why each option is right or wrong

A **Correct.** Clients should learn the nature and limits of confidentiality early in the relationship (NASW Code 1.07(e)). A refugee's fear of authorities is a reasonable response to past experience, and clear, honest information builds the trust needed to engage. Explaining who can see the record, and the narrow exceptions such as safety and abuse reporting, answers the client's question directly. It beats B because a blanket promise of privacy is false and would break trust when a limit applies; trustworthiness and transparency are core trauma-informed principles (SAMHSA, 2014).

B **Incorrect** (trap: Misreads law or ethics). A guarantee of total privacy ignores real limits such as imminent danger, abuse reporting and court orders. A false promise may feel reassuring now but damages trust if a limit ever applies.

C **Incorrect** (trap: Right action, wrong time). A detailed trauma history is part of assessment, but pressing for it before the client's question is answered and trust is built can feel like interrogation. It belongs later, once consent and safety are established.

Takeaway: When a client fears how information will be used, explain confidentiality and its limits honestly before gathering sensitive history.

Sources: NASW Code of Ethics (2021), 1.07(e); SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), six key principles: trustworthiness and transparency
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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96. INT-1541 (rev 1) · Best answer: D

A 78-year-old patient is admitted to a hospital medical unit after a stroke that weakened one side of the body. The patient lives with a partner in a two-story house. The medical team has not set a discharge date. What should the social worker do **FIRST**?

A. Ask the physician for an expected discharge date to guide planning
B. Give the patient's partner a list of inpatient rehabilitation facilities
C. Request a home safety evaluation from occupational therapy
D. Meet with the patient and partner to assess needs and supports

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A target date helps with timing, but planning should not wait for it. Starting the assessment now prevents rushed or unsafe arrangements when the date arrives.

B **Incorrect** (trap: Skips assessment). Rehabilitation may well be needed, but handing over a list assumes a placement before needs, preferences and options have been assessed. Facility choices come after the evaluation, with the patient's input.

C **Incorrect** (trap: Right action, wrong time). A home safety review may be needed, but it follows from a needs assessment and goes through the team's usual orders. The social worker first learns the patient's and partner's situation and goals.

D **Correct.** Discharge planning starts at admission: hospitals identify patients early who are likely to need help after discharge and complete a discharge planning evaluation promptly, so arrangements are ready in time and the patient and family take part (42

CFR 482.43(a)). New one-sided weakness and a two-story home signal likely needs, so the social worker first meets the patient and partner to assess function, caregiving capacity, home layout and preferences. It beats A because waiting for a date delays the assessment that the date and plan depend on.

Takeaway: Discharge planning begins at admission with an assessment of the patient's needs, supports and preferences, not when a discharge date is set.

Sources: Medicare Conditions of Participation: Discharge planning, 42 CFR 482.43, 42 CFR 482.43(a)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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97. ASM-1190 (rev 1) · Best answer: B

A 16-year-old has lived with the same foster parents for 4 years, and they ask to adopt. The youth tells the child welfare social worker, 'I love them, but I won't give up my name or act like my birth family is gone.' The youth wants to stay in the home. Agency staff prefer adoption as the most permanent option. What is the **MOST** appropriate next step?

A. Ask the court to waive the youth's consent to adoption
B. Review adoption and guardianship choices with the youth
C. Change the goal to independent living without a legal parent

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Some courts can waive an older child's consent, but seeking a waiver overrides a capable 16-year-old's clear wishes before anyone has explored them. It also risks a placement the youth resents and may undermine.

B **Correct.** Nearly every state requires an older child's consent to adoption, with set ages of 10 to 14 (Child Welfare Information Gateway, 2021). Self-determination also means informed choice (NASW Code 1.02). The youth's concerns can be met in more than one way: adoption does not always mean a name change, and legal guardianship keeps the home permanent while preserving identity. Reviewing both with the youth respects the youth's voice and still pursues permanency. It beats A because a waiver overrides the youth before options are explored.

C **Incorrect** (trap: Good but not best). Independent living without a legal parent gives up lifelong family ties the youth clearly wants. It honors the objection to adoption but drops permanency, when other legal options would keep the youth in this home.

Takeaway: An adolescent's objection to adoption calls for informed review of permanency options, including guardianship, not override or loss of permanency.

Sources: Child Welfare Information Gateway (2021). Consent to Adoption (State Statutes Series), Consent of the adoptee; NASW Code of Ethics (2021), 1.02
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.23 Client capacity, self-determination, self-monitoring (consent age, emancipation, permanency) · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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98. INT-0101 (rev 1) · Best answer: D

At a hospital care conference, the physician and nurse recommend discharging a patient with heart failure to a skilled nursing facility. The social worker's assessment shows that the patient has decision-making capacity, wants to return home, and has a relative willing to help with home services. What is the social worker's **BEST** action?

A. Accept the physician's recommendation as the team's decision
B. Encourage the patient privately to refuse the facility placement
C. Refer the disagreement to the hospital ethics committee
D. Present the patient's goals and a home-support plan to the team

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Deferring to the physician leaves out information the team needs and lets a plan go forward against a capable patient's wishes. Medical input matters, but placement also rests on psychosocial factors and the patient's choice.

B **Incorrect** (trap: Boundary slip or collusion). Coaching the patient behind the team's back splits the team before the social worker has even shared the assessment. Supporting the patient's voice works best openly, in the conference where the decision is made.

C **Incorrect** (trap: Right action, wrong time). An ethics committee consult is a reasonable later step if the team still overrides a capable patient's choice. Using it first skips the direct team discussion the Code expects (NASW Code 2.03(b)).

D **Correct.** Social workers on interdisciplinary teams contribute the social work perspective to decisions, including the client's own goals (NASW Code 2.03(a)). The decisive facts are that the patient has capacity, wants to go home, and has a workable support plan, so the social worker brings that assessment into the conference where the decision is being made, supporting self-determination (NASW Code 1.02). It beats C because the Code directs social workers to try resolving team disagreement through the team's own channels first; an ethics consult fits if the team still overrides a capable patient.

Takeaway: On an interdisciplinary team, bring the client's goals and the psychosocial assessment into team discussion before escalating elsewhere.

Sources: NASW Code of Ethics (2021), 2.03(a); NASW Code of Ethics (2021), 2.03(b); NASW Code of Ethics (2021), 1.02
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.3 Inter- and intradisciplinary teamwork (co-therapy, care conferences, IEPs) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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99. ETH-1612 (rev 1) · Best answer: C

At a community health center, a social worker completes an intake with an adult client who was resettled as a refugee 3 months ago after 6 years in a refugee camp. The client reports poor sleep, headaches and worry about a sibling still abroad. What is the **MOST** appropriate way for the social worker to structure the assessment?

A. Focus on current resettlement needs such as housing and work
B. Begin with detailed processing of the most painful war memories
C. Ask about life before leaving, in transit and since arriving

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Housing, work and benefits are pressing and belong in the plan. Assessing only the present, though, misses how years in a camp and losses before leaving may drive the client's symptoms.

B **Incorrect** (trap: Right action, wrong time). Trauma processing can help later with a trained provider and a stable client. Starting an intake with detailed exposure to the worst memories, before safety and trust exist, can overwhelm a newly resettled client.

C **Correct.** A stage-of-migration framework assesses experiences before departure, during transit and after arrival, because stressors and strengths from each stage shape current functioning (Drachman, 1992). This client's history spans the home country, 6 years in a camp and 3 months of resettlement, plus ongoing worry about a sibling abroad, so the symptoms may link to any stage. It beats A because current needs are urgent and must be met, but focusing only on them misses trauma, losses and strengths from earlier stages.

Takeaway: Assess refugees and immigrants across migration stages, before departure, in transit and after arrival, since each can shape current functioning.

Sources: Drachman, D. (1992). A stage-of-migration framework for service to immigrant populations. Social Work, 37(1), 68-72, whole article

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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100. ASM-1655 (rev 2) · Best answer: A

A 56-year-old client has lived in emergency shelters for 8 years. Shelter staff tell the social worker that the client has fallen twice this month, struggles to climb to an upper bunk, and recently wet the bed. Staff add that the client is 'too young' for the agency's senior services. What is the **MOST** appropriate action?

A. Arrange a geriatric assessment of function, falls and continence

B. Move the client to a lower bunk to prevent further falls

C. Plan to apply for senior services once the client reaches age 60

Why each option is right or wrong

A **Correct.** People who have been homeless for years often develop geriatric conditions early. In a study of shelter residents averaging about 56 years old, about half had fallen in the past year or had incontinence, and functional impairment and incontinence exceeded rates in housed cohorts about 20 years older (Brown et al., 2012). This client's falls, mobility trouble and new incontinence call for a geriatric assessment of function now, whatever the calendar age. It beats B because a lower bunk reduces one hazard but leaves the causes of the falls and incontinence unexamined.

B **Incorrect** (trap: Good but not best). A lower bunk is a sensible safety step that can be done right away, but it treats one symptom. Repeated falls, mobility loss and new incontinence together need assessment to find causes and plan care.

C **Incorrect** (trap: Ignores risk). Waiting for an age cutoff accepts the staff's view that the client is too young for these problems. Long-term homelessness speeds up aging, so the client needs assessment now, not in several years.

Takeaway: Long-term homelessness brings geriatric conditions early; assess function, falls and continence based on need, not calendar age.

Sources: Brown, R. T., Kiely, D. K., Bharel, M., & Mitchell, S. L. (2012). Geriatric syndromes in older homeless adults. Journal of General Internal Medicine, 27(1), 16-22, whole article
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.20 Displacement effects: homelessness, immigration, refugee status · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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101. ETH-0107 (rev 2) · Best answer: A

An adult client with bipolar I disorder left a psychiatric unit 3 weeks ago. The client now declines the mood stabilizer the psychiatrist prescribed, preferring to manage symptoms with sleep routines and a peer support group. The client sleeps normally, is not manic and denies suicidal thoughts. What is the social worker's **BEST** response?

A. Respect the choice and build a plan for early warning signs
B. Ask the psychiatrist to seek a court order for the medication
C. Tell the client that relapse is likely without the medication

Why each option is right or wrong

A **Correct.** Competent adults may refuse treatment, and the Code permits limiting self-determination when a client's actions pose a serious, foreseeable and imminent risk (NASW Code 1.02). The client is stable, not manic and not suicidal, so no override applies. Building a plan for early warning signs, with the client's own steps and contacts, respects the choice while reducing relapse risk, consistent with person-driven recovery (SAMHSA, 2012). It beats C because accurate information about relapse belongs in the conversation, but stating it as a prediction lectures the client and does not help them act on their choice.

B **Incorrect** (trap: Overrides the client's choice). Court-ordered medication is reserved for people who lack capacity or pose serious danger under state law. A stable, competent client who declines a medication does not meet that bar, so this overrides a lawful choice.

C **Incorrect** (trap: Too directive). Sharing relapse risk is part of informed choice, but leading with a warning becomes persuasion. It tends to raise defensiveness and may make the client less open with the team about symptoms later.

Takeaway: A competent adult may decline medication; respect the choice, share information and plan for warning signs unless risk becomes imminent.

Sources: NASW Code of Ethics (2021), 1.02; SAMHSA (2012) Working Definition of Recovery (PEP12-RECDEF), guiding principles: recovery is person-driven
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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102. ETH-1553 (rev 1) · Best answer: C

A supervisee in a group practice misses clear signs that a client's condition is worsening, and the client is harmed. The supervisor had met with the supervisee weekly and reviewed cases as agreed. The client's lawsuit names both the supervisee and the supervisor. Which statement about the supervisor's legal exposure is accurate?

A. Regular weekly review shields the supervisor from liability
B. Liability requires proof that guidance to the supervisee was faulty
C. The supervisor may share liability for the supervisee's acts

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Diligent, documented supervision lowers risk and helps a supervisor's defense, but it does not remove vicarious liability, which rests on the supervisory relationship rather than on the supervisor's own fault.

B **Incorrect** (trap: Wrong model or concept). This describes direct liability, which applies when a supervisor's own recommendations or assignments cause harm. A supervisor can also face vicarious liability for a supervisee's errors without any personal fault.

C **Correct.** Under vicarious liability (respondeat superior), a supervisor can be held responsible for a supervisee's negligent acts or omissions within the scope of supervision, even when the supervisor did nothing wrong (Supervision Standards, Standard 3: Liability). Here the supervisor met weekly as agreed, yet the claim can still reach the supervisor through the supervisee's error. It beats B because B describes only direct liability, which requires the supervisor's own faulty guidance; vicarious liability does not.

Takeaway: Supervisors can be vicariously liable for supervisees' errors even without personal fault; direct liability arises from the supervisor's own faulty guidance.

Sources: NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 3 (Legal and Regulatory Issues): Liability
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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103. ETH-1681 (rev 2) · Best answer: C

PRETEST — unscored on this form; do not count it

A former supervisee asks a clinical supervisor for a reference letter for a hospital job. During supervision, the supervisee was warm with patients but often missed documentation deadlines, and two written improvement plans addressed it. The supervisee asks the supervisor to 'focus on the positives.' Which response is **MOST** consistent with ethical standards?

A. Write a letter on strengths alone, leaving out the deadline issues
B. Write a balanced letter and send it straight to the hospital
C. Explain that an honest letter would include the deadline issues

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Leaving out a problem that took two improvement plans creates a misleading picture through omission. The hospital relies on the letter to judge fitness for a role in which documentation affects patient care.

B **Incorrect** (trap: Right action, wrong time). A balanced letter is the right content, so this is tempting. Sending it without first telling the supervisee, who asked for a positives-only letter, takes away the supervisee's informed choice to use it or ask someone else, which fairness in evaluation calls for.

C **Correct.** Social workers should not participate in or be associated with dishonesty, and evaluations of others should be fair and based on clearly stated criteria (NASW Code 4.04; 3.03). A letter that omits a documented, repeated problem misleads the hospital about a skill that affects patient care. Telling the supervisee up front what an honest letter would say lets the supervisee decide whether to use it. It beats B because a balanced letter is the right product, but sending it unannounced, after the supervisee asked for something else, removes the supervisee's choice about whether to use it.

Takeaway: References for supervisees should be honest; tell the requester what an accurate letter will include rather than omitting documented concerns.

Sources: NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 3.03
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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104. ASM-1071 (rev 2) · Best answer: A

In an integrated primary care clinic, an adult taking escitalopram for depression reports feeling much better after 2 months. Pharmacy records show doses are often skipped on weekends. When the social worker asks about the missed doses, the client hesitates and says, 'It's kind of personal.' What should the social worker do **NEXT**?

A. Ask plainly whether sexual side effects play a part
B. Notify the prescriber that the client skips doses
C. Remind the client that daily doses keep the mood stable

Why each option is right or wrong

A **Correct.** Sexual side effects are among the most common reasons people skip or stop SSRIs, and brief weekend breaks from some SSRIs can ease them, which makes weekend-only skipping a telling pattern (Rothschild, 1995). Clients often do not raise the topic unprompted, so after a hesitant 'it's personal,' a plain, nonjudgmental question makes disclosure easier. It beats B because the prescriber can do more once the reason is known, which opens options such as a dose or medication change (Kaplan & Sadock).

B **Incorrect** (trap: Right action, wrong time). The prescriber should hear about missed doses, and this step will likely follow. Reporting before the reason is known gives the prescriber little to act on and skips the client's chance to explain.

C **Incorrect** (trap: Too directive). Education about steady dosing has value, but repeating it before understanding the reason for skipping can feel like a lecture and may close off disclosure of an embarrassing side effect.

Takeaway: When a client hesitates about skipped antidepressant doses, ask plainly about sexual side effects before reporting or educating.

Sources: Rothschild, A. J. (1995). Selective serotonin reuptake inhibitor-induced sexual dysfunction: Efficacy of a drug holiday. American Journal of Psychiatry, 152(10), 1514-1516, whole article; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), SSRIs: adverse effects, sexual dysfunction
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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105. ETH-1186 (rev 2) · Best answer: C

A hospital social worker notices that uninsured patients discharged to shelters rarely leave with a follow-up appointment, while insured patients almost always do. For 3 months, the social worker has arranged appointments case by case and tracked the gap, which has not narrowed. What should the social worker do **NEXT**?

A. Keep arranging follow-up appointments for each affected patient
B. File a complaint about the hospital with a federal civil rights office
C. Present the findings to leadership with proposed changes

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Arranging appointments helps each patient and should continue, but 3 months of data show the pattern persists. Staying at the individual level leaves the process that produces the gap unchanged.

B **Incorrect** (trap: Skips assessment). Outside complaints may be appropriate if internal efforts fail. Going there first skips the hospital's own channels, which have not yet seen the data and may correct the process quickly.

C **Correct.** The NASW Code's Social Justice principle asks social workers to challenge injustice and ensure access to needed services, and 3.09(b) asks them to work to improve their organizations' policies and procedures. The social worker has already handled individual cases and documented a pattern that case-by-case work has not fixed, so the next step is to bring the data to the people who can change the process. It beats A because continuing case by case helps each patient but leaves the inequity in place for patients the social worker never sees.

Takeaway: When individual help cannot fix a documented pattern of unequal access, take the data to organizational decision-makers to change the system.

Sources: NASW Code of Ethics (2021), Ethical Principles: Social Justice; NASW Code of Ethics (2021), 3.09(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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106. INT-1660 (rev 1) · Best answer: C

A social worker is drafting a year-end report to a foundation that funded a caregiver support program. The grant target was 100 enrolled caregivers; 72 enrolled, and 40 more called for information but did not enroll. The program director suggests counting the callers as clients served to meet the target. How should the social worker report enrollment?

A. Report the callers as enrolled and add a footnote
B. Omit the enrollment figure and lead with outcome gains
C. Report 72 enrolled and explain the gap and next steps

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A footnote makes the change visible, but the headline number would still count people who never enrolled toward an enrollment target, and readers relying on it would be misled. Inquiries can be reported separately as interest in the program.

B **Incorrect** (trap: Misreads law or ethics). Leading with outcomes is fine, but leaving out a required figure because it falls short misrepresents the program by omission. Funders expect results against every stated objective, including those not met.

C **Correct.** Formal reports must describe results accurately: social workers should report evaluation findings accurately and not falsify them (NASW Code 5.02(o)), and should not take part in deception (4.04). Reporting the 72 enrolled caregivers, with the reasons for the gap and a plan to close it, keeps the report honest and gives the funder what it needs to judge the program; the 40 inquiries can appear as a separate line. It beats A because a footnote does not fix counting callers who never enrolled toward a target defined by enrollment.

Takeaway: Report program results accurately against the stated target, explaining shortfalls rather than redefining measures or omitting them.

Sources: NASW Code of Ethics (2021), 5.02(o); NASW Code of Ethics (2021), 4.04
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.8 Writing formal documents: grants, proposals, reports, evaluations · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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107. ETH-1055 (rev 2) · Best answer: A

After reviewing a copy of their record, a client in an outpatient substance use program asks in writing that a note be deleted. The note states that the client arrived for group smelling of alcohol and slurring words, which the social worker observed directly. The client says the note is unfair. What is the **BEST** response?

A. Explain the client's right to add a written statement of disagreement
B. Amend the note, since clients have a right to request amendment
C. Explain that signed notes are final and cannot be amended

Why each option is right or wrong

A **Correct.** Under HIPAA, clients may request amendment of their records, and a provider may deny the request when the record is accurate and complete. The written denial must tell the client of the right to submit a statement of disagreement, which then accompanies the record (45 CFR 164.526(d)). This note records what the social worker directly observed, so it is accurate and stays (NASW Code 3.04(a)). A beats C because C tells the client that no amendment process exists, denying a right the client holds.

B **Incorrect** (trap: Misreads law or ethics). The right is to request amendment, not to have every request granted; a provider may deny it when the record is accurate and complete (45 CFR 164.526(a)(2)(iv)). The worker observed these facts directly, so changing the note would make the record less accurate (3.04(a)); the client's remedy is a statement of disagreement.

C **Incorrect** (trap: Misreads law or ethics). Accurate notes need not be changed, but saying signed notes can never be amended is wrong. Clients may request amendment, inaccurate entries must be corrected, and here the client may add a statement of disagreement.

Takeaway: Do not change accurate records at a client's request; deny in writing and offer the right to file a statement of disagreement.

Sources: HIPAA Privacy Rule, 45 CFR 164.526(d); HIPAA Privacy Rule, 45 CFR 164.526(a)(2)(iv); NASW Code of Ethics (2021), 3.04(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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108. ETH-1134 (rev 1) · Best answer: C

An adult client asks a community agency for a copy of their record. During treatment, the client's sibling met once with the social worker to share family history, after the worker agreed to keep the sibling's comments private. Those comments appear in the record and would identify the sibling. What is the **BEST** response to the client's request?

A. Release the full record, including the sibling's comments
B. Offer a verbal summary of the record instead of copies
C. Release the record with the sibling's comments withheld

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). The client's right of access is strong, and most of the record should be released. But information a non-provider gave under a promise of confidentiality may be withheld when it would reveal the source, and the Code asks workers to protect others named in records.

B **Incorrect** (trap: Good but not best). A summary falls short of the access the client is entitled to. Only the sibling's comments may be withheld; the rest of the record must be provided as requested.

C **Correct.** Clients have a strong right to see their records (NASW Code 1.08(a)), and social workers protect the confidentiality of others named in them (1.08(c)). HIPAA allows denial of information obtained from a non-provider under a promise of confidentiality when access would reveal the source, but requires access to the rest of

the record (45 CFR 164.524(a)(2)(v), 164.524(d)(1)). C honors both duties. C beats A because releasing everything breaks the promise made to the sibling, which the rules let the worker keep.

Takeaway: Give clients their records, withholding only third-party information shared under a promise of confidentiality that would reveal its source.

Sources: NASW Code of Ethics (2021), 1.08(a); NASW Code of Ethics (2021), 1.08(c); HIPAA Privacy Rule, 45 CFR 164.524(a)(2)(v); HIPAA Privacy Rule, 45 CFR 164.524(d)(1)

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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109. INT-1613 (rev 2) · Best answer: B

At a youth residential program, a resident left the building at night and was injured. Required 15-minute checks had been logged but not done. The director asks the clinical social work manager to lead the response and revise the overnight safety procedures. What should the manager do **FIRST**?

A. Draft stricter overnight check rules for board approval
B. Review the incident with involved staff to find system causes
C. Retrain overnight staff on the check procedure right away
D. Gather sample procedures from similar programs

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Revised procedures are the goal, but writing them before learning why logged checks were not done risks a policy that fails the same way. Drafting follows the review.

B **Correct.** Risk-reducing policy starts with understanding why the failure happened. Root cause analysis looks at systems and processes, such as staffing, workload and how checks are logged and monitored, rather than only individual performance (The Joint Commission, Sentinel Event Policy). Checks logged but not done signals a system gap that new rules alone may not fix (Reamer, 2015). It beats A because rules written before the causes are known may repeat the same gap between written policy and actual practice.

C **Incorrect** (trap: Skips assessment). Retraining feels responsive and may be part of the fix. Done before the review, it assumes the cause was a knowledge gap, when checks logged but not done may reflect staffing, workload or monitoring problems that training will not touch.

D **Incorrect** (trap: Right action, wrong time). Other programs' procedures can inform the revision once this program's failure points are known. Collecting them first skips the analysis of what went wrong here.

Takeaway: Before revising a safety policy after an incident, analyze the incident with involved staff to find system causes.

Sources: The Joint Commission (2024). Sentinel Event Policy and Procedures. Comprehensive Accreditation Manual, Root cause analysis (comprehensive systematic analysis); Reamer, F. G. (2015). Risk Management in Social Work: Preventing Professional Malpractice, Liability, and Disciplinary Action. Columbia University Press, Ch. 1: Professional Risk Management: An Overview

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.4 Developing and evaluating risk-reducing policies and procedures · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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110. INT-1002 (rev 1) · Best answer: A

A client with a history of chaotic early caregiving often arrives furious and accuses the social worker of not caring. The social worker keeps the same session time and room, responds calmly without retaliating, and reliably resumes the work the following week. Which psychodynamic concept is the social worker's stance **MOST** likely drawing on?

A. Holding environment
B. Transitional object
C. Mirroring transference
D. Working through

Why each option is right or wrong

A **Correct.** Winnicott described the holding environment: the reliable, attuned care a good-enough caregiver provides, which the therapeutic frame can recreate (Winnicott, 1965). A steady schedule, a consistent room, and a calm, nonretaliating response to rage give the client a relationship that survives strong feelings, which chaotic early care did not provide. It beats D because working through is the client's repeated processing of insights over time, not the worker's reliable stance.

B **Incorrect** (trap: Wrong model or concept). A transitional object, such as a toddler's blanket, helps a child bridge the move from dependence toward separateness. It is also Winnicott's idea, but it names something the client uses, not the worker's reliable frame.

C **Incorrect** (trap: Wrong model or concept). A mirroring transference, from Kohut's self psychology, is the client's need to be affirmed and admired by the worker (Kohut, 1971). Here the client is attacking, and the worker's steadiness, not affirmation, is the point.

D **Incorrect** (trap: Wrong model or concept). Working through is the gradual, repeated processing of insights as they show up across situations. It describes the client's ongoing work after interpretation, not the consistency and survival the worker offers.

Takeaway: Winnicott's holding environment: a reliable frame and a nonretaliating response let the client experience a relationship that survives strong feelings.

Sources: Winnicott, D. W. (1965). The Maturational Processes and the Facilitating Environment. Hogarth Press, holding environment; Kohut, H. (1971). The Analysis of the Self. International Universities Press, mirror transference

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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111. ETH-1643 (rev 2) · Best answer: C

A family services agency director says, 'We treat everyone the same, so we don't collect race or ethnicity data. Asking would only make clients uncomfortable.' The social worker leads the agency's quality improvement work. What is the **MOST** appropriate response?

A. Offer staff implicit bias training instead of collecting data
B. Collect race data just from clients who report unfair treatment
C. Propose collecting self-reported race data to find gaps

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Bias training is a common response and may help staff reflect, which makes it appealing when data collection meets resistance. Training alone cannot show whether outcomes differ by group or whether anything changed afterward; finding and tracking gaps takes data broken down by race and ethnicity.

B **Incorrect** (trap: Misuses the technique). Complaint-based data reflect who feels safe to speak up, not how services work for each group. Disparities in waitlists, referrals or outcomes often go unreported and only appear in agency-wide data.

C **Correct.** Racial justice requires seeing disparities before fixing them, and disparities in care are found by collecting quality data broken down by race, ethnicity and language (Institute of Medicine, 2009). A 'treat everyone the same' policy without data cannot show whether outcomes differ by group. Voluntary self-report respects clients' comfort because they may decline to answer. It beats B because collecting data only from clients who complain captures a biased slice and misses patterns no one reports. The Code asks social workers to hold institutions accountable for culturally responsive practice (NASW Code 1.05(c)).

Takeaway: Colorblind policies hide disparities; voluntary, self-reported race and ethnicity data let agencies find and correct unequal outcomes.

Sources: Institute of Medicine (2009). Race, Ethnicity, and Language Data: Standardization for Health Care Quality Improvement. National Academies Press, Summary; NASW Code of Ethics (2021), 1.05(c)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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112. ETH-1031 (rev 1) · Best answer: C

When a parent gives legal permission for a 10-year-old's treatment, what term describes the child's own agreement to take part?

A. Proxy consent
B. Implied consent
C. Assent
D. Substituted judgment

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Proxy consent is permission given by someone authorized to decide for another person, such as the parent here. It names the parent's role, not the child's agreement.

B **Incorrect** (trap: Wrong model or concept). Implied consent is inferred from conduct or assumed in emergencies, such as treating an unconscious patient. It does not describe a child's stated agreement to planned services.

C **Correct.** Assent is a child's agreement to services, sought at a level the child can understand, while the parent provides legal permission (Katz & Webb, 2016). Seeking assent respects the child's growing autonomy and parallels the Code's duty to inform people who cannot give legal consent (NASW Code 1.03(c)). C beats A because proxy consent describes the parent's permission given on the child's behalf, not the child's own agreement.

D **Incorrect** (trap: Wrong model or concept). Substituted judgment is a standard a surrogate uses to decide as the person would have chosen. It guides decision-makers for adults who have lost capacity, not a child's agreement.

Takeaway: Parents give legal permission for a child's treatment; the social worker also seeks the child's assent at a developmentally suitable level.

Sources: Katz, A. L., Webb, S. A., & AAP Committee on Bioethics (2016). Informed consent in decision-making in pediatric practice. Pediatrics, 138(2), e20161485, Assent and parental permission; NASW Code of Ethics (2021), 1.03(c)

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.4 Informed consent: principles and how it is obtained · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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113. ASM-1161 (rev 1) · Best answer: A

An inpatient with bipolar I disorder who lives alone is being considered for discharge. The team asks the social worker to describe the client's judgment for the mental status exam. Which question serves this purpose?

A. 'What would you do if you ran out of medication at home?'

B. 'Do you believe you have an illness that needs treatment?'

C. 'What does it mean that the early bird catches the worm?'

Why each option is right or wrong

A **Correct.** Judgment is the ability to weigh a situation and choose a sensible course of action, while insight is awareness of having an illness (Kaplan & Sadock). For a client about to live alone, asking how they would handle running out of medication tests practical decision-making in a situation they are likely to face. It beats B because awareness of illness is a separate mental status area; a client may admit being ill yet still make unsafe choices.

B **Incorrect** (trap: Wrong model or concept). This question measures insight, the client's awareness of illness and need for treatment. Insight matters for discharge too, but it is recorded separately and does not show how the client solves real problems.

C **Incorrect** (trap: Wrong model or concept). Proverb interpretation tests abstract thinking, and results depend heavily on education, language and culture. It says little about whether the client can make safe choices at home.

Takeaway: Judgment is assessed by how a client would handle realistic decisions; insight is awareness of illness; proverbs test abstraction.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Mental status examination: judgment and insight

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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114. ETH-1042 (rev 1) · Best answer: A

An adult client in outpatient therapy discloses being sexually abused by a family friend from ages 8 to 11. The client recently learned that this person now coaches a youth soccer team. The client does not want to be part of any report. The social worker is unsure whether the law requires a report in this situation. What is the **BEST** way to proceed?

A. Seek consultation about the applicable reporting law
B. Report the family friend to child protective services now
C. Encourage the client to make the report personally
D. Explain that abuse of an adult in childhood cannot be reported

Why each option is right or wrong

A **Correct.** Whether an adult's disclosure of childhood abuse must be reported, especially when the alleged abuser has current access to children, varies by state. When the worker is unsure, the next step is consultation with a supervisor or legal counsel on the applicable law (NASW Code 2.05(a)), because the answer decides whether the worker must report regardless of the client's wishes. A beats C because the client has already declined involvement, and urging a self-report does not settle the worker's own possible duty.

B **Incorrect** (trap: Skips assessment). A report may prove to be required, and many states would allow one. Filing before learning the law risks disclosing an adult client's history without a legal basis and against the client's stated wishes (NASW Code 1.07(c)).

C **Incorrect** (trap: Good but not best). Supporting the client's own choice to report can be empowering and might be offered later. The client has already declined, and the worker's possible legal duty does not depend on what the client decides.

D **Incorrect** (trap: Misreads law or ethics). Some states do require or allow reports when an adult discloses past abuse and the abuser still has access to children. Telling the client nothing can be done misstates the law and may leave children unprotected.

Takeaway: When a reporting duty is unclear, as with adult disclosures of past abuse, consult on the applicable law before acting or dismissing it.

Sources: NASW Code of Ethics (2021), 2.05(a); NASW Code of Ethics (2021), 1.07(c)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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115. ETH-1010 (rev 2) · Best answer: B

A child welfare social worker returns to their car after a home visit and finds that the agency laptop has been stolen. The laptop holds case files on several families and lacks encryption. What should the social worker do **FIRST**?

A. List which families' files were stored on the laptop
B. Report the theft to the privacy officer
C. Call each affected family to warn them of the breach
D. File a police report and wait to see if it is recovered

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Identifying the affected files matters, and the agency will need that list. It should not delay the report, though: agency records can show which files were stored, and the privacy officer can start that review at once.

B **Correct.** A possible breach of client records is reported at once through the agency's breach procedures, so the agency can assess the risk and notify affected people as the law requires (NASW Code 1.07(o); 45 CFR 164.404). The privacy officer coordinates notification, police contact and mitigation. B beats A because listing the files is useful, but the agency can do it from its own records, and waiting slows a response that is time-limited by law.

C **Incorrect** (trap: Right action, wrong time). Affected families may need to be told, but notification follows the agency's breach assessment and legal requirements. Calling families personally before the agency acts can give them inconsistent or incomplete information.

D **Incorrect** (trap: Ignores risk). A police report is reasonable, and the agency may make one, but waiting for recovery delays the breach response. Unencrypted files may already be exposed, so the agency must be told right away.

Takeaway: Report a suspected breach of client records to the agency's privacy officer at once; the agency assesses and notifies affected clients.

Sources: NASW Code of Ethics (2021), 1.07(o); HIPAA Breach Notification Rule, 45 CFR 164.404
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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116. INT-1142 (rev 1) · Best answer: D

A family therapy team meets with parents, a grandparent, and a 16-year-old who has stopped eating meals with the family. At the end of the session, the team says the teen's refusal keeps the parents united and the grandparent's frequent advice keeps everyone close. Each member's behavior is described as a sacrifice for the family's unity. Which technique is the team **MOST** likely using?

A. Prescribing the symptom
B. Unbalancing
C. Circular questioning
D. Positive connotation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Symptom prescription is a paradoxical directive that tells the family to keep doing the problem behavior, used in strategic and Milan work. The team here describes the meaning of each person's behavior but gives no directive.

B **Incorrect** (trap: Wrong model or concept). Unbalancing is a structural move in which the worker temporarily sides with one member to shift the family's power arrangement (Minuchin & Fishman, 1981). The team treats all members alike rather than backing one.

C **Incorrect** (trap: Wrong model or concept). Circular questions ask members about differences and relationships, such as who reacts most when something happens. The team here makes a closing statement rather than asking questions.

D **Correct.** Positive connotation, a Milan systemic technique, gives a good motive to every family member's behavior, including the symptom, as serving the family's cohesion (Selvini Palazzoli et al., 1978). The team gives the teen's refusal and the grandparent's advice the same caring purpose, which avoids blaming anyone and joins the whole family. It beats A because prescribing the symptom directs the family to continue or exaggerate the problem; this team explains each behavior's function but gives no instruction.

Takeaway: Milan positive connotation frames every member's behavior, symptom included, as serving family cohesion, without blame or directives.

Sources: Selvini Palazzoli, M., Boscolo, L., Cecchin, G., & Prata, G. (1978). Paradox and Counterparadox. Jason Aronson, positive connotation; Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press, unbalancing

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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117. ASM-1011 (rev 1) · Best answer: B

An assertive community treatment team visits a client with schizophrenia who has stopped taking medication. The client calmly explains that the voices are real broadcasts from neighbors and that nothing is wrong with their mind. When the team reviews two past hospitalizations, the client agrees they happened but still sees no illness. Which concept **MOST** likely explains the client's view?

A. Denial
B. Anosognosia
C. Avolition

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Denial is a defense that keeps painful facts out of awareness, often with anxiety or avoidance. This client calmly reviews the evidence yet cannot see the illness, which points to a symptom of schizophrenia rather than a defense.

B **Correct.** Anosognosia is an illness-related inability to recognize one's own disorder. DSM-5-TR notes that unawareness of illness in schizophrenia is typically a symptom of the disorder itself rather than a coping strategy, comparable to unawareness of deficits after brain injury, and that it predicts nonadherence and relapse. The client's calm certainty, unchanged by clear evidence, fits. It beats A because denial guards against painful emotion and usually shows distress or avoidance; this client discusses the hospitalizations openly and simply does not register them as illness.

C **Incorrect** (trap: Near-miss diagnosis). Avolition, a negative symptom, is reduced drive to start and sustain activities. It can contribute to stopping medication, but it does not explain the client's firm belief that nothing is wrong.

Takeaway: In schizophrenia, unawareness of illness is often anosognosia, a symptom of the disorder rather than a defense, and it predicts nonadherence.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia (Associated Features)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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118. ETH-0117 (rev 1) · Best answer: B

A child welfare social worker is building a case plan with a parent whose children are in foster care. The parent agrees to every proposed goal without comment. Earlier, the parent described distrust of the agency after relatives lost custody of their children. What is the **MOST** appropriate response by the social worker?

A. Finalize the plan, since the parent agreed to each goal
B. Acknowledge the agency's authority and ask the parent's own goals
C. Tell the parent they seem to be holding back opinions
D. Reassure the parent that the agency's aim is reunification

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). Treating silent agreement as commitment assumes the plan reflects the parent's wishes. Given the agency's power and the parent's stated distrust, the agreement may be deference, and the plan may fail later.

B **Correct.** A parent in child welfare faces real power: the agency can affect custody. Agreement without comment may reflect that power and past distrust rather than true buy-in. Naming the agency's authority openly and inviting the parent's own goals shares power and makes honest input safer (NASW Code 1.02, 1.05(c)). It beats D because reassurance about reunification is kind and true, but it leaves the power gap unspoken and still does not draw out the parent's views.

C **Incorrect** (trap: Too directive). Pointing out that the parent seems to be holding back interprets the behavior before trust exists. It can feel like an accusation from someone with power over the family and may deepen the silence.

D **Incorrect** (trap: Good but not best). Reassurance is accurate and well-meant, but it speaks for the agency without addressing the power imbalance. The parent still has no clear invitation to disagree or add goals.

Takeaway: When a client defers to a worker with authority, name the power openly and invite the client's own goals before finalizing plans.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.05(c)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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119. INT-1581 (rev 2) · Best answer: A

Which type of validity is supported when a panel of experts judges that a scale's items represent every part of the construct it is meant to measure?

A. Content validity
B. Face validity
C. Construct validity
D. Concurrent validity

Why each option is right or wrong

A **Correct.** Content validity, which the current testing standards call evidence based on test content, concerns whether a measure's items adequately sample the whole domain of the construct, and it is usually judged systematically by experts in the area (AERA, APA & NCME, 2014; Rubin & Babbie, 2017). The decisive detail is that experts checked whether every part of the construct is covered. This beats B because face validity is only a surface judgment that the items look relevant, often made by test takers or laypeople, and it does not examine coverage.

B **Incorrect** (trap: Wrong model or concept). Face validity means a measure looks like it measures what it claims to, on the surface. It is an informal impression and does not involve experts checking that all parts of the construct are covered.

C **Incorrect** (trap: Wrong model or concept). Construct validity is shown when scores relate to other measures as theory predicts, correlating with similar measures and not with unrelated ones (Rubin & Babbie, 2017). It rests on patterns in data, not on expert review of items.

D **Incorrect** (trap: Wrong model or concept). Concurrent validity is shown when scores agree with a criterion measured at the same time, such as an established diagnostic interview. No outside criterion is involved here.

Takeaway: Content validity asks whether items cover the whole construct, judged by experts; face validity asks only whether items look relevant.

Sources: AERA, APA & NCME (2014). Standards for Educational and Psychological Testing. American Educational Research Association, Ch. 1: Validity, evidence based on test content; Rubin, A., & Babbie, E. R. (2017). Research Methods for Social Work (9th ed.). Cengage, Measurement: types of validity (face, content, criterion-related, construct)
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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120. INT-1576 (rev 1) · Best answer: B

An adult client with panic disorder has logged panic attacks daily since intake. Over 3 baseline weeks the count held steady at about 6 per week. During 6 weeks of exposure-based treatment, it has fallen steadily to 2 per week. The client read about a newer therapy online and asks about switching to it. Which response is **BEST**?

A. Agree to switch to the newer therapy as the client requests
B. Review the progress data together and discuss continuing
C. Blend the newer therapy's techniques into current sessions

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). The client does make the final choice, but an informed choice needs the data. Switching without reviewing a clear downward trend risks abandoning a treatment that is working. Agreeing would be reasonable if the client still prefers the change after seeing the results.

B **Correct.** Practice evaluation data are meant to guide decisions. A stable baseline followed by a steady drop in level and trend is the pattern that supports continuing the current intervention (Bloom, Fischer & Orme, 2009). Showing the client the data lets the client make an informed choice, which respects self-determination (NASW Code 1.02). This beats C because adding new techniques while the current plan is working changes two things at once and could dilute an approach that is clearly helping.

C **Incorrect** (trap: Misuses the technique). Mixing in new techniques mid-treatment changes two things at once. If progress continues or stalls, no one can tell which element was responsible, and the extra material may water down an approach that is already reducing panic attacks.

Takeaway: When progress data show clear improvement, share them with the client and support continuing what works before adopting something new.

Sources: Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, visual analysis of single-system data: level and trend; NASW Code of Ethics (2021), 1.02

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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121. INT-1153 (rev 3) · Best answer: C

A couple seeks help for frequent arguments about parenting. In separate screening interviews, each partner describes one argument 8 months ago in which both pushed each other, and each takes responsibility for their part. Both say they feel safe at home, make money and social decisions together, and want to stay together. What is the **MOST** appropriate plan?

A. Refer each partner to a separate violence intervention program first

B. Begin couples sessions once both sign a no-violence agreement, with no further screening

C. Begin couples sessions that include periodic individual safety check-ins

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Separate services are the right path when screening shows coercive control, fear, or serious violence, because joint sessions could then endanger a victim. These interviews describe one mutual, low-level incident without those features, so this plan withholds appropriate couples care.

B **Incorrect** (trap: Misuses the technique). Couples work fits this couple, and a no-violence agreement can be part of it. A signed promise is not a safety assessment, though. After any physical incident, safety is monitored privately throughout conjoint treatment, because a partner may not voice fear in front of the other.

C **Correct.** Johnson (2008) distinguishes situational couple violence, which grows out of conflict without a pattern of control, from coercive, controlling violence. Conjoint treatment may be considered for low-level situational violence when both partners feel safe and accept responsibility, with continued individual safety screening (Stith, McCollum & Rosen, 2011). The separate interviews show one mutual push, shared decisions, no fear, and a shared wish to stay together. It beats A because nothing here shows the coercive control or fear that would make joint sessions unsafe.

Takeaway: Mutual, isolated, low-level partner violence without fear can permit joint sessions, provided safety is rechecked privately throughout.

Sources: Johnson, M. P. (2008). A Typology of Domestic Violence: Intimate Terrorism, Violent Resistance, and Situational Couple Violence. Northeastern University Press, situational couple violence versus intimate terrorism; Stith, S. M., McCollum, E. E., & Rosen, K. H. (2011). Couples Therapy for Domestic Violence: Finding Safe Solutions. American Psychological Association, assessment and screening for inclusion in conjoint treatment

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.27 Couples treatment approaches · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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122. ASM-1128 (rev 1) · Best answer: B

Which DSM-5-TR tool rates how well a person gets around, cares for self, gets along with others and takes part in community life?

A. Global Assessment of Functioning (GAF)
B. WHODAS 2.0
C. Level 1 Cross-Cutting Symptom Measure
D. Cultural Formulation Interview

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The GAF was a single 1-to-100 rating in the old multiaxial system. DSM-5 removed it, partly because it blended symptoms, risk and functioning, and DSM-5-TR uses the WHODAS 2.0 instead.

B **Correct.** The World Health Organization Disability Assessment Schedule 2.0 (WHODAS 2.0) is the functioning measure in DSM-5-TR Section III. It rates six areas: understanding and communicating, getting around, self-care, getting along with people, life activities, and participation in society. Repeating it over time can show whether functioning improves with treatment. It beats A because the GAF scale was dropped along with the multiaxial system in DSM-5 and does not appear in DSM-5-TR (DSM-5-TR, Section III).

C **Incorrect** (trap: Wrong model or concept). The Level 1 Cross-Cutting Symptom Measure screens symptom domains such as mood, anxiety and sleep across diagnoses. It flags symptoms for follow-up rather than rating how well a person functions in daily life.

D **Incorrect** (trap: Wrong model or concept). The Cultural Formulation Interview gathers the client's cultural view of the problem, its causes and help-seeking. It informs diagnosis and planning but does not score daily functioning.

Takeaway: DSM-5-TR rates functioning with the WHODAS 2.0, a Section III measure; the GAF score left with the multiaxial system.

Sources: DSM-5-TR (APA, 2022), Section III, Assessment Measures: WHODAS 2.0
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3